BERNSTEIN
SOCIETE GENERALE GROUP

18 March 2026

Initiation

European Biopharmaceuticals

Novo Nordisk A/S

Rating

Underperform

Price Target

NOVOB.DC **175.00 DKK**


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Novo Nordisk: Initiate at Underperform: Knife Still Falling?

Although the stock is close to five year lows, we don't think the EPS downgrade cycle is over. Moreover, our bull-bear analysis is negatively skewed and the catalyst cupboard looks bare to us. We also don't believe that the 10x PE26 implied by our Price Target is extreme as it implies premia to stocks with deeper moats like Sanofi (O) on global sales regression analysis.

Why do we project up to 29% downside to the consensus '26-36 EPSe? We expect the US obesity market to peak at $85bn in 2036, but our group sales forecasts are upto 18% < consensus (BBG throughout) as we expect approved drugs (in particular Ozempic, Wegovy, and Oral Sema) and the pipeline (mostly CagriSema) to underperform in the US due to lower volume share & more price deflation. We also expect the US semaglutide compound patent expiry in 2032 to drive greater mix deterioration and thus more margin contraction.

What's the risk reward? Our bull-bear analysis implies +43% upside vs -53% downside to our '35e EBIT (DCF proxy). The approved portfolio accounts for 11% of the upside and discounts Novo delivering consensus for Ozempic, Wegovy, and Oral Sema. Similarly, the remaining 32% upside from pipeline discounts CagriSema consensus (but unlike 'the market' we believe this is a niche, rather than mass consumer, obesity and diabetes drug).

Why can't we find any material catalysts? We don't expect new data to inflect expectations for CagriSema. Moreover, considering that amycretin phase 3 data isn't expected until 2028/2029 we believe this will also just be a niche product. And we can't get excited by ziltivekimab (phase 3 data due in 2027) due to competitive crowding in IL-6.

Investment Implications

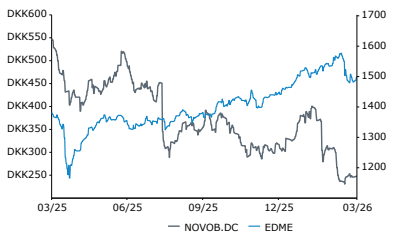
Why's a 10x target PE appropriate? We calculate our Price Target via the average of our DCF (DKK 191; 8% WACC & 0% perpetuity growth) and '26-'28 EV/EBITA multiples (DKK 160; 36% discount to EU peers) to be consistent with our existing EU biopharma coverage. Our PT implies a 10x PE26 for discounts to EU peers, Lilly (O; Courtney Breen) and Novo's 20-year history of 40%, 67% and 61% respectively. (On PE30 the PT discounts to EU peers and Lilly are 30% and 53% respectively). We believe these implied relative valuations are robust considering Novo's below average growth, portfolio concentration risk and mixed track record. Value investors should heed that our Novo PT implies a premium to Sanofi (O) on our global '26e EV/sales to sales CAGR (26-31e) regression even though the latter enjoys a deeper moat because of its large vaccines franchise.

Close Date	16 Mar 2026
NOVOB.DC Close Price (DKK)	248.30
Price Target (DKK)	175.00
Upside/(Downside)	(30)%
52-Week Range	573.30/224.25
EDME	1,496.09
FYE	Dec
Div Yield	4.7%
Market Cap (DKK) (B)	1,112.45
EV (DKK) (B)	1,216.45

Performance	YTD	1M	6M	12M
Absolute (%)	(23.4)	(19.6)	(32.6)	(54.3)
EDME (%)	1.7	(4.1)	8.7	8.6
Relative (%)	(25.1)	(15.5)	(41.3)	(62.9)

Source: Bloomberg, Bernstein estimates and analysis.

Price Performance, 1YR



Adjusted EPS	F25A	F26E	F27E
NOVOB.DC (DKK)	23.02	17.51	18.83

Financials	F25A	F26E	F27E	CAGR
Revenues (M)	309,064	270,525	271,032	--

Valuation Metrics	F25A	F26E	F27E
Adjusted P/E (x)	10.8	14.2	13.2
Reported P/E (x)	10.8	14.2	13.2

Source: Bloomberg, Bernstein estimates and analysis.

See the Disclosure Appendix of this report for required disclosures, analyst certifications and other important information. Alternatively, visit our Global Research Disclosure Website.

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DETAILS

FUNDAMENTALS DEEP DIVE: NOVO NORDISK (UP; UNDERPERFORM); WHY'S THE KNIFE STILL FALLING?

WHY DO WE PROJECT BELOW AVERAGE GROWTH?

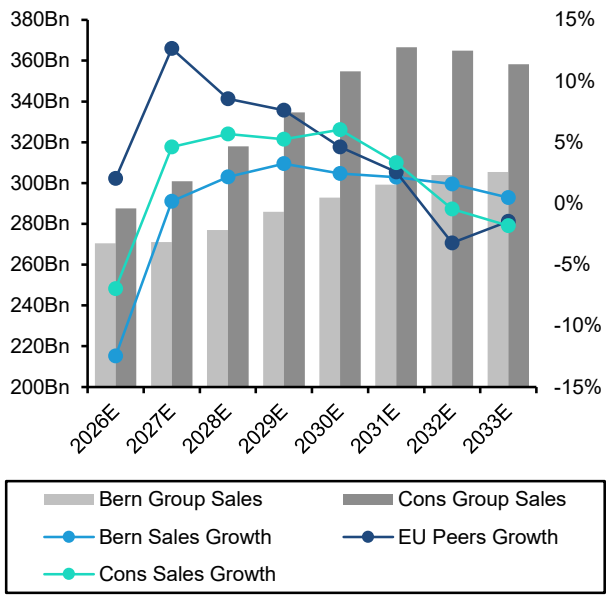
We project Novo Nordisk to deliver sales growth below EU biopharma peers 2026-2031 inclusive (Exhibit 1) for two reasons:

1/Semaglutide loss of exclusivity: In 2025, products containing semaglutide (Ozempic, Rybelsus, and Wegovy) accounted for 74% of Novo's group sales. Novo has hitched its growth prospects on the Wegovy Pill (oral semaglutide) and CagriSema (cagrilintide + semaglutide) over the medium term. Incorporating loss of exclusivity (LOE) in key international markets in 2026 (Canada, Brazil, India, and China), and compound patent expiries in the EU and US in 2031 and 2032 (respectively), we estimate that 1/3 of group sales are at risk of generic competition in 2026, rising to 2/3 in 2032 (Exhibit 3).

2/Weakening positions in obesity and diabetes markets: Having entered the GLP-1 market for both diabetes (2017, through Ozempic) and obesity (2021, through Wegovy), Novo's semaglutide has lost considerable market share to Lilly in both diabetes (vs Mounjaro) and obesity (vs Zepbound). We think these dynamics are entrenched and see no prospect of a share reversal. We forecast -12% group sales growth in 2026 (driven by US price deflation) and then anemic growth across to 2033 (Exhibit 1).

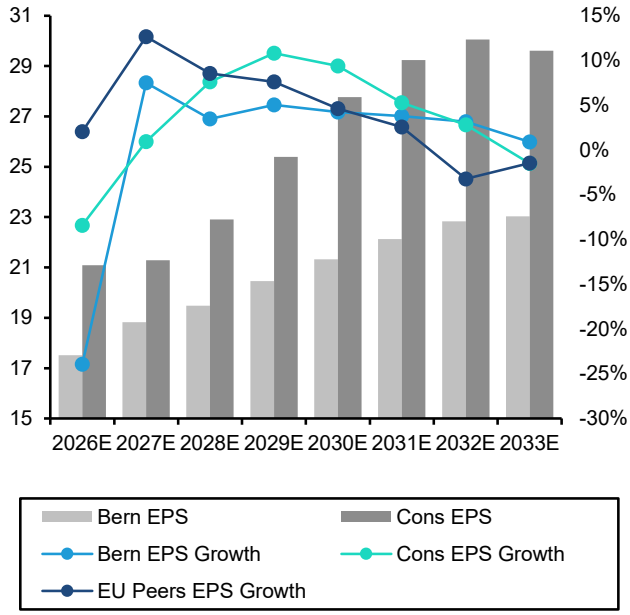
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EXHIBIT 1: **Novo Nordisk Group Sales and Sales Growth: Bernstein vs Consensus vs EU Peers**



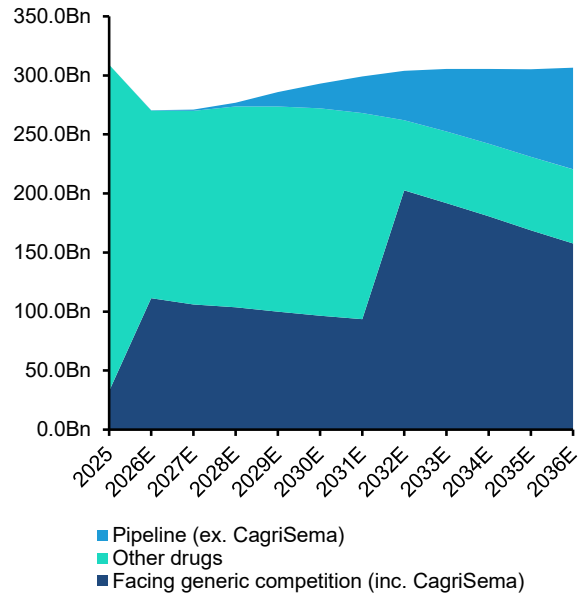
EU peers are AZN, GSK, ROG, NOVN, SAN, MRK GY, ARGX and GMAB
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 2: **Novo Nordisk EPS and EPS Growth: Bernstein vs Consensus vs EU Peers**



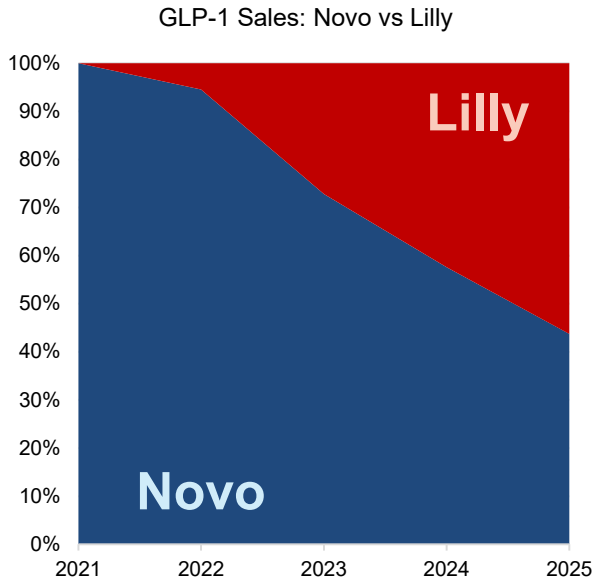
EU peers are AZN, GSK, ROG, NOVN, SAN, MRK GY, ARGX and GMAB
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 3: **Semaglutide (active ingredient in Ozempic, Rybelsus, Wegovy, Oral Sema, and CagriSema) is facing loss of exclusivity from 2026**



Semaglutide LOE ex. US in 2026 and US in 2032
Source: Company Reports, Bernstein analysis and estimates

EXHIBIT 4: **Novo vs Lilly US sales shares of GLP-1s**



Novo = US Ozempic and Wegovy Sales

Lilly = US Mounjaro and Zepbound Sales
Source: Company Reports, Bernstein analysis

We project EPS growth for Novo below EU peers '26-29e inclusive (Exhibit 2)**.** The variance is wider than for sales because we forecast margin contraction caused by US deflation (creating a negative mix effect in the diabetes and obesity GLP-1 franchises) and semaglutide patent expiries across key regions ex. US.

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Consequently, we forecast an EPS CAGR of 5% '26-2031e and 0.6% '32-36e. In our view, US pricing headwinds in obesity and diabetes GLP-1 markets will lead to a -24% drop in EPS in 2026 (Exhibit 5). Over the mid-term (until 2031) we believe that products such as the Wegovy Pill (Oral Sema), CagriSema (which is due to launch in both obesity and diabetes), and increased penetration of the US obesity markets can help deliver mid-single digit (MSD) EPS growth pre the LOE (loss of exclusivity) of semaglutide in the US (which is due in 2032).

Post the semaglutide US compound patent expiry in 2032 we expect EPS to almost flat-line until 2036 (which is the end of our bottom up forecast period). This is because we do not think Novo's remaining approved portfolio and pipeline will be able to mitigate the semaglutide patent expiries.

EXHIBIT 5: **Novo Nordisk EPS (DKK): Bernstein vs Consensus**

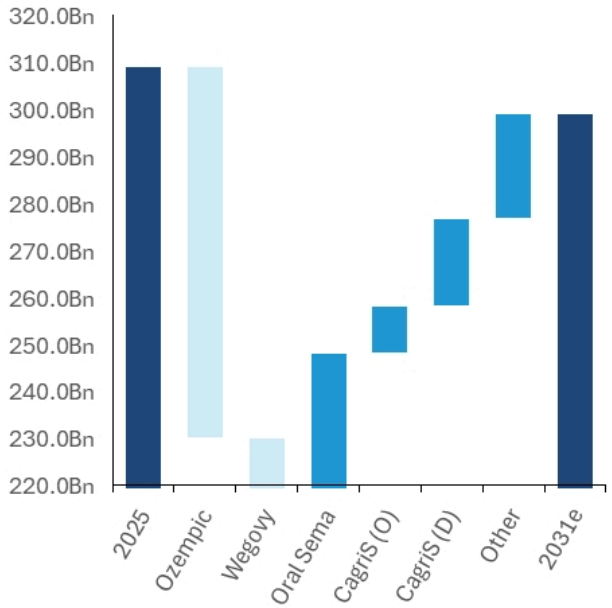
Basic Diluted EPS	2025	2026E	2027E	2028E	2029E	2030E	2031E	CAGR (26-31e)
Bernstein	23.02	17.51	18.82	19.48	20.46	21.32	22.13	4.8%
Consensus	23.03	21.09	21.29	22.91	25.39	27.77	29.24	6.7%
% difference vs consensus	0%	-17%	-12%	-15%	-19%	-23%	-24%	
Bernstein growth		-24%	8%	3%	5%	4%	4%	
Consensus growth		-8%	1%	8%	11%	9%	5%	

Source: Bloomberg, company reports, Bernstein analysis and estimates

We do not think the Wegovy Pill and CagriSema launches will be enough to offset the loss of sales from Ozempic and Wegovy. We thus do not expect Novo to deliver sustainable group sales growth between 2025 and 2031e (Exhibit 6).

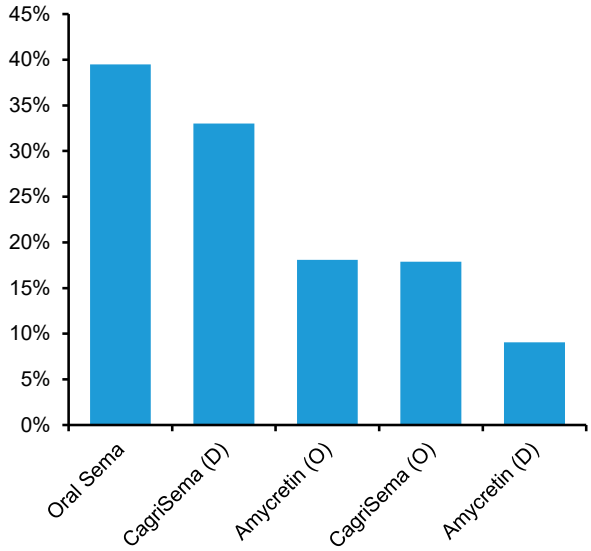
Between 2026 and 2031 we project cumulative sales for the Wegovy Pill and CagriSema (Novo's highest profile launch prior to the 2032 US patent expiry) of DKK60bn (Exhibit 6). But over this period we expect DKK 90bn of cumulative lost sales from Ozempic and Wegovy. From 2026-31E, we thus forecast that the Wegovy Pill will account for 40% of group sales CAGR (Exhibit 7).

EXHIBIT 6: **We forecast group sales to fall over the next five years (DKK)**



O = Obesity; D = Diabetes
Source: Company Reports, Bernstein Bernstein analysis and estimates

EXHIBIT 7: **We project the Wegovy Pill (Oral Sema) to account for 40% of our group sales CAGR '27-31e**

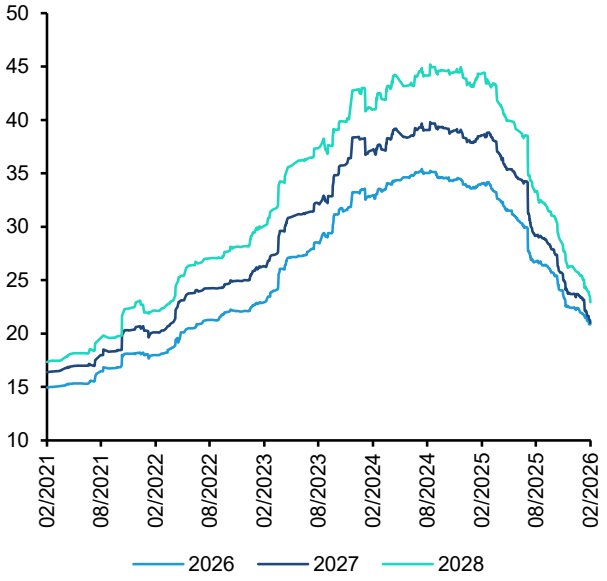


O = Obesity; D = Diabetes
Source: Company Reports, Bernstein Bernstein analysis and estimates

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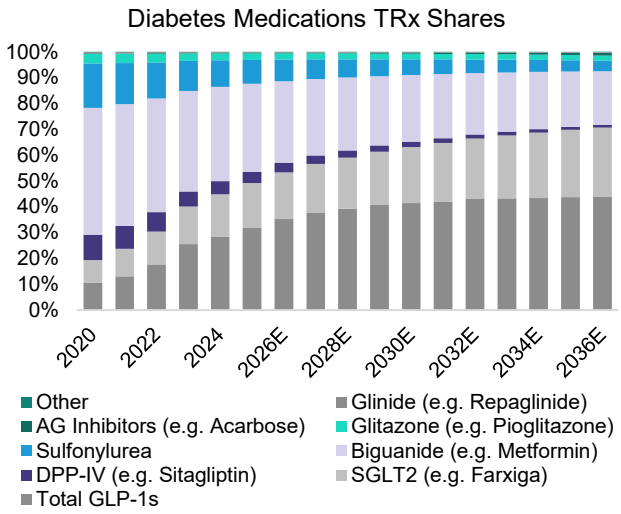
WHY DO WE EXPECT DOWNGRADES TO CONSENSUS?

EXHIBIT 8: **Change in consensus EPS estimates (DKK)**



Source: Bloomberg, Bernstein Analysis

EXHIBIT 9: **We expect GLP-1s to secure 40% of US diabetes TRx volumes**



GLP-1s include our forecasts for generic Ozempic and Rybelsus

TRX = Total US Prescriptions
Source: IQVIA, Bernstein Bernstein analysis and estimates

Novo's Earnings Story so Far: Meteoric Rise, Unfortunate Fall

Why did Novo deliver a sustained period of earnings upgrades? To recap, Novo was an early player in the GLP-1 market, with Victoza (active ingredient liraglutide) gaining FDA approval in 2010. This was the first once-daily human GLP-1 approved for type 2 diabetes.

1. **GLP1 penetration of US diabetes market.** Novo launched Ozempic in the US in 2017, a once weekly GLP-1 for type 2 diabetes management. Since then, GLP-1s penetration in the US diabetes market has steadily increased (Exhibit 9). In our view this is primarily because, unlike insulin, these products lead to weight loss rather than weight gain.

2. **Inflection of the US obesity market for GLP-1s.** The weight-loss benefits enjoyed by patients on Ozempic increased investors' expectations of the potential for GLP-1s to be used as broader weight management medications - with many patients using Ozempic "off-label" (i.e. for weight-loss and not for diabetes management). Subsequently, data from the phase 3 SELECT outcomes trial in 3Q23 inflected perceptions of the potential of GLP-1s in weight loss. This is because in SELECT Wegovy lowered cardiovascular risk (a composite of reducing heart attacks, strokes, and other CV related deaths) by 20%. This proved that GLP-1s deliver medicinal value beyond weight-loss.

What has caused this downgrade cycle? There have been three major events in our view:

- **Poor execution on manufacturing.** We think Novo's strategy for semaglutide production is a prime study for "failure to prepare is preparation to fail". Here is a recap of key events:
  - Novo let demand for semaglutide products (namely Ozempic and Wegovy) run ahead of their (known) installed capacity for quality checking and fill-finishing. In March 2022, the FDA announced a shortage of semaglutide, under section 503b. This led to compounding pharmacies entering the US market to ease the semaglutide shortage.
  - Novo has invested $25bn since 2022 in capex to bolster fill-finish (putting the active pharmaceutical ingredient (API) into pen injection devices). Then, in early 2025, the FDA announced that the semaglutide shortage was over. However, compounding pharmacies have proven very difficult to remove from the market. Our channel checks indicate are these drugs still command 1/3 of the US market on a volume basis.

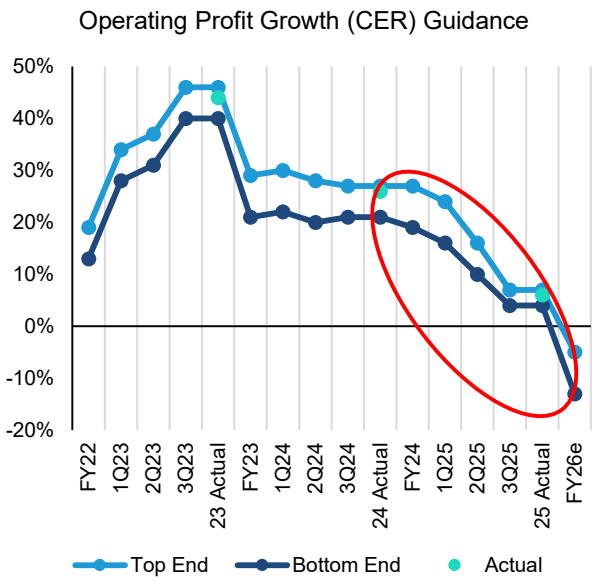
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- We think Novo has suffered heavily on three fronts: **A/** loss of Ozempic and Wegovy sales 2022-present, **B/** greater than expected capex investments from a position of relative weakness, and **C/** allowing competitors (compounders and Lilly) to take share.

- **Three profit warnings in 2025 and another one in 2026** (Exhibit 10)**.** Novo cut its sales and EBIT growth guidance for FY25 three times across 2025 (at 1Q, 2Q, and 3Q), citing lower penetration of GLP-1s in the US (1Q), in international markets (2Q), and the slower than expected exit of compounders (1Q, 2Q and 3Q). Management also announced a large restructuring program in early September 2025, cutting 9k jobs. Then the first wave of public channel GLP-1 price cuts in the US led by The White House in November contributed to the guidance cut at 3Q25. Novo is now guiding for negative group sales growth at constant currency (in the 5-13% range) in 2026 because of more US price cuts needed to stimulate long term volume growth.

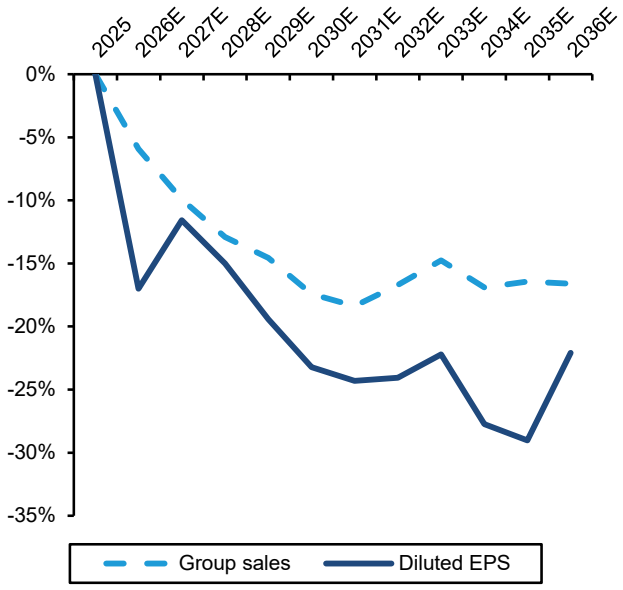
- **CagriSema disappointment(s) eroding confidence in the pipeline.** CagriSema was initially billed as Novo's answer to tirzepatide (active ingredient in Lilly's Mounjaro (diabetes) and Zepbound (obesity)). More specifically, Novo had hoped CagriSema (cagrilintide (amylin) + semaglutide) could achieve 25% weight loss. (For context, Zepbound delivered 21%, placebo adjusted weight loss, in the phase 3 SURMOUNT-1 obesity trials). But in REDEFINE-1 (which read out December 2024), CagriSema disappointed investors, achieving 20.4% placebo adjusted weight-loss. In REDEFINE-4, which was a head-to-head trial of CagriSema vs Zepbound (which read out in February 2026) the former failed to meet its primary endpoint of non-inferiority (i.e. it was inferior to Zepbound for weight loss). We believe this chain of events has dented investors' confidence in Novo's R&D execution.

EXHIBIT 10: **Novo Nordisk: Operating Profit Guidance 2022-Present**



CER = Constant Exchange Rates
Source: Company Reports, Bernstein Analysis

EXHIBIT 11: **Bernstein sits well below consensus on both group sales and EPS**



Source: BBG Consensus, Company Reports, Bernstein analysis and estimates

We forecast upto 29% downside to the consensus EPS forecasts as we expect top line underperformance to be exacerbated by worse profitability and lower cash generation (Exhibit 11)**.**

On the top line we forecast material downside vs consensus across Novo's five biggest franchises (three of which accounted for 74% of 2025 group sales). For the products currently on the market (Ozempic, Wegovy, and Oral Sema), we sit 30%, 15%, and 13% below consensus respectively (Exhibit 12).

- **Our Wegovy estimates** (read more here) assume a +4% CAGR (26-36e) of US prescriptions' ('script'; volume) and -10% annual US price declines across all channels across 2027-36e. We think the current negative share trends vs Zepbound

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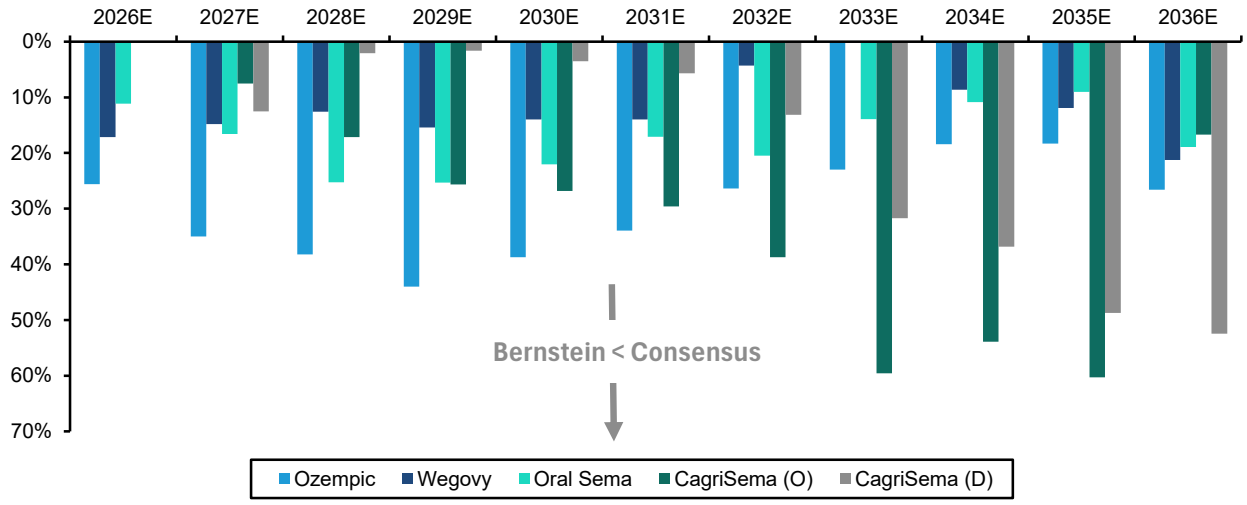
will continue and that Wegovy US price declines will not cause mid-to-long term volumes to surprise on the upside. All in all, we think consensus is too bullish on both Wegovy US TRx (prescription) volume growth and the magnitude of on US price declines.

- **Our Oral Sema ('Wegovy in a Pill') forecast** for 2026 (launch year) is 25% below consensus (more here). From 2027 onwards, we project further downside to consensus. Similarly to our Wegovy forecasts, we are more conservative on US prescription volumes than consensus (we think Oral Sema will command 10% of TRx volumes in the US obesity market in 2032 and 7% in 2036; Exhibit 84).

- **Our Ozempic projections** (see more here) assume -2% US script growth until 2032 (when loss of exclusivity occurs) and then -3%-5% volume declines until 2036. In 2026 and 2027 we assume US deflation of 30% and 15% respectively. We also model -10% US price declines from 2028-2036.

- **We also expect CagriSema** (obesity and diabetes) to underperform consensus. This is because we view this as a niche, not a mass market, drug in both diabetes and obesity. We forecast a 3% share of US TRx volumes in 2036e in the US obesity and diabetes markets respectively. Despite the recent disappointing REDEFINE-4 headline data, in our view, consensus still hasn't properly discounted CagriSema's likely niche market position in our view.

EXHIBIT 12: **Sources of sales variance: Bernstein vs Consensus**



O = obesity; D = diabetes
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

We project double-digit downgrades to the consensus EPSe until 2033 (Exhibit 13)**due to:**

1. **Lower group sales growth projections:** we forecast weaker US volume growth and more US deflation for GLP-1s in both obesity and diabetes, with a more bearish outlook on Novo's pipeline assets (as discussed above).

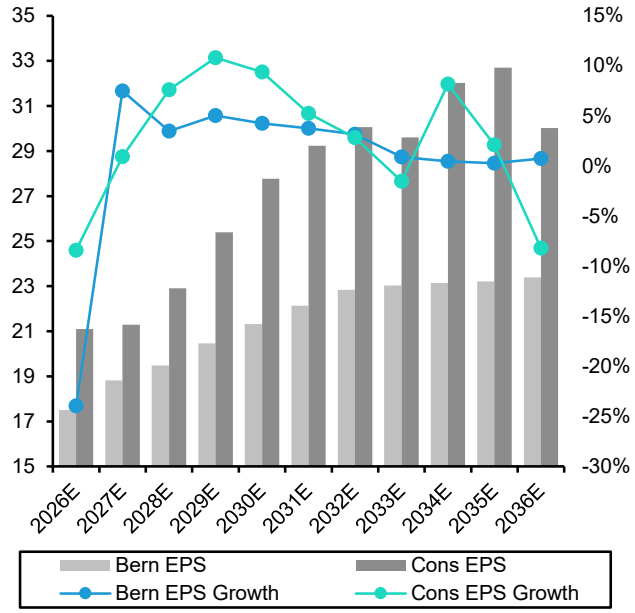
2. **More aggressive margin contraction:** we believe that consensus is underestimating the level of margin contraction which will occur post the semaglutide US compound patent expiry in 2032. In our view, Novo will likely suffer from a combination of **a)** worsening product mix and **b)** SG&A headwinds (Exhibit 14).

- We forecast a greater mix hit at COGS because we think the injectables vs orals share split will impair product mix. As semaglutide is a peptide (short chain protein), it suffers from low bio-availability in the oral formulation. Whilst we think this will help protect volumes against generic erosion, we believe this will come at a cost at the margin level (Oral Sema is more expensive to make as it contains much more active pharmaceutical ingredient (API) than Wegovy does).

- We also forecast a profitability hit at SG&A because we believe that this line contains significant fixed costs. Hence, we do not believe that Novo will be able to deliver a percentage reduction in absolute SG&A costs which will mirror the percentage decline at group sales).

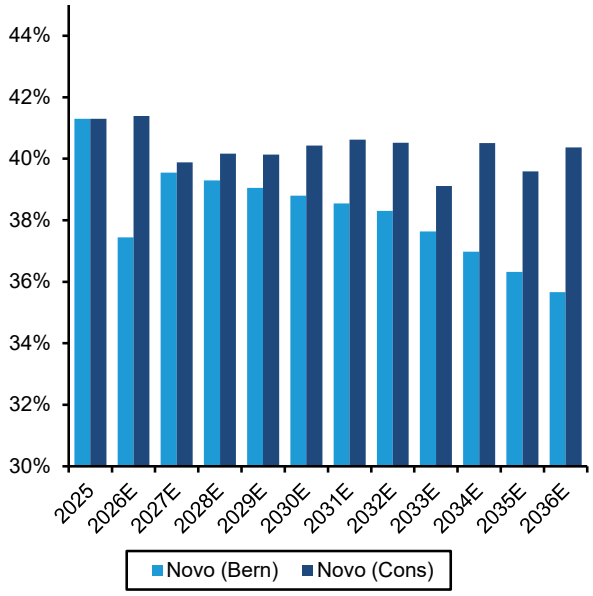
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EXHIBIT 13: **Novo Nordisk EPS and EPS growth: Bernstein vs Consensus**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 14: **Novo Nordisk Operating Profit Margin: Bernstein vs Consensus**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

WHAT'S THE RISK REWARD?

An Overview of Our Bull-Bear Scenario Analysis

We have constructed a bull-bear scenario approach for Novo sales and EBIT impact across to 2035 (we believe 2035e EBIT is a good DCF proxy). We have focused our bull-bear on the key portfolio products (Ozempic, Wegovy, Oral Sema) and major late stage pipeline assets (CagriSema in obesity and diabetes). In Exhibit 27 we provide a summary table.

Our bull scenario:

- **+29% upside to 2035E sales and 43% upside to 2035E EBIT**
  - Considering the variance of Bernstein vs consensus across portfolio and pipeline assets, we have taken consensus estimates as our "bull case" for Ozempic, Wegovy, Oral Sema, and CagriSema. In Exhibit 15 and Exhibit 16 we illustrate how our sales estimates compare to consensus for 2035e sales and EBIT respectively.
  - **In the approved portfolio**, we project upsides of 4%, +2%, and +2% to Ozempic, Wegovy, and Oral Sema 2035E sales respectively. The corresponding EBIT upsides are +5%, +3%, and +3% respectively.
  - **In the pipeline**, our bull case implies +12% and +10% upside to 2035E sales for CagriSema in obesity and diabetes respectively. The corresponding EBIT upsides are +17% and +14% respectively. But we believe that consensus is still too optimistic on the positioning of CagriSema in both obesity and diabetes (we believe consensus is unwisely discounting mass-market, rather than niche, use). We forecast peak US CagriSema TRx volumes to reach 3% of the market in diabetes and obesity respectively (Exhibit 84 and Exhibit 92).

Our bear scenario:

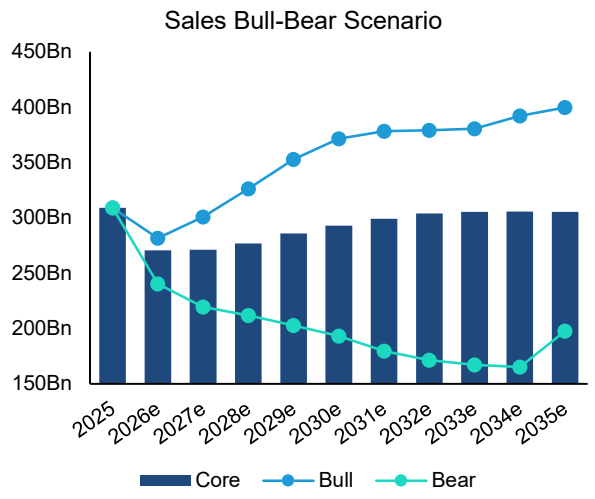
- **-36% downside to 2035E sales and -53% downside to EBIT**
  - To model downside scenarios, we have softened the assumptions in our underlying US obesity and diabetes models. In Exhibit 15 and Exhibit 16 we illustrate how our core forecasts compare to our bear case estimates for sales and EBIT respectively.

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- **For the on-market products**, we model US TRx and price dynamics and make a revenue adjustment for ex. US sales. We assume faster US volume declines for Ozempic and Wegovy (-2% and -5% respectively) across 2030-35E and US price deflation towards the top end of the normal range in the diabetes and obesity markets (-15% and -20% yoy respectively across from 2026). For Oral Sema, we have modeled slower penetration (5% US TRx growth over 2030-35E) and the same price deflation as Wegovy (-20% yoy). All in all, this results in respective downsides to 2035E sales of -4%, -9%, and -9% (Exhibit 27). The corresponding respective downside at 2035e EBIT are -6%, -13%, and -14%(also Exhibit 27).

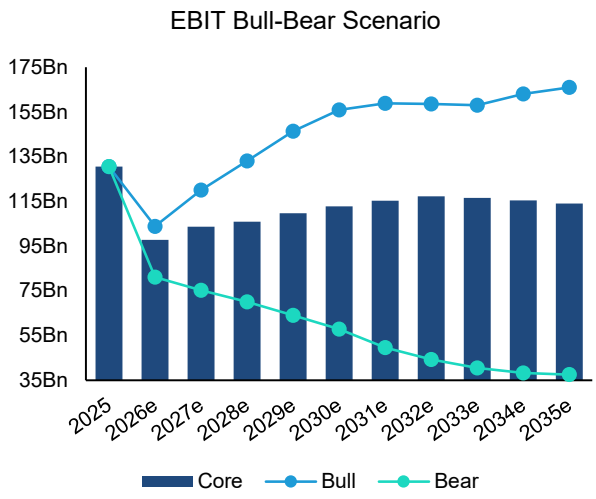
- **In the pipeline**, we modeled a worst-case scenario of phase 3 projects not launching (i.e. 0 sales across 2026-36E. Collectively, this implied -14% sales risk and -20% EBIT risk (both in 2035E).

EXHIBIT 15: **Bernstein's Novo Nordisk Bull-Bear: Sales (DKK)**



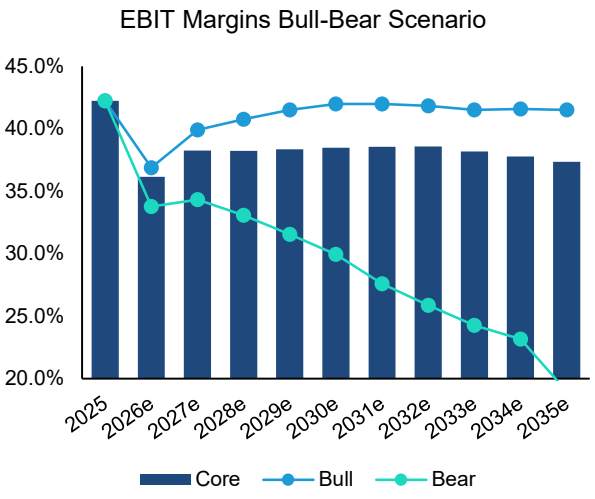
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 16: **Bernstein's Novo Nordisk Bull-Bear: EBIT (DKK)**



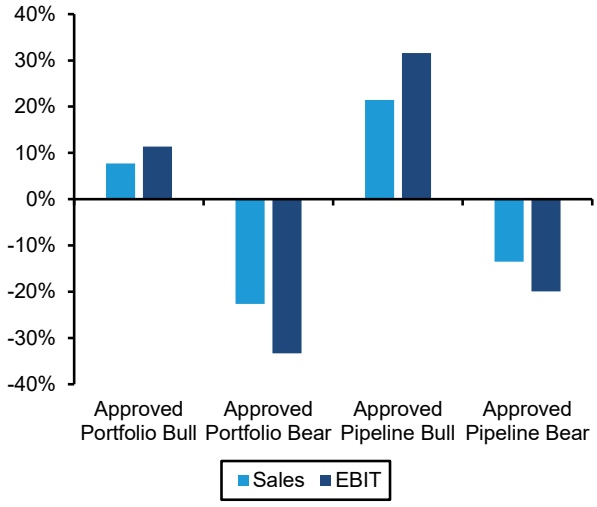
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 17: **Bernstein's Novo Nordisk Bull-Bear: EBIT Margin**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 18: **Bernstein's Novo Nordisk Sales and EBIT scenario analysis: Sources of Bull-Bear**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

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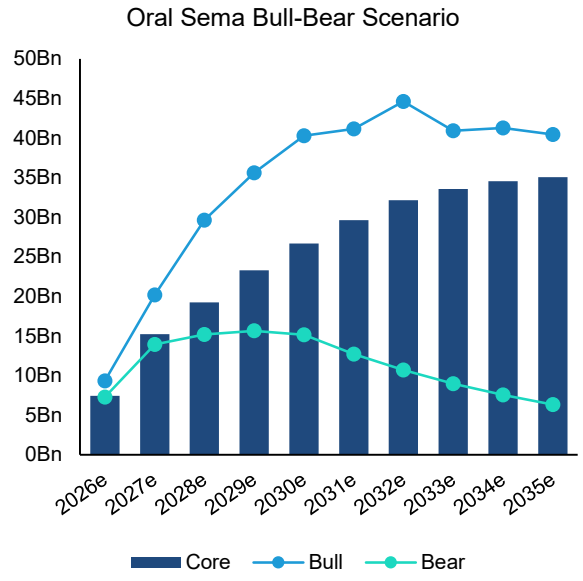
Where do we see the Most Risk to our Bull-Bear analysis?

The component of our bearish investment thesis which alerts us most is the potential for Oral Sema out-performance vs consensus. We have dedicated significant bandwidth to derive our assumptions for US volume growth, US price deflation and US market share (all of which can be read in depth here). Hence, we believe that they are robust.

In the short term ('26-27E) our bull, core, and bear scenarios are somewhat similar to consensus in terms of sales and TRx share of the US obesity market. We provide more detail on Oral Sema vs orforglipron here. In our view, the launch of Oral Sema in the US (January 2026) will not be able to mitigate aggressive US price cuts across Novo's broader obesity portfolio and drive top-line growth into 2026 and 2027. We project that group revenues will not grow next year.

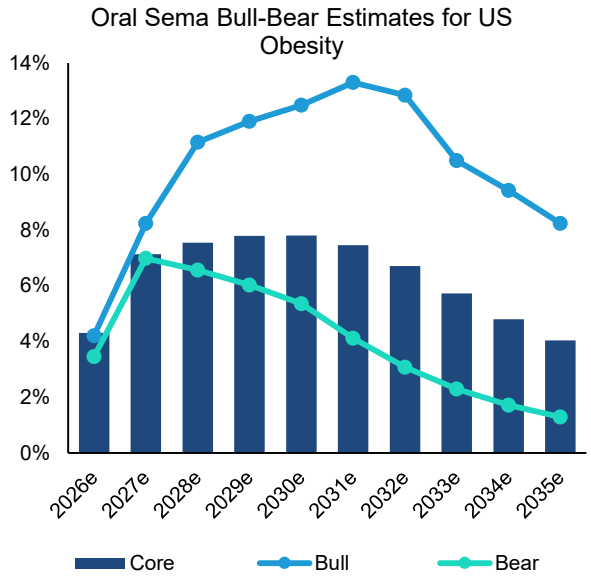
Over the medium term, we think Oral Sema will command a peak 11% share of US obesity prescriptions (with the peak coming in 2030e). We think consensus is much more bullish on where Oral Sema will land over the medium term (Exhibit 20). For clarity, in our 2031e base case, we estimate the US obesity market shares by volumes for 'high profile' tablets, 'high profile' injectable GLP1s and 'others' (ie players other than Novo and Lilly, but including compounders) to be 34%, 44% and 23% respectively. Within the 34% we expect to orforglipron (Lilly (O) to outperform Oral Sema by 2 to 1.

EXHIBIT 19: **Bernstein Oral Sema bull-bear analysis (sales, DKK)**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 20: **Implied Oral Sema shares in our US obesity model across our bull-bear scenarios**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

We are mindful of Novo's ambitions for Wegovy 7.2mg (high dose Wegovy). In November 2025, Novo submitted Wegovy (7.2mg) to the FDA for approval, using the Commissioner's National Priority Voucher (CNPV) gained as part of The White House deal (announced in November 2025). The CNPV reduces the FDA review time from 10-12 months to 1-2 months, from acceptance of the submission. Novo thus expects an FDA decision in 2Q26.

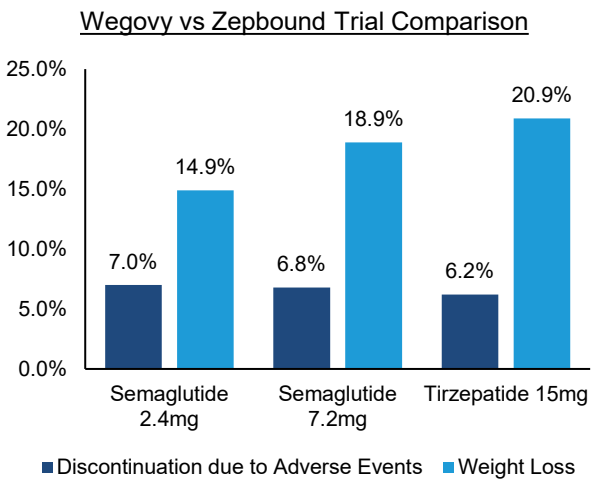
But we don't believe new trial data is enough to shift share dynamics. Data from the STEP-UP trial (published September 2025) demonstrated that the higher Wegovy dose (semaglutide 7.2mg) did increase patient weight loss vs semaglutide 2.4mg (Exhibit 21). We note, however, that headline weight loss was still below that of Zepbound. Thus, we do not believe the higher Wegovy dose has the potential to inflect US volume (script) trends materially. In our view, Zepbound remains the most effective weight loss drug on the US market, with a better tolerance profile. We expect both patients and physicians to share this view. Hence, we believe that script trends will continue to reflect patients' preference for Zepbound over Wegovy.

Our "bull-case" discounts consensus and implies +4% upside to our 2035e EBIT. We assume that the consensus estimates include Wegovy 7.2mg sales. Including the consensus' US Wegovy sales into our US obesity model, we estimate that consensus assumes Wegovy will have a benign share loss vs 2025 prescription volumes (Exhibit 22 (refer to the numbers inside

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the red circle) and Exhibit 24). As we will discuss in detail later, we believe this level of inflection is unrealistic.

EXHIBIT 21: **Zepbound achieved 5% more weight loss in SURMOUNT-1 than Wegovy did in SELECT-1**



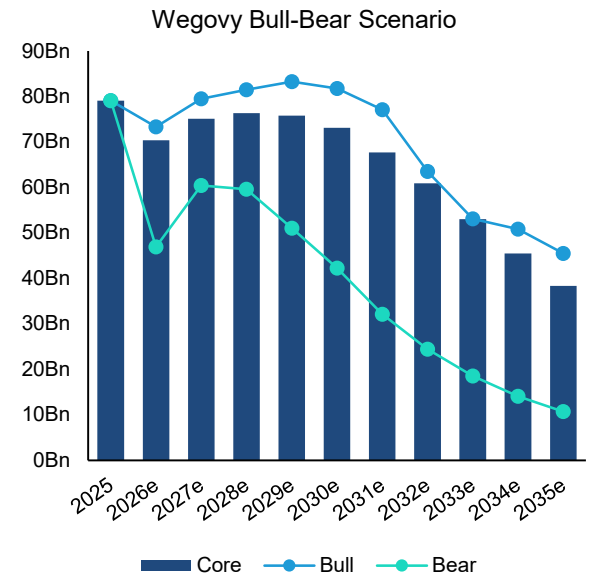
Sema 2.4 (STEP-1), Sema 7.2 (STEP-UP), Tirzepatide (SURMOUNT-1)
Source: NEJM, Bernstein Analysis

EXHIBIT 22: **We think consensus figures imply an unrealistic inflection of Wegovy scripts**

		Share of US Obesity TRx		
Drug	Scenario	2025A	2026E	2027E
	Bull (Consensus)	25.3%	23.4%	18.6%
	YoY Change		*-1.9%*	*-4.7%*
Wegovy (inj)	**Core (Bern)**	**25.3%**	**21.7%**	**17.0%**
	YoY Change		-3.6%	-4.7%
	Bear (Bern)	25.3%	14.0%	11.0%
	YoY Change		-11.3%	-3.0%
Zepbound		41.3%	51.8%	52.1%

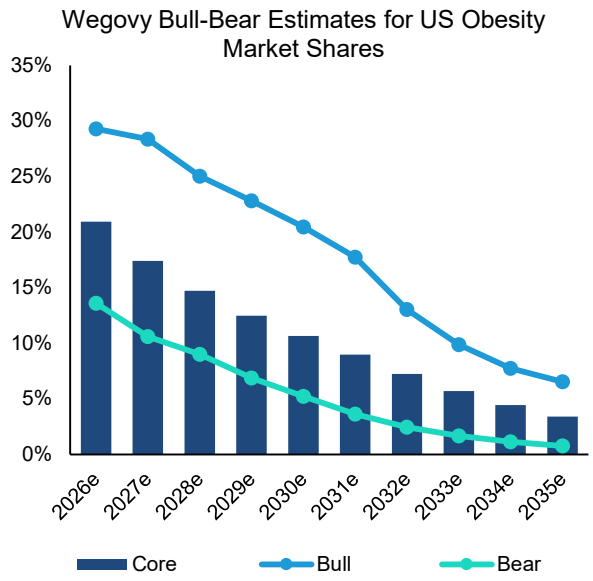
Shares are based on our US Obesity Model
Source: IQVIA, Company Reports, Bernstein analysis and estimates

EXHIBIT 23: **Bernstein Wegovy bull-bear analysis (sales, DKK)**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 24: **Implied Wegovy shares in our US obesity model across our bull-bear scenarios**

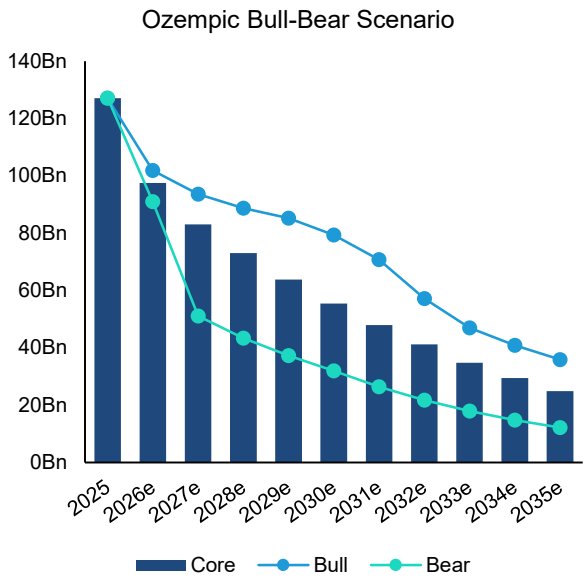


Source: Bloomberg, Company Reports, Bernstein analysis and estimates

To conclude, it is the accumulation of several smaller differences that lands us well below consensus at group sales. We are also very comfortable with our views on the value propositions for each of these products. **1/** we believe that Ozempic will continue to US lose share vs Mounjaro (Lilly), and **2/** we also believe that CagriSema will be a niche obesity and diabetes medication (we assume a peak of 3% US volume share in each indication).

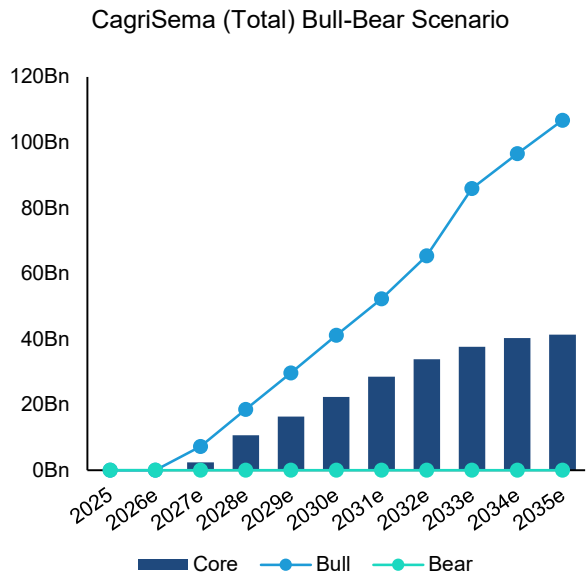
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EXHIBIT 25: **Bernstein Ozempic bull-bear analysis (sales, DKK)**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 26: **Bernstein CagriSema bull-bear analysis (sales, DKK)**



Source: Bloomberg, Company Reports, Bernstein analysis and estimates

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Justin Smith +44 20 7762 5899 justin.smith@bernsteinsg.com

EXHIBIT 27: **Bernstein Bull-Bear Scenarios: Novo Nordisk**

Novo Nordisk Bull Bear scenarios		2025	2026e	2027e	2028e	2029e	2030e	2031e	2032e	2033e	2034e	2035e	Sales	EBIT	2030-35 TRx Volume CAGR	2027-35 Price Deflation	Other
Major products risks																	
Ozempic	bull	127,089	101,859	93,558	88,723	85,231	79,349	70,826	57,214	46,955	40,873	35,913	**4%**	**5%**			Consensus Figures
Diabetes (GLP-1 once weekly)	**core**	**127,089**	**97,495**	**83,108**	**73,044**	**63,803**	**55,413**	**47,971**	**41,153**	**34,792**	**29,424**	**24,908**			-2%	-10%	Exit rate of share loss vs Mounjaro carried forward. Price declines in-line with historic market dynamics
	bear														-3%	-15%	Faster TRx volume collapse and larger price decreases from a) accelerated share loss vs Mounjaro, b) weak cash channel demand, c) large numbers of generic entrants, d) more than expected CagriSema cannibalization
		127,089	91,033	51,069	43,409	37,267	31,993	26,378	21,749	17,932	14,785	12,190	**(4%)**	**(6%)**			
Wegovy (US Only)	bull	79,106	73,325	79,487	81,467	83,258	81,768	77,107	63,503	53,117	50,849	45,513	**2%**	**3%**			Consensus Figures
Obesity (GLP-1 once weekly)	**current**	**79,106**	**70,404**	**75,075**	**76,310**	**75,765**	**73,138**	**67,709**	**60,885**	**53,048**	**45,480**	**38,359**			-2%	-10%	TRx share of 9.2% in 2031 and 4.4% in 2035. Price declines in line with historic dynamics
	bear														-5%	-20%	TRx share of 8.4% in 2031 and 3.7% in 2035. Faster TRx volume collapse and larger price decreases from a) weak cash channel demand, b) lower rates of medicare/medicaid take-up, c) a lot of generic entrants
		79,106	46,939	60,401	59,589	51,017	42,294	32,144	24,429	18,566	14,110	10,724	**(9%)**	**(13%)**			
Oral Sema obesity (25mg)	bull	0	9,337	20,174	29,621	35,623	40,311	41,143	44,623	40,927	41,291	40,448	**2%**	**3%**			Consensus Figures
Obesity (GLP-1 once daily)	**Core**	**0**	**7,439**	**15,218**	**19,260**	**23,313**	**26,676**	**29,617**	**32,161**	**33,576**	**34,553**	**35,047**			13%	-10%	TRx share of 10.6% in 2031 and 7.3% in 2035. Price dynamics in line with historic dynamics
	bear														5%	-20%	TRx share of 9.6% in 2031 and 6.1% in 2035. Lower TRx volumes due to slower than expected Orals market and increased generics entrances
		0	7,300	13,925	15,189	15,647	15,152	12,728	10,691	8,981	7,544	6,337	**(9%)**	**(14%)**			
Sales revision risk		0	9,183	19,819	31,197	41,231	46,201	43,778	31,142	19,583	23,556	23,560					
		0	(30 066)	(48 004)	(50 428)	(58 950)	(65 787)	(74 047)	(77 329)	(75 937)	(73 017)	(69 063)					
	Bull	**0%**	**3%**	**7%**	**11%**	**14%**	**16%**	**15%**	**10%**	**6%**	**8%**	**8%**	**8%**				
	Bear	**0%**	**-11%**	**-18%**	**-18%**	**-21%**	**-22%**	**-25%**	**-25%**	**-25%**	**-24%**	**-23%**	**(23%)**				
EBIT revision risk		0	5,051	10,901	17,158	22,677	25,411	24,078	17,128	10,771	12,956	12,958					
		0	(16 536)	(26 402)	(27 735)	(32 423)	(36 183)	(40 726)	(42 531)	(41 765)	(40 160)	(37 985)					
	Bull	**0%**	**5%**	**11%**	**16%**	**21%**	**23%**	**21%**	**15%**	**9%**	**11%**	**11%**		**11%**			
	Bear	**0%**	**-17%**	**-25%**	**-26%**	**-30%**	**-32%**	**-35%**	**-36%**	**-36%**	**-35%**	**-33%**		**(33%)**			
Pipeline risks																	
Cagrisema	bull	0	0	3,536	10,752	17,987	24,268	29,960	35,778	45,783	49,547	53,545	**12%**	**17%**			Consensus Figures
Obesity	**Core**	**0**	**0**	**2,357**	**3,713**	**5,513**	**7,691**	**10,037**	**12,195**	**14,268**	**16,052**	**17,336**					Market share of c.3% in 2035. Priced c.2X of Wegovy but is held as a "niche" weight-loss product
	bear	0	0	0	0	0	0	0	0	0	0	0	**(6%)**	**(8%)**			Doesn't launch
Cagrisema diabetes	bull	0	0	3,733	7,767	11,683	16,880	22,345	29,671	40,198	47,096	53,239	**10%**	**14%**			Consensus Figures
Diabetes	**Core**	**0**	**0**	**0**	**6,916**	**10,893**	**14,705**	**18,529**	**21,678**	**23,413**	**24,232**	**23,990**					c.3% of diabetes medications market in 2035
	bear	0	0	0	0	0	0	0	0	0	0	0	**(8%)**	**(12%)**			Doesn't launch
Sales revision risk		0	1,898	9,868	18,252	25,574	32,387	35,266	44,038	55,652	63,097	70,860					
		0	(139)	(3 650)	(14 700)	(24 073)	(33 921)	(45 455)	(55 344)	(62 277)	(67 293)	(70 036)					
	Bull	**0%**	**1%**	**4%**	**7%**	**9%**	**11%**	**12%**	**14%**	**18%**	**21%**	**23%**	**21%**				
	Bear	**0%**	**0%**	**-1%**	**-5%**	**-8%**	**-12%**	**-15%**	**-18%**	**-20%**	**-22%**	**-23%**	**(14%)**				
EBIT revision risk		0	1,044	5,427	10,039	14,065	17,813	19,396	24,221	30,609	34,703	38,973					
		0	(76)	(2 007)	(8 085)	(13 240)	(18 656)	(25 000)	(30 439)	(34 252)	(37 011)	(38 520)					
	Bull	**0%**	**1%**	**5%**	**9%**	**13%**	**16%**	**17%**	**21%**	**26%**	**30%**	**34%**		**32%**			
	Bear	**0%**	**0%**	**-2%**	**-8%**	**-12%**	**-17%**	**-22%**	**-26%**	**-29%**	**-32%**	**-34%**		**(20%)**			
Total sales revision	**Bull**	**0%**	**4%**	**11%**	**18%**	**23%**	**27%**	**26%**	**25%**	**25%**	**28%**	**31%**	**29%**				
	Bear	**0%**	**(11%)**	**(19%)**	**(24%)**	**(29%)**	**(34%)**	**(40%)**	**(44%)**	**(45%)**	**(46%)**	**-46%**	**(36%)**				
Total EBIT revision	**Bull**	**0%**	**6%**	**16%**	**26%**	**34%**	**38%**	**38%**	**35%**	**35%**	**41%**	**46%**		**43%**			
	Bear	**0%**	**(17%)**	**(27%)**	**(34%)**	**(42%)**	**(49%)**	**(57%)**	**(62%)**	**(65%)**	**(67%)**	**-67%**		**(53%)**			

Source: Source, Bloomberg, Company Reports, Bernstein analysis and estimates

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WHAT ARE THE PIPELINE MILESTONES?

We expect benign news flow with very few drug catalysts during the next few years (Exhibit 28)**.**

- **CagriSema: too little, too late.** The REDEFINE-4 (CagriSema vs Zepbound head-to-head) headline read out has already happened (February 2026; click here to be taken to Novo's press room). This means that CagriSema has now delivered two sets of disappointing phase 3 results (REDEFINE-1 and REDEFINE-4). Hence, we have not included REDEFINE-11 (longer duration trial with more aggressive dose escalation, read out expected in 1H27) in our catalyst calendar. Moreover, we now don't believe that CagriSema can achieve better (or even equivalent) weight loss to Zepbound - regardless of what is reported in REDEFINE-11.

- **Amycretin: too early to provide material upside.** Novo's other obesity pipeline prospect has only just entered phase 3 trials. Amycretin has a dual mechanism of action (GLP-1 + amylin analogue) and we acknowledge the phase 2 data thus far has been solid (Exhibit 44). But we also recognize that Amycretin will be entering a much more crowded obesity market. We estimate a 2029 launch, by which time there could be 9 major obesity medications on the market (see our obesity market summary page here).

- **Ziltivekimab: not differentiated enough.** This drug is a fully human monoclonal antibody which targets interleukin 6 (IL-6) to manage cardiovascular disease, chronic kidney disease, and systemic inflammation. (IL-6 helps cells to communicate with each other of which one (of many outputs) is regulation of glucose (simple sugar) and lipid (fat) metabolism). But we understand that there are five other IL-6 inhibitors in phase 3. In addition, Actemra (Roche; O) has the same mechanism of action but is now facing biosimilar competition so potentially could get used 'off label' to manage cardiometabolic conditions. (Considering that this drug was launched in the US in 2010 it benefits from many years of safe patient use).

EXHIBIT 28: **Novo Nordisk: Mid-term Newsflow (DKK bn)**

Date	Drug	Segment	Phase III details	Comment	Bern peak	Cons peak	Cons Sour	Bern v cor
2026	Amycretin	Obesity	Phase 3 trial initiating in 1H26	Phase 2 data showed -14.5% weight loss (obesity) and -1.8% reductions in HbA1c (diabetes) with a good tolerance profile across the oral and subcutaneous administration. Not enough data (and too early in the pipeline) to make a "blockbuster" call.	22.1	30.2	IR	-27%
2026	Ziltivekimab	ASCVD/CKD	ZEUS; Ziltivekimab vs Placebo on Cardiovascular Outcomes in Participants With Established Atherosclerotic Cardiovascular Disease, Chronic Kidney Disease and Systemic Inflammation. N=6200. Primary endpoint = time to 3-point major adverse cardiovascular event (death, MI, stroke) at 48months	Five competing anti IL-6 MABs in phase 3 (Actemra (Roche but now biosimilar) has same mechanism of action)	4.2	6.9	BBG	-39%
2026	Ziltivekimab	AMI	ARTEMIS; Ziltivekimab vs Placebo on Cardiovascular Outcomes in Patients With Acute Myocardial Infarction. N=10,000. Primary endpoint = Primary endpoint = time to 3-point major adverse cardiovascular event (death, MI, stroke) at 25months	Five competing anti IL-6 MABs in phase 3 (Actemra (Roche but now biosimilar) has same mechanism of action)	4.2	6.9	BBG	-39%
2027	Ziltivekimab	CHF	HERMES; Ziltivekimab vs Placebo on Morbidity and Mortality in Patients With Heart Failure (HFmrEF/HFpEF). N=5600. Primary endooint = mcomposite of CV death, HF hospitalisation or urgent HF visit	Five competing anti IL-6 MABs in phase 3 (Actemra (Roche but now biosimilar) has same mechanism of action)	4.2	6.9	BBG	-39%

Source: Bloomberg, IR Consensus, Company Reports, Bernstein analysis and estimates

Why else aren't we excited about the late stage pipeline?

In the short term Novo Nordisk intends to launch Mim8 for use hemophilia A. These patients are at risk of (potentially fatal) uncontrolled bleeding events because they cannot produce enough Factor VIII (protein which facilitates blood clotting) on their own. Mim8 has the same mechanism of action as Roche's Hemlibra (launched in 2017; 2025 sales $5bn).

Once administered to hemophiliacs, Hemlibra behaves like naturally occurring Factor VIII (the drug is a mimetic). Mim8 has the same bispecific antibody mechanism of action (in effect the drug's 'two keys' fit into two different padlocks inside the body) and was invented by Genmab (UP). Novo in-licensed this drug in 2015.

We acknowledge that from the 'get go' Mim8 will be available in a prefilled syringe (PFS) which can be directly injected under the skin (subcutaneous formulation). Hemlibra is also administered subcutaneously but via vial and syringe so is arguably less convenient. However, we believe that Hemlibra's 9 years of safe use represents a formidable barrier to entry to Mim8. We also

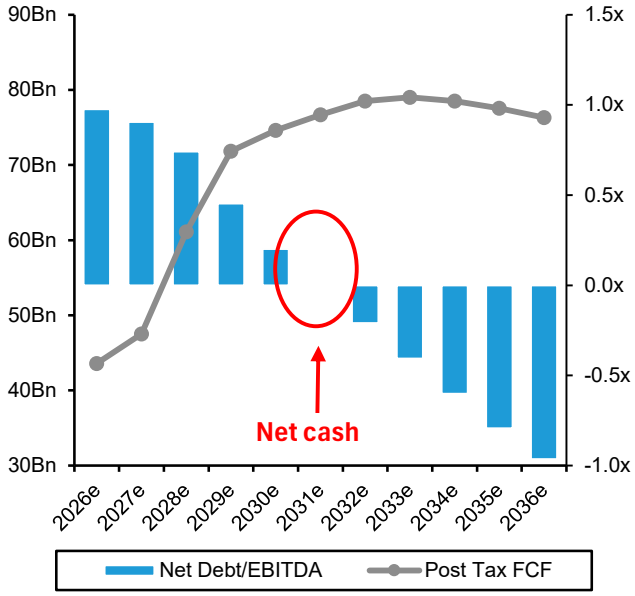
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that note Roche and Chugai (O; Miki Sogi) are about to start a phase 3 trial of Hemlibra vs their own next generation Factor VIII mimetic called NXT007. In phase 1 this drug was 30x more effective than Hemlibra. But in phase 3 Novo Nordisk didn't directly compare Mim8 to Hemlibra.

All things considered, we do not believe that Mim8 can materially alter Novo's mid to long term sales growth trajectory. Our 2035 sales estimate for this drug of DKK 4.4 bn is 60% below consensus (Bloomberg). We believe that other commentators are either underestimating Hemlibra's first mover advantage and/or the potential competitive threat from NXT007.

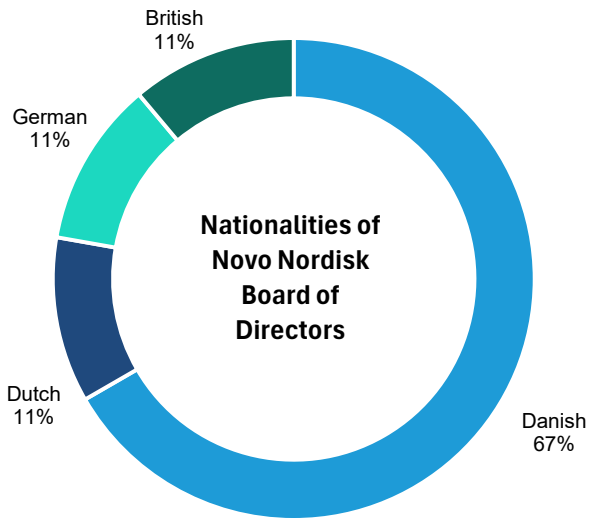
WHAT ABOUT M&A?

EXHIBIT 29: **Novo annual post tax FCF and Net Debt/ EBITDA 2026-36E (DKK bn)**



Source: Company Reports, Bernstein analysis and estimates

EXHIBIT 30: **Nationalities of Novo Board of Directors**



Source: Company Website, Bernstein Analysis

Between 2026 and 2031 we forecast that Novo Nordisk will generate $60bn in post-tax free cash flow. We also project 2026 and 2027 EBITDA of $17bn and $19bn respectively and that the company will be 'net cash' in 2031 (Exhibit 29). Moreover, at 31/12/25 the company's net debt/EBITDA was 0.7x which is far below peak financial leverage in EU Pharma (at the bookend Shire geared its balance sheet to 6x in 2016 to facilitate the acquisition of Baxalta). Hence, in our view, theoretically Novo Nordisk has the financial resources to engage in material 'bridging' M&A. (Recent radical examples include AstraZeneca's (O) $3bn rights issue in 2Q19 to fund a 50/50 partnership with Daiichi Sankyo (O; Miki Sogi)) on Enhertu (HER2 breast cancer) and Genmab's (UP) $9bn acquisition of cancer specialist Merus last year. Both of these transactions involved late stage assets which were/are close to market).

We acknowledge that in 2025 Novo Nordisk spent $11bn to acquire three former Catalent manufacturing plants from the Novo Foundation. But in our view this was a 'one off' transaction to alleviate Novo Nordisk's GLP1 manufacturing constraints. But history suggests that, for pipeline bolstering purposes, Novo tends to adopt a conservative approach to M&A which strongly favors drugs in early to mid-stage clinical development. We hope that we're wrong, but considering this track record and the composition of Novo's Board of Directors (Exhibit 30), we'd be surprised if this the company's strategic approach changes radically going forwards.

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EXHIBIT 31: **Novo Nordisk: Significant M&A**

Announced Date	Target		Size ($mm)
Oct-09-2025	Akero Therapeutics, Inc.	Acquired Efruxifermin (MASH) for the Rare Disease portfolio	4,862
Aug-10-2023	Inversago Pharma Inc.	Biopharma company focusing on CB1 receptor blockers (Obesity and Diabetes portfolio)	1,075
Sep-01-2022	Forma Therapeutics Holdings, Inc.	Biopharma company focusing on therapeutics for rare hematologic diseases (Rare Disease portfolio)	1,060
Nov-18-2021	Dicerna Pharmaceuticals, Inc.	Biopharma company developing RNAi pharmaceuticals (Rare Disease and Diabetes)	3,211
Nov-06-2020	Emisphere Technologies, Inc.	Purchased the rights to use oral formulations technologies (used in Wegovy Pill)	1,338
Jun-11-2020	Corvidia Therapeutics, Inc.	Biotech company developing cardiovascular therapies	2,100

Source: CapitalIQ, Bernstein Analysis

WHAT'S THE STOCK WORTH?

Consistent with our approach to valuing the other stocks in Bernstein's EU pharma coverage universe, we calculate our price target (PT) via a straight average of 2026-2028e EV/EBITA multiples and a DCF. In the latter, our bottom-up forecasts extend to 2036 after which we apply fade rates until 2045. We thus only apply a perpetuity growth rate (of 0% for an exit multiple of 12.5x) in 2045. This means that the exit value accounts for just 23% of the total EV.

EXHIBIT 32: **Bernstein Target Price (DKK 180) calculation**

Valuation Summary	New	Assumptions
DCF	191	WACC = 8%; Perpetuity growth = 0%
EV/EBITA (2026-28e)	160	Exit multiple of 12.5x and 36% discount to EU peers due to: 1. Below average sales growth 2. Portfolio concentration risk 3. Mixed track record in M&A
Price Target (Simple Average)	**175**	
Novo implied core PE26	10.0x	
EU Pharma core PE26	16.6x	
Implied Relative Valuation vs EU Peers	-40%	

Source: Company Reports, Bernstein analysis and estimates

Could Novo become a single-digit PE stock? Not Quite in our view

In our EV/EBITA valuation, we assume that the stock should trade at a 36% discount to EU peers to account for the portfolio concentration risk, meek M&A heritage, sustained below average sales growth and poor operational execution.

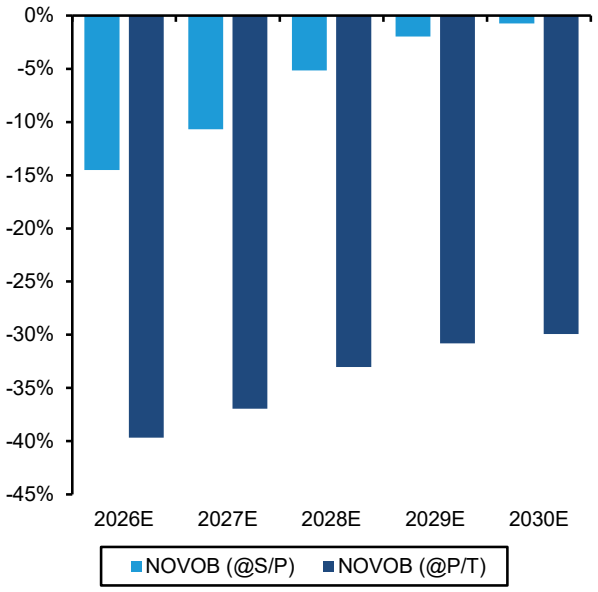
Compared to EU peers: Our PT implies an adjusted ('core') PE26 of 10x for a -40% discount. Meanwhile, in 2030 the implied discount is -30% (Exhibit 33). Our PT also implies that Novo trades on a 15-year low PE multiple (Exhibit 34). We believe this is appropriate considering the significant challenge which the company faces in returning to sustainable above sector growth following the semaglutide US compound patent expiry in 2032 and the company's weakened competitive positioning in the obesity and diabetes markets.

Relative to Lilly: our Novo PT implies a -67% discount to Lilly in 2026 and a -53% discount in 2030 (Exhibit 35). This also implies a 15-year low PE relative to Lilly (Exhibit 36). We believe this is warranted considering Lilly's far better operational and R&D execution in metabolic diseases and its superior therapeutic diversification. (Lilly launched its first GLP1 in diabetes in the US four years after Novo did).

At our Price Target, Novo would still trade at a premium to Sanofi (O) on PE26 even though the latter's business model is more durable as it includes a large vaccines franchise (which, in our view, enjoys the deepest moat in all of biopharma due to the highest capex, toughest R&D, and no generic risk; Exhibit 37). We thus do not think that our Novo PT is extreme.

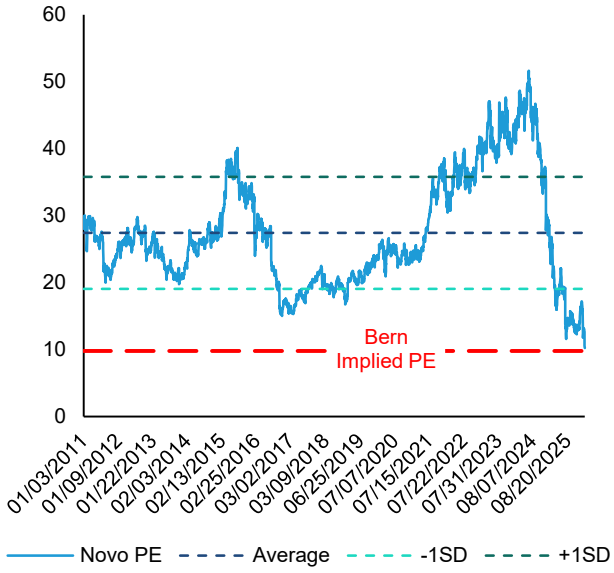
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EXHIBIT 33: **Novo Nordisk vs EU Peers: Adjusted Core PE Discounts**



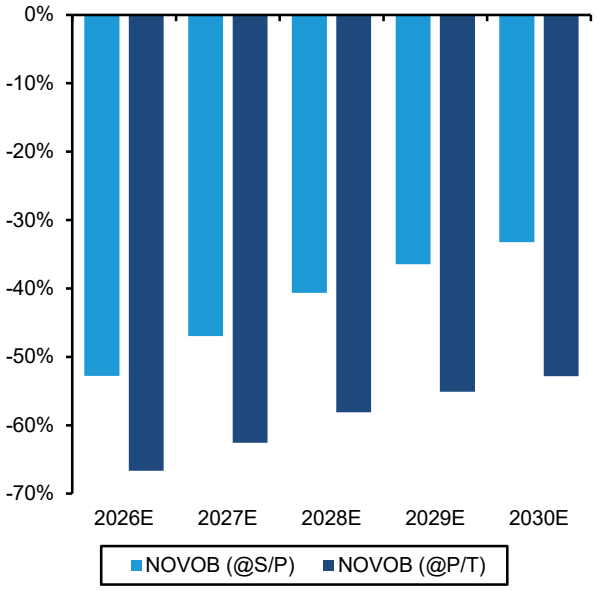
S/P = Share Price; P/T = Price Target
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

EXHIBIT 34: **Novo Nordisk: PE performance over the past 15 years**



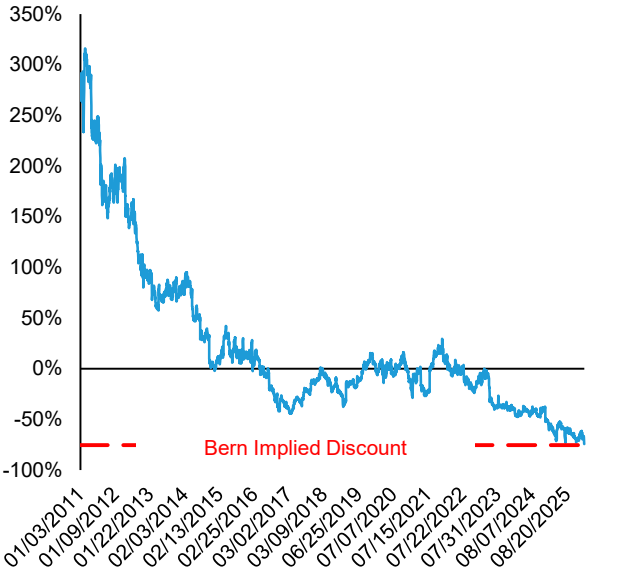
Source: Bloomberg, Bernstein Analysis

EXHIBIT 35: **Novo Nordisk vs Lilly: Adjusted Core PE Discounts**



S/P = Share Price; P/T = Price Target
Source: Bloomberg, Company Reports, Bernstein analysis and estimates

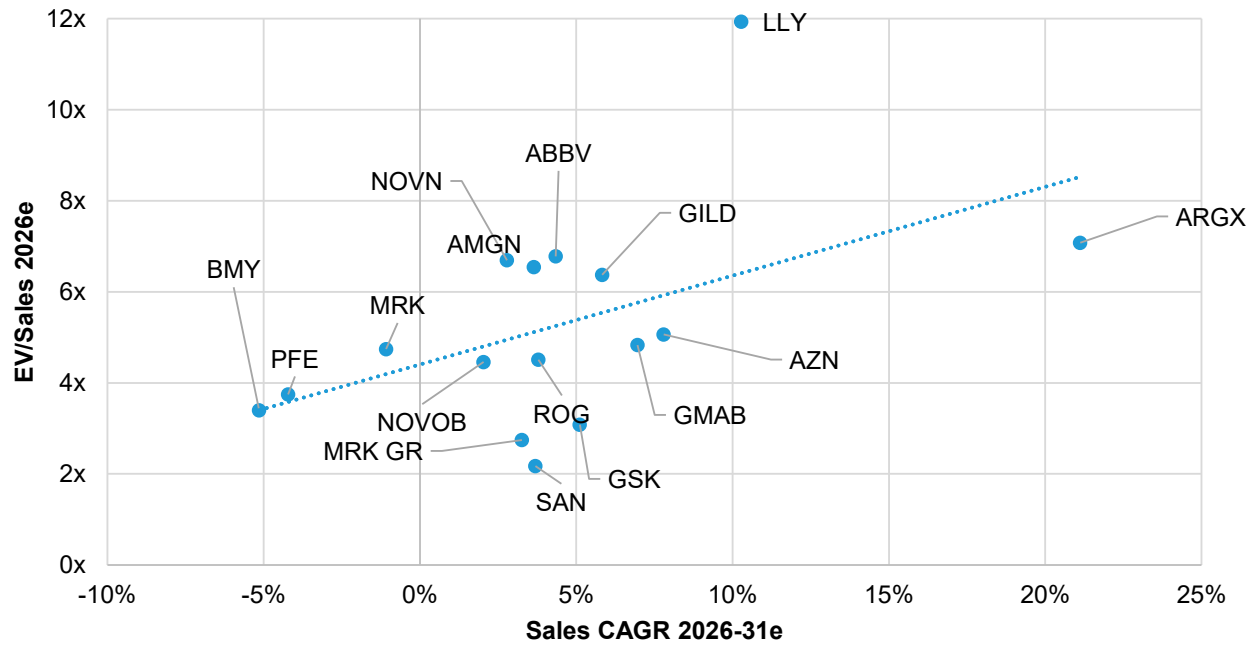
EXHIBIT 36: **Novo Nordisk: Relative PE performance vs Lilly**



Source: Bloomberg, Bernstein analysis

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EXHIBIT 37: **US/EU pharma sales regression analysis**



Source: Bloomberg, Bernstein analysis and estimates

EXHIBIT 38: **EV/EBITDA Multiples Calculation**

Multiple Calculation (EV/EBITA)	2026e	2027e	2028e	Discount (to EU peers)
EBITA	100,129	106,067	108,262	
Sector Multiples	12.9x	12.0x	11.1x	-36%
Applied Multiples	8.2x	7.7x	7.1x	
EV	**826,062**	**812,302**	**771,289**	
Net Debt (Inc. Provisions)	114,410	114,411	98,618	
Equity Value	711,652	697,892	672,671	
Diluted Shares (m)	4,412	4,351	4,291	
Equity per Share	161	160	157	

Source: Company Reports, Bernstein Estimates and Analysis

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EXHIBIT 39: **Novo Nordisk DCF Summary**

Discounted Cash Flow (DKKm)	2020	2021	2022	2023	2024	2025	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E	2037E	2038E	2039E	2040E	2045E
Sales	126,946	140,800	176,954	232,261	290,403	309,064	270,525	271,032	276,906	285,896	292,924	299,191	303,965	305,388	305,514	305,244	306,416	306,416	306,416	306,416	306,416	306,416
% growth		11%	26%	31%	25%	6%	-12%	0%	2%	3%	2%	2%	2%	0%	0%	0%	0%	0%	0%	0%	0%	0%
EBITA	**54,226**	**60,146**	**70,661**	**106,508**	**129,703**	**133,213**	**100,129**	**106,067**	**108,262**	**112,137**	**115,262**	**117,919**	**119,907**	**119,243**	**118,072**	**116,666**	**115,797**	**115,797**	**115,797**	**115,797**	**115,797**	**115,797**
% Margin	42.7%	42.7%	39.9%	45.9%	44.7%	43.1%	37.0%	39.1%	39.1%	39.2%	39.3%	39.4%	39.4%	39.0%	38.6%	38.2%	37.8%	37.8%	37.8%	37.8%	37.8%	37.8%
Tax Rate	-21%	-19%	-20%	-20%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%	-21%
Tax	-10,992	-11,323	-13,537	-20,991	-26,203	-26,893	-20,536	-21,782	-22,232	-23,029	-23,673	-24,219	-24,628	-24,486	-24,240	-23,945	-23,761	-23,761	-23,761	-23,761	-23,761	-23,761
EBITA Post Income Tax	**43,234**	**48,823**	**57,124**	**85,517**	**103,500**	**106,320**	**79,594**	**84,285**	**86,030**	**89,107**	**91,589**	**93,699**	**95,279**	**94,757**	**93,832**	**92,720**	**92,036**	**92,036**	**92,036**	**92,036**	**92,036**	**92,036**
Working Capital	40,553	51,394	59,361	70,975	95,658	101,805	89,110	89,277	91,212	94,173	96,489	98,553	100,125	100,594	100,636	100,547	100,932	100,932	100,932	100,932	100,932	100,932
Margin (as % of sales)	32%	37%	34%	31%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%	33%
Change in WC	-4,353	-8,656	-5,336	-12,245	2,589	3,737	12,695	-167	-1,935	-2,961	-2,315	-2,064	-1,573	-469	-42	89	-386	0	0	0	0	0
Net Capex	-6,297	-7,091	-14,654	-31,431	-52,185	-65,002	-48,694	-36,589	-22,983	-14,295	-14,646	-14,960	-15,198	-15,269	-15,276	-15,262	-15,321	-15,321	-15,321	-15,321	-15,321	-15,321
Capex Margin (as % of sales)	-5%	-5%	-8%	-14%	-18%	-21%	-18%	-14%	-8%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%	-5%
Operating FCF	**32,584**	**33,076**	**37,134**	**41,841**	**53,904**	**45,055**	**43,594**	**47,529**	**61,112**	**71,852**	**74,628**	**76,676**	**78,508**	**79,019**	**78,514**	**77,547**	**76,329**	**76,715**	**76,715**	**76,715**	**76,715**	**76,715**
Discount Factor	159%	147%	136%	126%	117%	108%	100%	93%	86%	79%	74%	68%	63%	58%	54%	50%	46%	43%	40%	37%	34%	23%
NPV	**51,707**	**48,599**	**50,520**	**52,708**	**62,874**	**48,660**	**43,594**	**44,008**	**52,393**	**57,038**	**54,854**	**52,184**	**49,473**	**46,107**	**42,419**	**38,793**	**35,355**	**32,902**	**30,465**	**28,208**	**26,118**	**17,776**

Sum of NPV (2023-2042)	738,195	
Perpetuity Value	222,198	23%
Total EV	**960,392**	

DCF Implied Share Price	
Total EV	960,392
Net Debt (Inc. Provisions)	110,100
Equity Value	850,292
Diluted Shares (m)	4,449
Implied Share Price (DKK)	**191**

Assumptions 2036-45	
WACC	8%
WCR %	33%
Capex %	-5%
Tax %	-21%
Sales Growth	0%
EBITA %	38%
Perp. Growth	0%
Exit Multiple	12.5x

Sensitivity Analysis

		Perpetuity Growth				
	191	-5.0%	-3.0%	0.0%	4.0%	5.0%
	7.0%	191	200	222	319	404
	7.5%	180	188	205	277	331
WACC	8.0%	170	176	191	245	281
	8.5%	161	166	178	220	245
	9.0%	153	157	167	200	218

Source: Company Reports, Bernstein analysis and estimates

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EXHIBIT 40: **Bernstein summary of financial forecasts (DKK millions except share data)**

	2025	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E
Income Statement												
Total Revenues	309,064	270,525	271,032	276,906	285,896	292,924	299,191	303,965	305,388	305,514	305,244	306,416
COGS	-58,788	-54,105	-54,545	-56,074	-58,251	-60,050	-61,708	-63,073	-64,376	-65,411	-66,360	-67,626
Gross profit	**250,276**	**216,420**	**216,487**	**220,833**	**227,644**	**232,875**	**237,483**	**240,892**	**241,012**	**240,104**	**238,884**	**238,790**
SG&A	-64,310	-56,269	-56,713	-58,289	-60,538	-62,393	-64,102	-65,504	-66,819	-67,855	-68,802	-70,077
R&D	-52,039	-45,448	-45,533	-46,520	-48,030	-49,211	-50,264	-51,066	-51,305	-51,326	-51,281	-51,478
Operating Profit	127,658	101,285	107,193	108,824	111,642	113,655	115,338	116,419	114,948	112,979	110,865	109,268
Pre Tax Profit	130,540	97,789	103,723	105,867	109,664	112,728	115,331	117,278	116,602	115,429	114,025	113,146
Diluted EPS	23.02	17.51	18.82	19.48	20.46	21.32	22.13	22.83	23.03	23.14	23.21	23.39
Net Profit	**102,434**	**77,254**	**81,941**	**83,635**	**86,634**	**89,055**	**91,111**	**92,649**	**92,115**	**91,189**	**90,080**	**89,385**
Balance Sheet												
Cash and equivalents	26,464	22,154	22,153	37,946	69,228	102,631	131,566	158,807	183,844	208,848	230,378	251,076
Accounts receivable	70,856	63,680	63,800	65,182	67,299	68,953	70,428	71,552	71,887	71,917	71,853	72,129
Inventories, net	49,623	44,150	44,509	45,756	47,533	49,000	50,354	51,467	52,531	53,375	54,150	55,183
Other current assets	25,510	25,135	25,242	25,612	26,139	26,575	26,977	27,307	27,623	27,874	28,104	28,410
Total Current Assets	**172,453**	**155,119**	**155,703**	**174,497**	**210,199**	**247,160**	**279,325**	**309,133**	**335,885**	**362,013**	**384,485**	**406,798**
PP&E	208,378	246,251	269,289	275,658	267,081	258,293	255,302	255,302	258,355	261,411	267,515	273,644
Intangible assets & Goodwill	110,208	108,169	106,870	105,521	104,094	102,606	101,064	99,481	97,885	96,288	94,694	93,089
Other assets	32,018	32,018	32,018	32,018	32,018	32,018	32,018	32,018	32,018	32,018	32,018	32,018
Total Non-Current Assets	**370,449**	**386,438**	**408,177**	**413,197**	**403,193**	**392,917**	**388,384**	**386,800**	**388,258**	**389,717**	**394,227**	**398,751**
Total Assets	**542,902**	**541,557**	**563,881**	**587,693**	**613,392**	**640,077**	**667,708**	**695,934**	**724,143**	**751,730**	**778,712**	**805,549**
Short term debt	12,017	12,017	12,017	12,017	12,017	12,017	12,017	12,017	12,017	12,017	12,017	12,017
Accounts payable	19,758	25,973	26,185	26,918	27,964	28,827	29,623	30,278	30,904	31,401	31,856	32,464
Other current liabilities	39,721	39,721	39,721	39,721	39,721	39,721	39,721	39,721	39,721	39,721	39,721	39,721
Total Current Liabilities	**215,661**	**221,876**	**222,088**	**222,821**	**223,867**	**224,730**	**225,526**	**226,181**	**226,807**	**227,304**	**227,759**	**228,367**
Long term debt	118,941	118,941	118,941	118,941	118,941	118,941	118,941	118,941	118,941	118,941	118,941	118,941
Other long term liabilities	13,202	13,202	13,202	13,202	13,202	13,202	13,202	13,202	13,202	13,202	13,202	13,202
Total Non-Current Liabilities	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**	**133,194**
Total Liabilities	**348,855**	**355,070**	**355,282**	**356,015**	**357,061**	**357,924**	**358,720**	**359,375**	**360,001**	**360,498**	**360,953**	**361,561**
Total Equity	**194,047**	**186,487**	**208,599**	**231,678**	**256,331**	**282,153**	**308,988**	**336,558**	**364,142**	**391,232**	**417,759**	**443,988**
Total Equity + Liabilities	**542,902**	**541,557**	**563,881**	**587,693**	**613,392**	**640,077**	**667,708**	**695,934**	**724,143**	**751,730**	**778,712**	**805,549**
Cash Flow												
Net profit	102,434	77,254	81,941	83,635	86,634	89,055	91,111	92,649	92,115	91,189	90,080	89,385
Income Tax	-28,106	-20,536	-21,782	-22,232	-23,029	-23,673	-24,219	-24,628	-24,486	-24,240	-23,945	-23,761
Depreciation + amortization	21,982	13,161	15,896	19,010	25,345	25,968	20,539	17,828	14,857	14,863	11,798	11,843
Other Operating Cash Flows	-33,420	-12,984	-27,092	-29,310	-31,134	-31,131	-31,427	-31,344	-30,098	-29,425	-28,999	-29,290
Net Operating Cash Flows	**119,102**	**97,966**	**92,527**	**95,566**	**103,875**	**107,565**	**104,444**	**103,761**	**101,361**	**100,868**	**96,824**	**95,699**
Net Cash used in Investing	**-79,158**	**-48,889**	**-36,784**	**-23,178**	**-14,490**	**-14,841**	**-15,155**	**-15,393**	**-15,464**	**-15,471**	**-15,457**	**-15,516**
Dividends	-51,763	-38,842	-41,198	-42,050	-43,558	-44,775	-45,809	-46,582	-46,314	-45,848	-45,291	-44,941
Pay out (% core Net Income)	-51%	-50%	-50%	-50%	-50%	-50%	-50%	-50%	-50%	-50%	-50%	-50%
Net Cash used in Financing	**-28,408**	**-53,842**	**-56,198**	**-57,050**	**-58,558**	**-59,775**	**-60,809**	**-61,582**	**-61,314**	**-60,848**	**-60,291**	**-59,941**
Cash at the beginnning of the year	15,655	26,464	22,154	22,153	37,946	69,228	102,631	131,566	158,807	183,844	208,848	230,378
Cash at the end of the year	**26,464**	**22,154**	**22,153**	**37,946**	**69,228**	**102,631**	**131,566**	**158,807**	**183,844**	**208,848**	**230,378**	**251,076**

Source: Source: Company Reports, Bernstein analysis and estimates

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EXHIBIT 41: **Bernstein summary of product sales estimates (DKK millions)**

	2025	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E
Ozempic	127,089	97,495	83,108	73,044	63,803	55,413	47,971	41,153	34,792	29,424	24,908	21,099
Victoza	3,020	2,157	1,720	1,371	1,094	873	830	790	752	716	682	649
Rybelsus	22,093	19,596	18,723	18,026	17,240	16,387	15,479	14,428	13,259	12,003	10,693	9,360
Awiqli	410	1,128	2,315	3,339	4,204	5,141	5,609	6,125	6,692	7,004	7,200	7,335
Tresiba	12,049	12,740	12,977	13,218	13,371	13,416	13,405	13,400	13,383	13,354	13,312	13,256
Xultophy	4,619	4,850	5,091	5,342	5,604	5,878	6,162	6,460	6,648	6,839	7,035	7,234
Levemir	1,677	1,340	1,205	1,144	1,086	1,032	980	951	922	895	868	842
Ryzodeg	5,382	5,813	5,987	6,152	6,305	6,447	6,576	6,691	6,792	6,877	6,945	6,997
NovoMix	4,933	4,440	4,193	3,982	3,801	3,647	3,518	3,411	3,325	3,258	3,209	3,177
Fiasp	2,818	2,799	2,790	2,791	2,803	2,825	2,856	2,899	2,952	3,016	3,092	3,180
NovoRapid	15,765	12,612	12,345	12,227	12,185	12,202	12,036	11,879	11,540	11,218	10,910	10,443
Human	5,484	4,500	4,363	4,357	4,352	4,348	4,345	4,343	4,342	4,341	4,342	4,343
Other Diabetes	1,770	1,668	1,603	1,553	1,505	1,458	1,412	1,368	1,326	1,285	1,245	1,206
Wegovy	79,106	70,404	75,075	76,310	75,765	73,138	67,709	60,885	53,048	45,480	38,359	32,049
Saxenda	3,241	1,985	1,221	755	470	295	186	119	77	50	33	22
Oral Semaglutide	0	7,439	15,218	19,260	23,313	26,676	29,617	32,161	33,576	34,553	35,047	34,686
Rare Blood Disorders	11,955	12,060	12,047	12,053	12,077	12,118	12,177	12,252	12,344	12,453	12,578	12,720
Haemophillia A	2,414	2,415	2,443	2,477	2,515	2,558	2,606	2,658	2,715	2,775	2,839	2,907
Haemophillia B	1,412	1,483	1,539	1,597	1,657	1,720	1,786	1,854	1,925	1,999	2,076	2,156
NovoSeven	7,326	6,434	6,258	6,089	5,927	5,772	5,623	5,480	5,342	5,211	5,084	4,963
Rare Endocrine Disorders	5,959	5,636	5,917	6,213	6,400	6,592	6,789	6,857	6,926	6,995	7,065	7,136
Other Rare Disease	1,694	1,671	1,727	1,785	1,844	1,906	1,970	2,037	2,106	2,177	2,250	2,327
Approved products	**309,064**	**270,330**	**267,625**	**262,923**	**257,221**	**249,790**	**239,630**	**228,208**	**214,800**	**201,936**	**189,771**	**178,062**
efruxifermin (MASH; from Akero)	0	0	0	0	758	2,274	3,790	4,927	5,685	6,064	6,443	6,822
Amycretin (Obesity)	0	0	0	0	3,742	6,549	10,151	14,719	19,870	24,838	29,806	35,767
Amycretin (Diabetes)	0	0	0	0	1,871	3,274	5,075	7,359	9,935	12,419	14,903	17,883
CagriSema (Obesity)	0	0	2,357	3,713	5,513	7,691	10,037	12,195	14,268	16,052	17,336	18,723
CagriSema (Diabetes)	0	0	0	6,916	10,893	14,705	18,529	21,678	23,413	24,232	23,990	23,750
Mim8 (once a weeky/monthly haemophillia)	0	2	150	912	1,525	2,082	2,848	3,175	3,527	3,898	4,348	4,576
Ziltivekimab	0	0	0	386	1,157	1,929	3,086	4,244	5,015	5,787	6,944	7,716
Icosema	0	0	514	1,286	2,058	3,086	4,115	5,144	6,173	7,202	8,230	9,259
Alhemo	0	193	386	772	1,157	1,543	1,929	2,315	2,701	3,086	3,472	3,858
Pipeline	**0**	**195**	**3,407**	**13,984**	**28,675**	**43,134**	**59,560**	**75,757**	**90,588**	**103,578**	**115,472**	**128,354**
Group Sales	**309,064**	**270,525**	**271,032**	**276,906**	**285,896**	**292,924**	**299,191**	**303,965**	**305,388**	**305,514**	**305,244**	**306,416**

Source: Company Reports, Bernstein analysis and estimates

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US OBESITY MARKET: SOLID VOLUME GROWTH OFFSETTING STICKY DEFLATION

The obesity market is already worth $25bn in 2025. We forecast a 12% value CAGR over the next decade, where the US obesity market will be worth $85bn (Exhibit 42). So far, this story has been dominated by the share battle between Novo and Lilly (O; covered by Courtney Breen). But even with new entrants arriving at the end of the decade, we believe the market will remain a duopoly, with Lilly the dominant player (Exhibit 43).

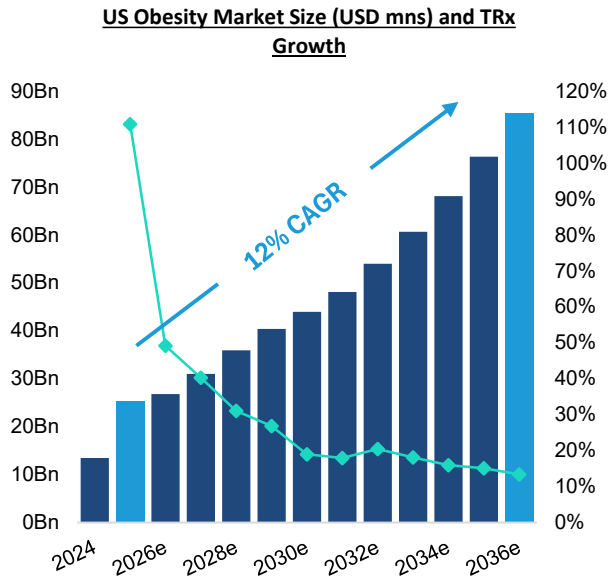
Our key takeaway from the past five years is simple: in injectables, weight loss wins. Tolerance profile and whether the drug comes with restrictions (e.g. fasting before taking it) are of secondary importance - headline weight loss is the primary needle mover. We think this is largely for two reasons.

1. Obesity drugs are prescribed in primary care settings, where practitioners are relatively time poor. Practitioners will prescribe their "favored" GLP-1 and only move patients onto a different product should their side effects become intolerable.

2. The direct-to-consumer channel in the US has increased the importance of headline weight loss. Patients seeking to lose weight (reasonably) want the product which maximizes the speed and magnitude of their weight loss. The tolerance profile is a "second order" consideration, whereby a degree of discomfort is tolerated for additional weight loss. (Proven 'outcomes' benefits such as stroke reduction are even further down the list).

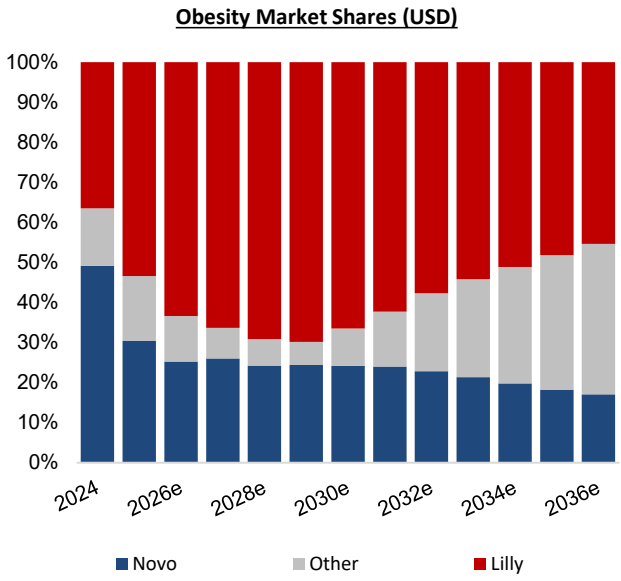
We think the market dynamics are tough to disrupt. We don't foresee a share reversal back to 2023 or 2024 levels (where Novo had 60% of the US obesity market). Nor do we think a major disruptor is waiting in the wings. Instead, we think Lilly will remain the dominant player, Novo the (distant) number two, and a tail of players with smaller shares. In our view, both Lilly and Novo should benefit from existing positions on the market, which should be protected by pipeline launches.

EXHIBIT 42: **We believe the obesity market could reach $85bn in 2036e**



Source: IQVIA, Company Reports, Bernstein analysis and estimates

EXHIBIT 43: **We think the market will essentially remain a duopoly until 2030e**



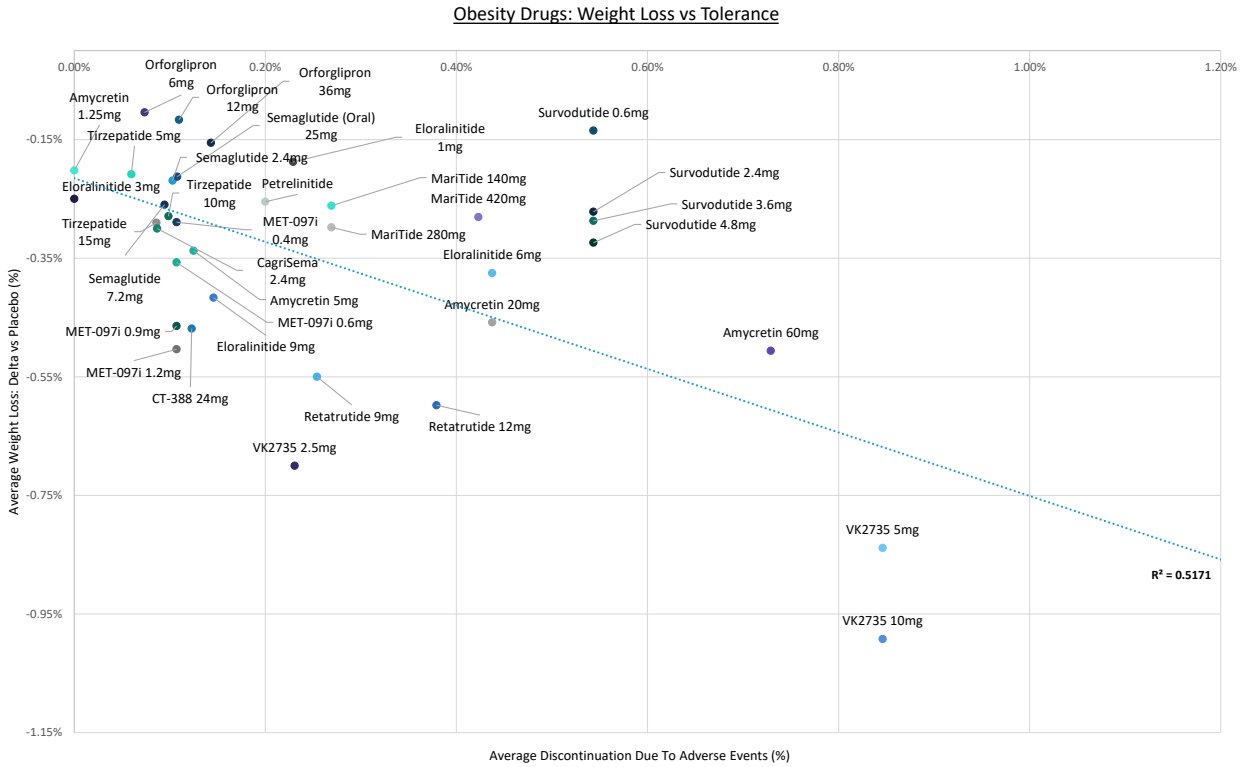
Source: IQVIA, Company Reports, Bernstein analysis and estimates

In our obesity model, we have used Exhibit 44 to guide our thinking about the quality of obesity drugs. We have tried to normalize weight loss and discontinuation rates (due to adverse events) between studies (which run for varying levels of time) by taking averages of both measures across trial length. We recognize this is an imperfect tool, and readers should be wary of indirect comparisons between drugs due a variety of trial design differences. But we still believe this provides an intuitive understanding of how weight loss drugs (on the market and in the pipeline) compare to each other. We also note that numerous trials are run on each drug (with differing results for weight-loss and tolerance). We have provided trial names (where available)

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and NCT numbers in the footnote of Exhibit 44.

EXHIBIT 44: **Early data indicates drugs achieving more weight loss (average weekly delta vs placebo) with comparable tolerance profiles (average trial discontinuation rate) to Wegovy and Zepbound are coming to market**



Y axis = average weight loss per week; X axis = average trial discontinuation due to adverse events

Trial names where given: Semaglutide (STEP-1 (injectable; 2.4mg), STEP-UP (injectable; 7.2mg), and OASIS-4 (oral)), Tirzepatide (SURMOUNT-1), Orforglipron (ATTAIN-1), CagriSema (REDEFINE-1), Retatrutide (TRIUMPH-4), MET-097i (VESPER-1), VK2375 (VENTURE), Petrilintide (ZUPREME-1)

NCT numbers where trial names are not disclosed: Amycretin (NCT06064006), CT-388 (NCT06628362), MariTide (NCT05669599), Survodutide (NCT04667377), Eloralinitide (NCT06230523)
Source: Company Reports, New England Medical Journal, ClinicalTrials.Gov, Bernstein analysis

PRICING: FY26'S DOMINANT THEME

The US price environment has deteriorated faster than expected. Between compounders undercutting, growth in the direct-to-consumer cash channel, and agreements with the US government, we believe that prices will fall far quicker than is usual in pharmaceuticals this year. We expect returns to "normal" levels of deflation (10-20% each year) to return from 2027 onwards.

Recap: TrumpRx and The White House Deal

In November 2025, Novo and Lilly agreed to the White House's drug pricing deal terms for GLP-1s. Here, we outline briefly what the deal contained and how it has informed our modeling assumptions. For a more in-depth look at what the deal means, you can read our US colleague Courtney Breen's note from just after the deal was announced here.

Novo and Lilly benefited from:

1. **Medicare and Medicaid coverage for obesity drugs (for both on-market and pipeline assets):** Medicare (>65s) and Medicaid (low income) coverage comes with a $50 co-pay for patients. Expansion through the Medicare channel could add 18m new patients (with coverage limitations depending on patient BMI).

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2. **3-year tariff exemption:** Novo and Lilly will not be subject to tariff costs for three years.

3. **One Comissioner's National Priority Voucher (CNPV) each:** This offers each company the opportunity to reduce FDA review time from 10-12 months down to 1-2 months.

In exchange for:

1. **Deep discounts for primary care (obesity and diabetes) drugs:**

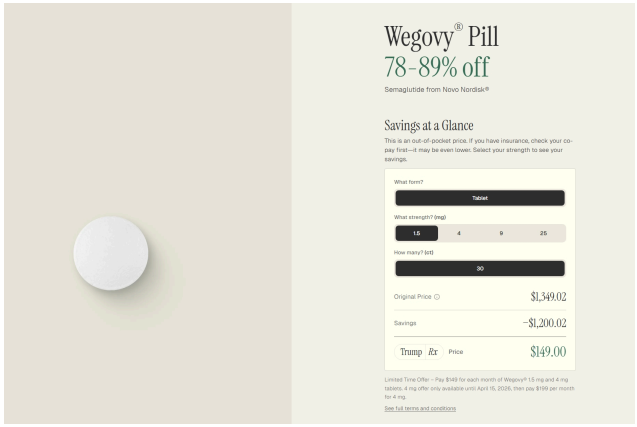
   - **Through TrumpRx** (a direct-to-consumer channel for prescription drugs which launched in January 2026), Ozempic and Wegovy are $350 (highest doses) and the Wegovy Pill is $150 for the initial dose ($299 for the highest dose). We understand the deal also stipulated that injectables will trend towards a blended net price of $245 by the end of 2027.

   - **Through Medicare/Medicaid** Ozempic, Wegovy, Mounjaro, and Zepbound will be $245 with a $50 co-pay - available from April 2026. For Medicaid, this requires an opt-in from states to participate (California has not opted in).

2. **MFN pricing guarantees on all future new medicines:** Lilly and Novo guaranteed the US market will receive MFN pricing on future medicine, increasing pricing on existing products in ex. US markets, and using MFN pricing for all state Medicaid programs.

3. **Large manufacturing investments into the US:** Novo announced $10bn investment in manufacturing (primarily through Wegovy Pill production). Lilly had previously announced $30bn of investment earlier in 2025.

EXHIBIT 45: **Wegovy Pill on TrumpRx**



Source: TrumpRx

EXHIBIT 46: **Wegovy Injectable on TrumpRx**



Source: TrumpRx

How have we modeled pricing?

We have modeled pricing dynamics channel by channel for Wegovy, Zepbound, Oral Sema, and orforglipron in order to generate a blended net price for each.

- **Wegovy and Zepbound:** We have applied a -30% price discount across Medicaid, Medicare, and Commercial (i.e. private insurance) to our 4Q25 prices in to get our 1Q26 prices. We have then assumed a price of $245 in the Medicare and Medicaid channels to take effects from 2Q26 until 4Q26. After applying a -10% discount on our 4Q25 estimates of commercial channel prices, we keep commercial channel prices flat across the rest of 2026. In the cash pay channel, we have calculated the average price (weighted by dosage volumes) using TrumpRx and the cash channel sites of Novo (NovoCare) and Lilly (LillyDirect), we have assumed no "in-year" price changes here. Going forward, we assume -10% yoy price erosion across each channel. Due to the channel mix (i.e. more prescriptions going through higher paying channels (Commercial), blended prices don't immediately normalize to -10% (prices normalize around -10% in 2030).

- **Oral Sema:** We have created a dose-weighted (using the prescription data available) average price and assumed the only active channel in 1Q26 is the cash channel - the price seems artificially low because most scripts are coming through for the lowest (i.e. cheapest) dose. We assume these same dynamics as other channels come into play across the year - with the

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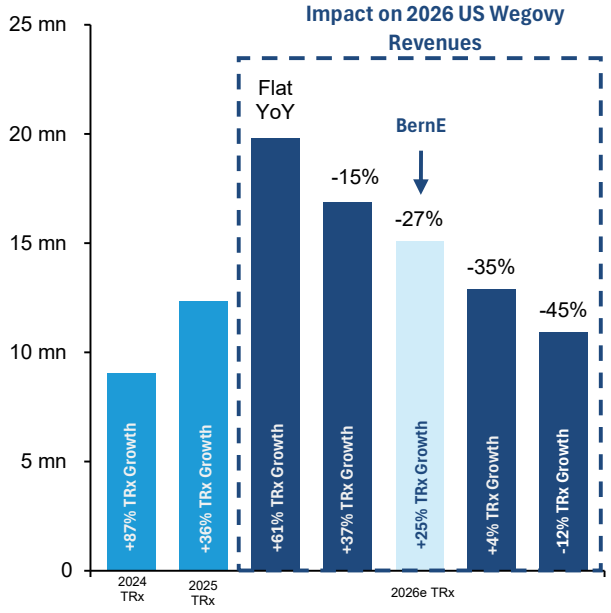
commercial channel having a slight premium. We assume prices normalize towards a blended net price of $245 by 4Q26 and then apply -10% yoy.

- **Orforglipron:** We have taken the price assumptions from our US colleague Courtney Breen for 2026. We have then applied the same -10% price erosion assumptions yoy thereon.

How have we modeled prescription volumes?

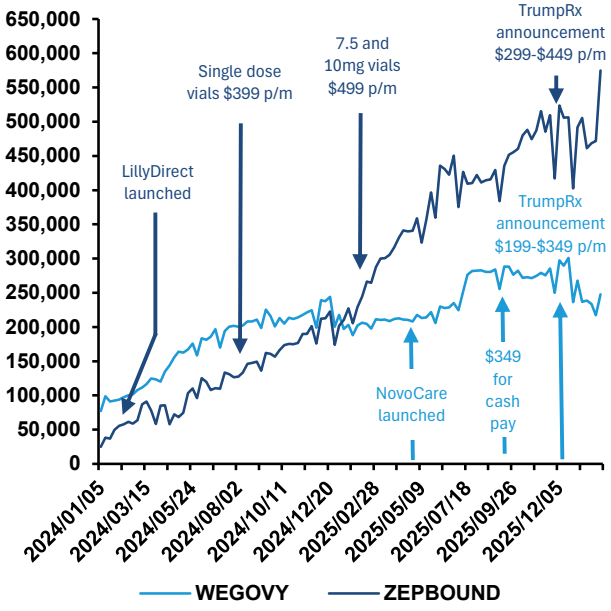
We forecast +50% TRx (total prescription; 'scripts') volume growth for the US obesity market in '26 (Exhibit 42)**.** Despite the price cuts announced for this following year, we forecast a modest TRx volume uptick. Following +111% growth in total TRx volumes in FY25 (both branded (Wegovy and Zepbound) and compounding pharmacy TRx volumes grew at +111%), we forecast +63% growth in branded injectables (Wegovy and Zepbound), -25% growth in compounding TRx volumes, and strong oral launches to drive overall US obesity market TRx volumes to +50% yoy in 2026.

EXHIBIT 47: **We estimate that Wegovy prescriptions would need to grow c.60% YoY for revenues to be flat in FY26**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

EXHIBIT 48: **Price reductions for consumers have failed to translate into material TRx growth for Novo**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

We expect the real "uptick" in total TRx volumes to materialize in FY27 (+40% total TRx volume growth in the US obesity market). This represents a -9% slowing in the market growth rate. Relative to previous years in obesity, and other pharma markets, this represents a material increase in script volumes. We think increased penetration of the US cash channel and increased access to Medicare and Medicaid patients to materialize in 2027 scripts and are key for driving penetration of the US obesity market.

We forecast +25% growth in Wegovy scripts in FY26 but, given our pricing assumptions, this translates into -27% yoy US sales growth (Exhibit 47)**:**

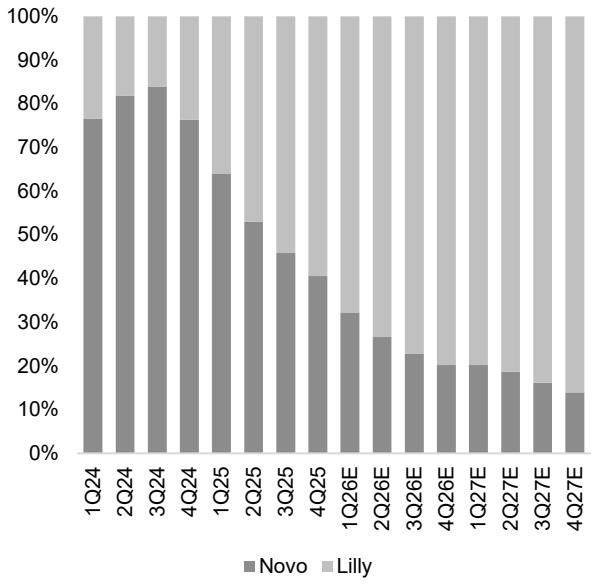
- **Price cuts impact the top-line immediately, while TRx uplifts only accrue in the future.** +25% yoy script growth still represents a more begin slowdown in TRx growth than we have seen historically. In our view, Lilly's market position (40% of TRx volumes and growing) means it can shoulder -30% price cuts and maintain positive sales growth through FY26. We don't think Novo will be able to take enough share of a) "organic" new patient starts, b) compounding patients moving to branded medications, or c) patients using Lilly's Zepbound to deliver positive Wegovy US sales growth in FY26.

- **The early evidence shows that price cuts can increase volumes… for Lilly.** We are careful to mix causation and correlation (there is a strong underlying structural trend supporting Zepbound (Exhibit 48) that shouldn't be understated)

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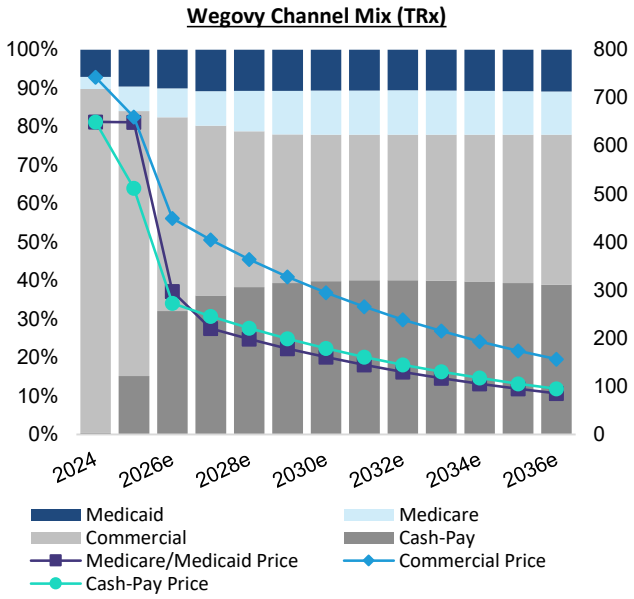
but all evidence of price cuts so far suggest to us that the benefits fall disproportionately to Lilly. Consumers (and physicians) seem to view the slightly higher price for Zepbound (c$669 vs $625 blended net prices in FY25) as worth it, given the increased weight loss vs Wegovy (-21% vs -15% weight loss; SURMOUNT-1 & STEP-1 respectively Exhibit 55). We think this perception of value is entrenched, so long as Lilly continues to keep prices within bound for the additional weight loss to be deemed "worth it".

EXHIBIT 49: **We forecast that Lilly will continue to capture share in the Medicare and Medicaid channels**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

EXHIBIT 50: **The cash channel is becoming a larger share of prescriptions but are typically priced lower than other channels**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

- **Medicare and Medicaid coverage is a tailwind beginning in 2Q26, but we believe their importance may have been overstated.** We have modeled +50% and +30% TRx volume growth in the Medicare and Medicaid channels respectively for Wegovy this year. In 4Q25, we estimate that Novo accounted for 41% of total Medicare and Medicare (combined) prescription volumes. This had fallen from 64% in 1Q25 (-23% share loss). We forecast this trend to continue over the next two years with -12% and -6% share in FY26 and FY27 respectively.

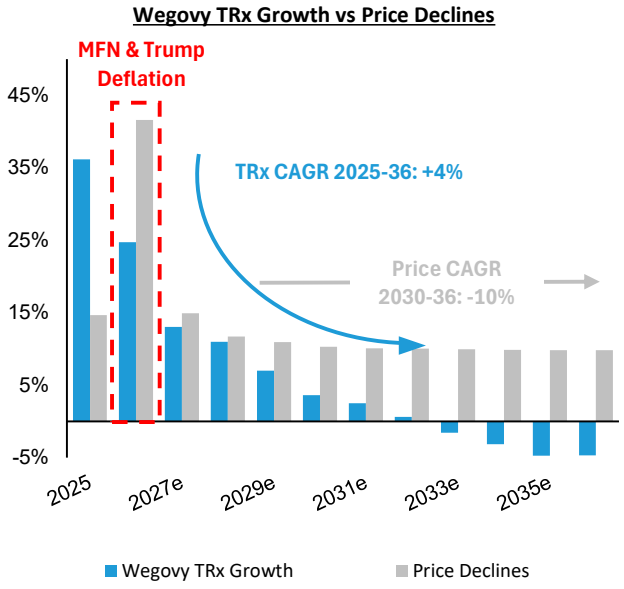
- **The cash-channel is much more sensitive to consumer sentiment and its price deflation spills into other channels.** In 2025, 15% of Wegovy prescriptions were derived from the cash channel (Exhibit 50). In early 2026, 30% of TRx came via the cash channel. Going forward, we model the cash channel to continue to increase up to 40% by 2027, primarily at the expense of the commercial (private insurance) channel (Exhibit 50).

Beyond FY26, we still do not think US Wegovy TRx growth will be enough to generate positive US revenue growth for Novo. Price declines of 10-20% yoy are standard in the obesity market (take the price decline into FY25 in Exhibit 51) and we have modeled -10% yoy 2027-36. To deliver positive US revenue growth, TRx growth needs to run ahead of the rate of price declines. But in our model, we do not forecast Wegovy TRx growth to return to >10% across the next decade (Exhibit 51). The Wegovy molecule, semaglutide, loses compound patent protection (its most robust intellectual property) in 2032. This is why we forecast volumes to negatively inflect. We also believe that we have been generous not to assume more aggressive price and volume declines for this drug during this period.

Net net, we think price declines will erode 79% of the US market's (and 87% of Novo's) growth value over the next decade. Exhibit 53 and Exhibit 54 show that, using today's net prices, our total prescription volume forecasts (across all on-market and pipeline obesity products) would add $335bn in market value ($60bn to Novo) by 2036. But we expect Novo to suffer particularly acutely from deflationary pressures due to the semaglutide loss of exclusivity (LOE) in the US in 2032, which will allow generic players to enter the market (The US Zepbound compound patent doesn't expire until 2036 - Exhibit 52).

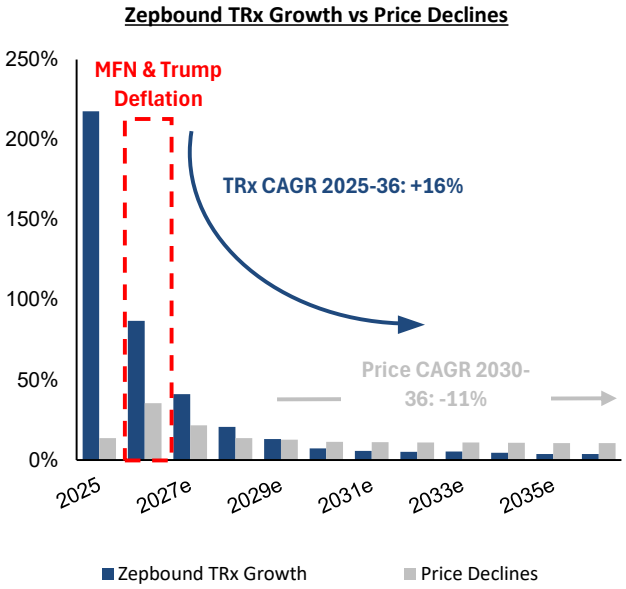
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EXHIBIT 51: **We think US Wegovy revenues are entering a secular decline**



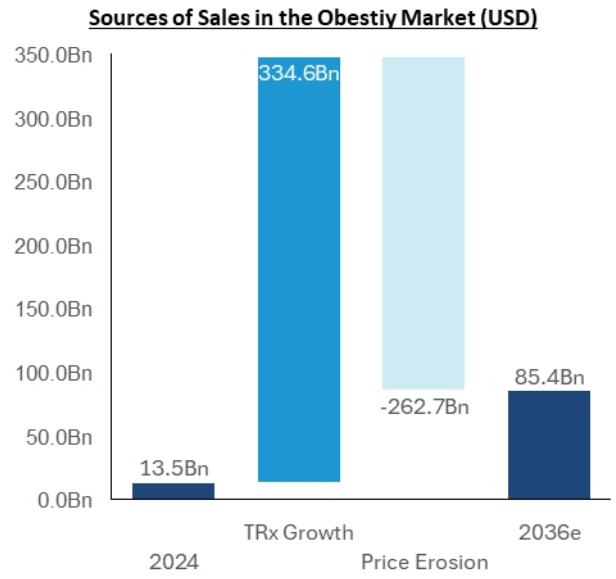
Source: Company Reports, IQVIA, Bernstein analysis and estimates

EXHIBIT 52: **We forecast Zepbound revenue growth to go negative much later than Wegovy**



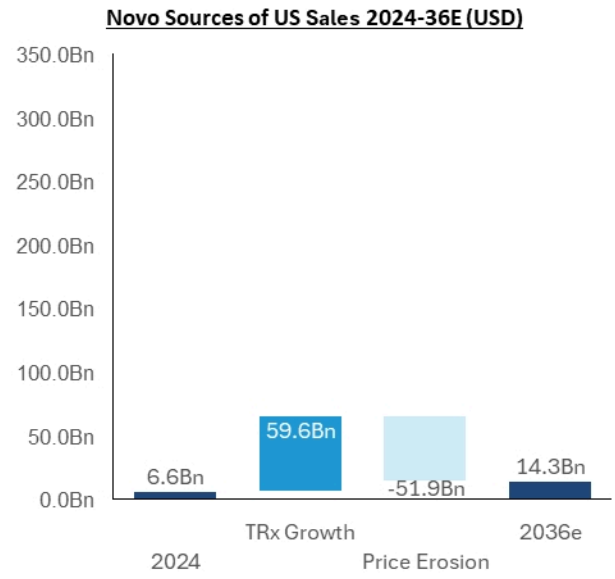
Source: Company Reports, IQVIA, Bernstein analysis and estimates

EXHIBIT 53: **US obesity market sources of value 2024-36e**



Source: IQVIA, Company Reports, Bernstein analysis and estimates

EXHIBIT 54: **Novo sources of value 2024-36e**



Source: IQVIA, Company Reports, Bernstein analysis and estimates

THE STATE OF PLAY FOR 2026: THREE BIG THEMES BEYOND DEFLATION

1/ Wegovy vs Zepbound: Still Important, More of the Same

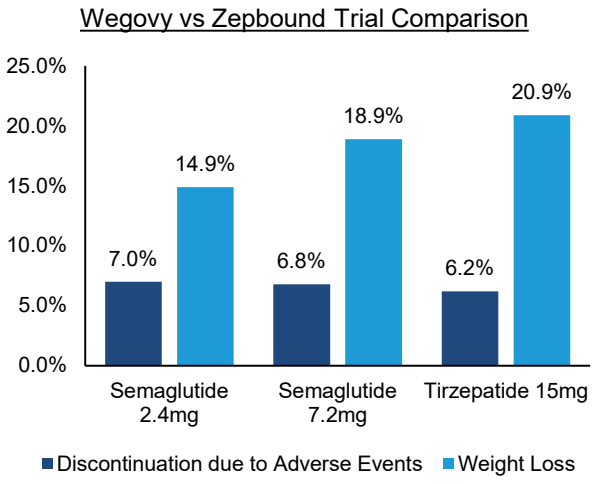
- **In our view Wegovy will continue to lose share to Zepbound in the US.** We think injectables (including compounders) will account for 88% of prescriptions volumes in FY26 and injectables will remain the driving force behind the US obesity market. We think Wegovy and Zepbound are competitively priced and patient perceptions about better weight loss (for Zepbound) are now fully baked in (Exhibit 55). Therefore, we think a continued (but steady) share loss vs Zepbound is

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the most likely scenario. Across 2025, weekly Wegovy scripts were (almost) flat, while Zepbound has continued to build momentum Exhibit 56.

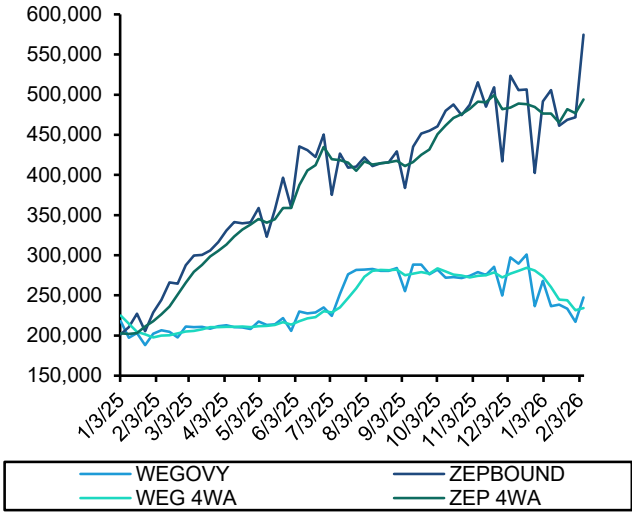
- **Wegovy's higher (7.2mg) dose will not be enough to reverse share losses.** After submission using Novo's CNPV in November 2025, management is expecting an FDA decision in 2Q26. We acknowledge the higher dose achieves weight loss using semaglutide which is closer to tirzepatide (active ingredient in Lilly's Zepbound). But in our view Zepbound remains more efficacious (and has a better tolerance profile (Exhibit 55)). Thus, we are unconvinced that a higher Wegovy dose will be able to drive enough share capture of new or existing patients to materially inflect current share trends between Zepbound and Wegovy (Exhibit 56). In our view, Lilly is managing Zepbound's life-cycle very effectively. This includes recently launching the KwikPen, which contains four doses in the same pen. We also do not believe that the Wegovy 7.2mg has strong enough weigh loss data to materially change patient and physician perceptions.

EXHIBIT 55: **Zepbound continues to achieve better weight loss than Wegovy**



Sema 2.4 (STEP-1), Sema 7.2 (STEP-UP), Tirzepatide (SURMOUNT-1)
Source: NEJM, Bernstein Analysis

EXHIBIT 56: **We think general script trends from 2025 will continue into 2026…**



Source: IQVIA, Bernstein Analysis

We believe consensus is too bullish on TRx volumes or is not pricing in enough deflation

Using consensus revenue estimates for US Wegovy, we have backed out implied TRx and price assumptions. We thus believe that consensus is either:

a) overestimating the potential for Wegovy Trx's to inflect in the short term

or

b) underestimating the amount of deflation coming into the market across the next decade.

In our view, consensus appears to be optimistic on both counts.

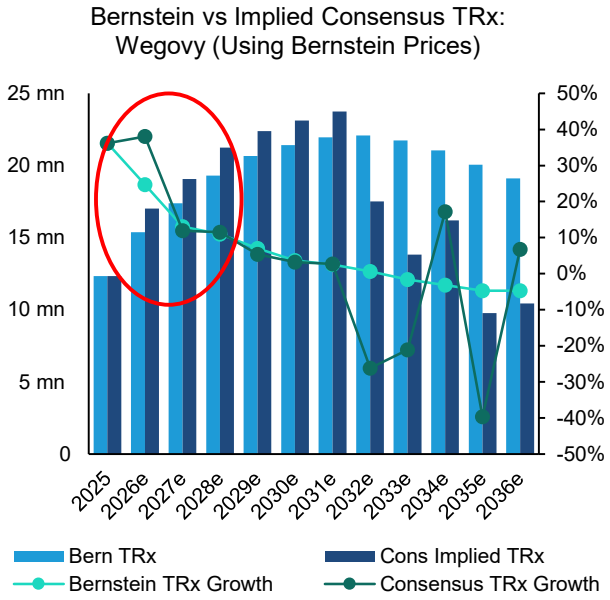
US prescriptions: We believe that consensus has overly bullish assumptions on FY26 US Wegovy script growth and share loss to Zepbound. We have spoken already about our reasoning for expecting Wegovy to continue to lose share vs Zepbound and why we think the expected TRx uplift will come in FY27, not FY26 (for example, lawsuits with compounders will take longer to resolve in our view and increase in medicare/medicaid scripts will take time to inflect).

In Exhibit 57 we use the blended (by channel) net price assumption in our full Novo earnings model ($365 in FY26 and -10% annual declines 2027-36) to calculate the implied prescription volume trend. The prescription growth rates between Bernstein and consensus are very similar until the LOE (where we model a benign erosion from generics; more here).

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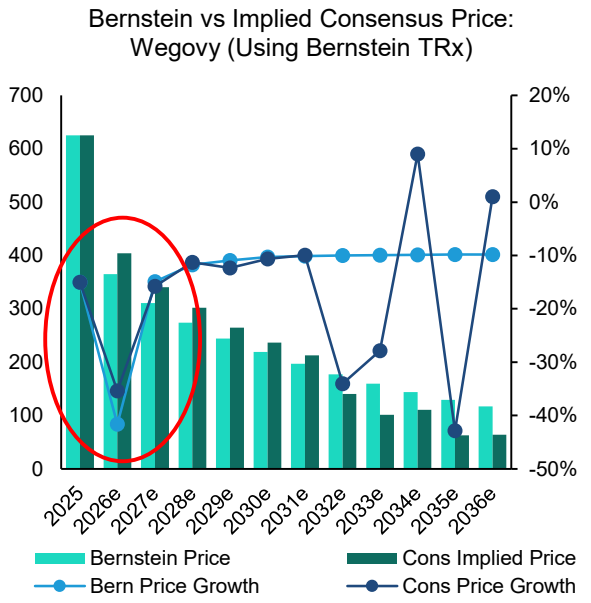
The main divergence between Bernstein and consensus is Wegovy US total prescriptions in FY26. We think consensus figures imply 38% TRx growth in FY26. This would be higher yoy than FY25 (+36%). Given our discussion on TRx growth in FY26 and FY27, we believe it is unlikely that growth in FY26 will be higher than FY25 (we opt for a softer decline in growth rates). Further, plugging the implied consensus TRx figures into our obesity model, we think this implies minimal loss vs Zepbound in 2026. In our view this is too bullish as we don't expect any near term milestones to inflect the 2025 trends.

EXHIBIT 57: **We think consensus is pricing in a large TRx inflection this year in US Wegovy scripts but we have modeled softer than expected decline in TRx growth**



Source: BBG Consensus, Company Reports, Bernstein analysis and estimates

EXHIBIT 58: **We think consensus has not priced in enough deflation over the short-to-medium term**



Source: BBG Consensus, Company Reports, Bernstein analysis and estimates

Price: whilst we have modeled benign price deflation, consensus seems even more optimistic. In our 'base case' Novo earnings model, we assume use a -10% yoy price decline for Wegovy in the US through 2027-36. We believe that leaves prices low enough at the point of the LOE to mitigate aggressive generic entry. But we believe that consensus may be underestimating the level of deflation coming into the market across the next decade.

In Exhibit 58 we use our own US Wegovy TRx assumptions (as shown in Exhibit 51) to back out the consensus implied blended net price. Our assumptions imply a 42% yoy blended net price decline (-30% on our 4Q25 prices). We think consensus is pricing in a -35% price decline in 2026. Given that the majority of prices have been announced (i.e. Medicare/Medicaid/TRumpRx), we believe consensus is underestimating spillovers into the commercial (typically the highest-priced) channel.

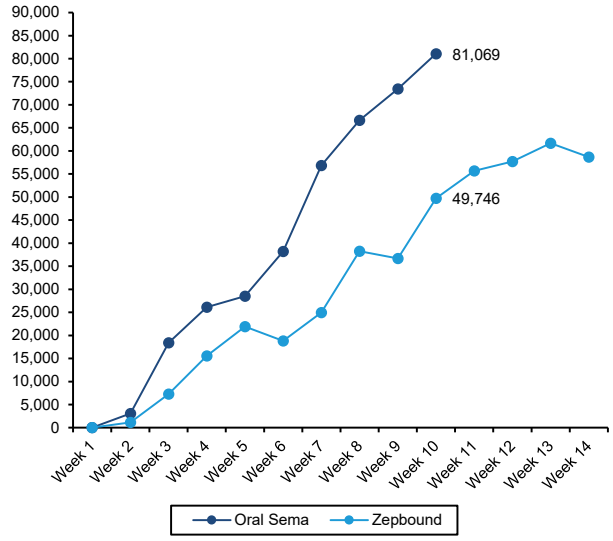
Net net, this implies a much lower net price in FY26. But we also think consensus is discounting softer declines through to the LOE (HSD declines). Hence, in our view, consensus is being too optimistic on the impact of deteriorating mix. We expect the lowest price channel (i.e. 'cash') to consistently grow much quicker than the highest price (i.e. 'commercial') channel.

2/ Orals Entering the US Obesity Market

- **The Wegovy Pill (oral semaglutide) launched in the US in January 2026.** So far, the launch has exceeded expectations (weekly TRx sit well above Zepbound launch; Exhibit 59). Early indications also indicate that oral semaglutide is expanding the obesity market (given such a large % are on starter doses; Exhibit 60) rather than cannibalizing the injectables market.

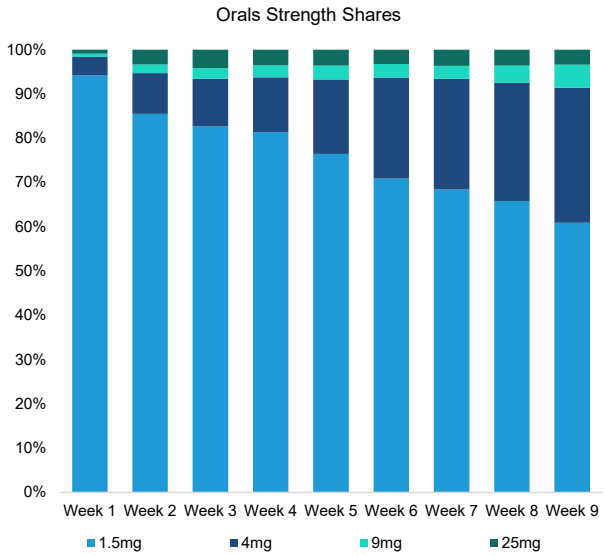
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EXHIBIT 59: **Wegovy Pill weekly TRx are sitting well above Zepbound on a like-for-like basis**



Source: IQVIA, Bernstein analysis

EXHIBIT 60: **Most Wegovy Pill prescriptions have been for the starter dose so far**



Source: IQVIA, Bernstein analysis

- We have used our own net price assumptions channel by channel for FY26 (which equates to $253 per month on a blended basis) to calculate the US TRx ramp implied by consensus forecasts for the Wegovy Pill in 2026 (Exhibit 61). We have also included our own TRx ramp, the first year of Zepbound's launch, and the current Wegovy Pill scripts total. The launch so far has also landed above consensus expectations.

- We sit below consensus for Oral Sema's launch year of TRx volumes. We think that the above expectations launch will normalize towards the end of the year as a) the initial momentum garnered from the "all-in" marketing approach will fade and b) the launch of orforglipron coming in 2H26.

- **We think the Wegovy Pill can prolong Novo's fight in the obesity market, but it won't change its fortunes.** Novo has overcome investor concerns about the Wegovy Pill's US launch, continuing to outperform market expectations (Exhibit 59). We also take comfort from the early indications that the market is expanding and not just changing structure (Exhibit 60). But despite this, we remain cautious of the ability of the Wegovy Pill to inflect Novo's group-wide fortunes. In particular, we note that this launch has not been "cost-free" because of **1/** capex remaining above "steady-state" until the end of the decade **2/** material increases in SG&A to compete with Lilly **3/** negative mix: 'Wegovy in a Pill' is lower margin than injectable Wegovy because of its lower selling-price per unit and higher COGS due to more API in the Wegovy Pill:

- **Despite the strong execution thus far on US TRx volumes, the success of the Wegovy Pill launch has not been cost free.**

  - As a reminder, the stakes on the Wegovy Pill launch were so high given the unforeseen challenges of the Wegovy Injectable launch. In 2021, Novo were first to market with a weekly GLP-1 for obesity in the US, but insufficient supply of semaglutide allowed competitors such as Lilly, through Zepbound, and compounding pharmacies, through compounded semaglutide, to take share.

  - **Capex is set to remain elevated for the next few years.** Considering the supply constraints Novo faced with the Wegovy injectable, a $25bn capex program was initiated in 2022 to expand Novo's in-house production and fill-finish capabilities. It is our understanding that we are at the end of this particular capex cycle (which would suggest a peak capex of 21% of sales in 2025). But we don't expect capex to return to steady state levels (of ~5% of sales) until 2029 as sites continue to ramp up.

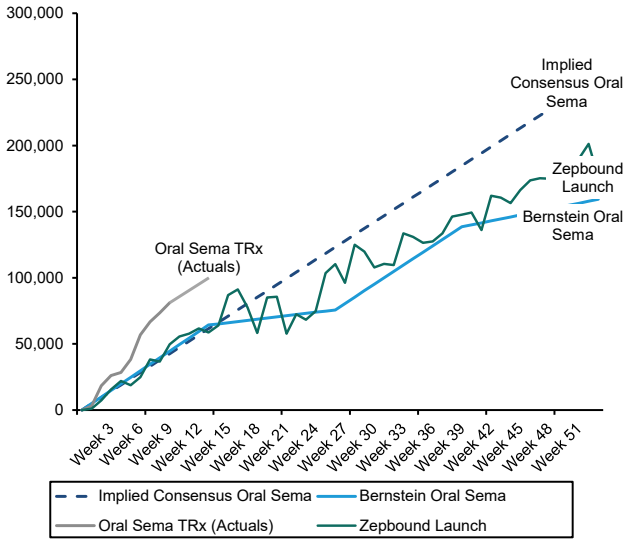
  - **Management said they went "all-in" on the launch.** So, by management's own admission, this is best the Wegovy Pill could've performed. The launch has taken significant resources and investment, including extensive advertising on the New York Subway and at the Super Bowl. We think the advertising push has contributed to the Pill's successful launch.

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But we think this level of momentum and investment is unsustainable over the medium and long term - especially when orforglipron enters the market (expected launch could be as early as 2Q26).

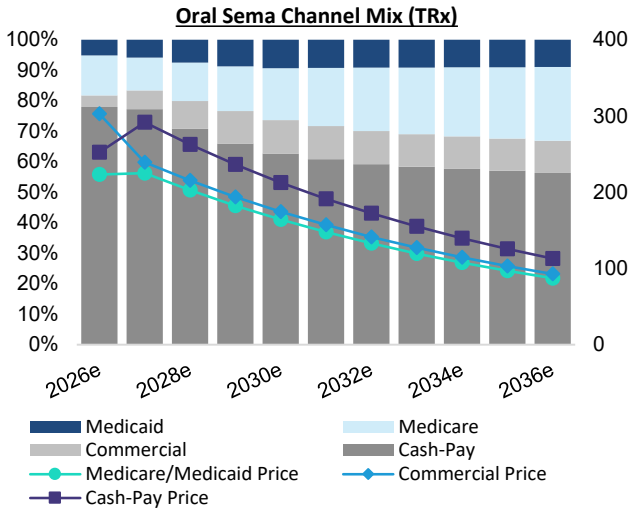
- **We also note that the orals market will face the same pricing pressures as the injectables, but from a lower base and lower margin.** We assume the same -10% price deflation in the oral segment as we do in the injectables from 2027 onwards. The first two doses of the Wegovy Pill cost $149 in the cash channel (the first two doses of the injectable are $199), but the amount of active pharmaceutical ingredient (API) is much higher in the Wegovy Pill, meaning that the gross margin is considerably lower on a per-unit basis.

EXHIBIT 61: **We sit slightly above consensus on our forecast for the Wegovy Pill TRx ramp up this year**



Source: IQVIA, Bernstein analysis

EXHIBIT 62: **We think the majority of Wegovy Pill sales will come through the cash channel**



Source: IQVIA, Company Reports, Bernstein analysis and estimates

- **What else do you need to know about the Wegovy Pill?**

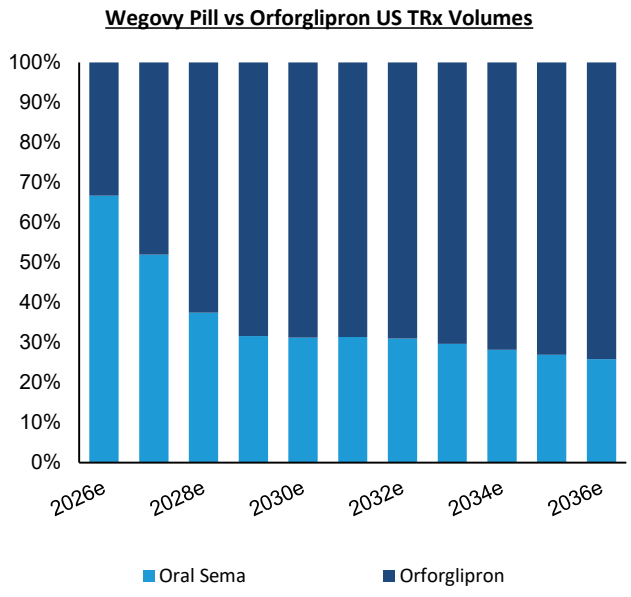
  - The Wegovy (injectable; 2.4mg) and the Wegovy Pill, have comparable weight loss (-15% vs -14%; STEP-1 and OASIS-4 respectively). We believe that some patients will prefer to take a daily oral because it is more discreet than taking an injection, although others will prefer the convenience of a "one-and-done" approach they can have with the weekly injection. As the weight loss is comparable, we think these effects will net out.

  - Semaglutide is a peptide, which means that it has relatively low bioavailability. As a result, the Wegovy Pill requires much more API (active pharmaceutical ingredient) than the injectable version. Reformulating into a tablet has required Novo to develop a proprietary technology to ensure that the stomach doesn't completely break down the semaglutide before it can be absorbed into the bloodstream. (We understand from Novo that the technical challenges here were significant).

  - The Wegovy Pill comes will a 30-minute fasting restriction between taking the pill and eating. In our view, this will not be a significant impediment for patients, given the same restrictions apply to Rybelsus (which has become $3bn drug despite being launched during the pandemic and receiving limited marketing resources). The Wegovy Pill package insert benefits from the exact same positive (relative risk reduction of 20%) cardiovascular outcomes' data (SELECT) as the Wegovy injectable.

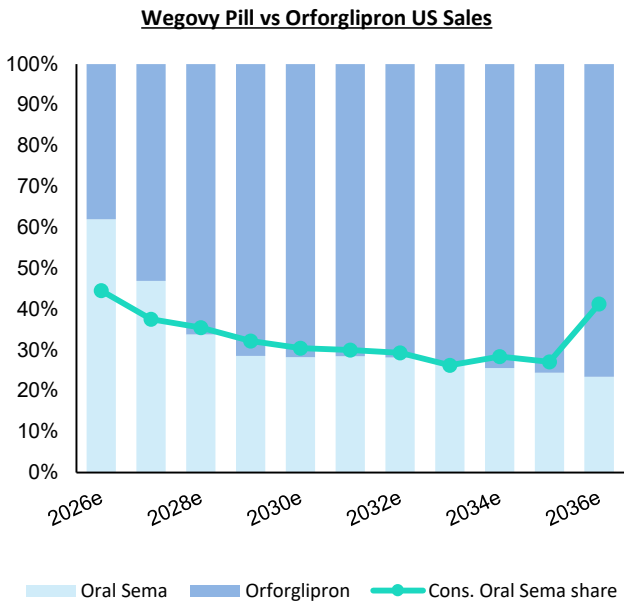
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EXHIBIT 63: **We think Orforglipron will dominate TRx volumes vs Oral Sema**



Source: Company Reports, Bernstein analysis and estimates

EXHIBIT 64: **And we sit slightly below consensus for Oral Sema share of US sales vs Orforglipron**



Source: BBG, Company Reports, Bernstein analysis and estimates

- **Orforglipron is due to come onto the US market by 2Q26.** Orforglipron was invented by Chugai (O; Miki Sogi) and licensed to Lilly in 2018. As orforglipron is a non-peptide, it doesn't have the same bioavailability challenge as the Wegovy Pill. We also expect the orforglipron CVOT phase 3 data to be published in March 2026.

  - With orforglipron, across the 6mg, 12mg, 36mg doses, the ATTAIN-1 P3 trial recorded -7.5%, -8.4%, and -11.2% weight loss. The headline weight loss compares poorly to Zepbound (-20%) and both formulations of Wegovy (-15% and -14% for the injectable and oral respectively). In ACHIEVE-3, orforglipron demonstrated much stronger weight loss than Oral Sema (-9.2% vs -5.3%). Whilst we do not have the full publication (so haven't been able to fully scrutinize the data), we expect orforglipron to be the dominant oral in the market in terms of script volumes (Exhibit 63) and in US sales (Exhibit 64). We expect the former to command 80% of the US volume share between the two drugs by2036e.

  - If the weight-loss data is "muddy" (i.e. no clear winner), why do we project orforglipron to enjoy a much higher share than Wegovy Pill? Well, we believe that Lilly's sales force benefits from stronger economic incentives because of historic stock price performance. Our channel checks indicate that its much more difficult to retain good sales people during periods of share price underperformance because of the resulting reduction in the value of stock options. We understand that for US bio-pharma employees, stock options are very important component of total financial remuneration.

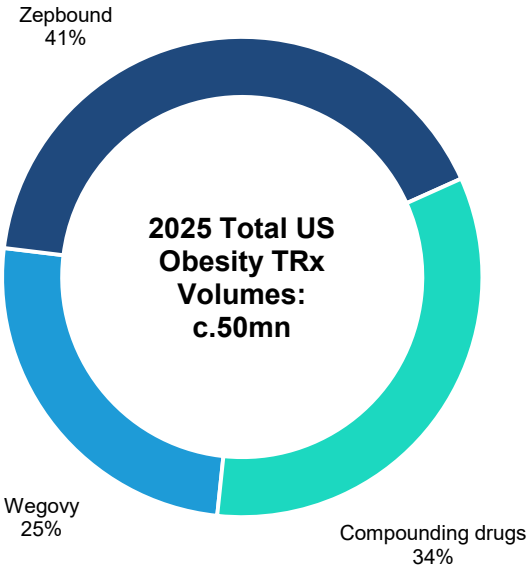
3/ Compounders finally out?

- **A quick recap: why are there compounders in the US market?** The FDA allowed compounders into the US obesity market under section 503B, which allows compounding in cases where there is a supply shortage of the molecule. In February 2025, the FDA officially announced there was no longer a semaglutide supply issue. Since then, compounders have been slow to retreat from the market (our channel checks indicated that these discounters still account for 33% of prescription volumes in FY25).

- **We believe that 2026 is the year compounders will be pushed out by the FDA.** The strategy employed by Novo (file lawsuits and cut prices) failed to make a material change to the staying power of compounding pharmacies in FY25 (despite filing 140 lawsuits and already having won >40 of them). But in our view, the FDA's announcement in February 2026, marks a significant shift in the willingness of the FDA to take action. We believe that a) compounders will be unable to enter the orals market at all and that b) their share of total prescription volumes will fall sharply this year (Exhibit 66).

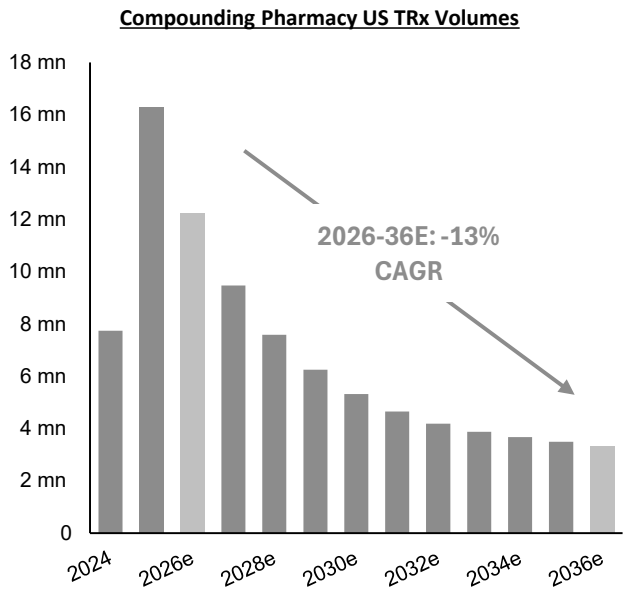
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EXHIBIT 65: **Our channel checks indicate that compounders held 33% of TRx volumes in 2025**



Source: IQVIA, Company Reports, Bernstein analysis and estimates

EXHIBIT 66: **We think compounding pharmacy TRx will fall sharply over the next few years**



Source: Bernstein analysis and estimates

EXHIBIT 67: **Latest FDA press release on illegal compounders**



FDA STATEMENT

FDA Intends to Take Action Against Non-FDA-Approved GLP-1 Drugs

More Press Announcements

For Immediate Release: February 06, 2026
Statement From: Martin A Makary, M.D., M.P.H.
Commissioner of Food and Drugs - Food and Drug Administration

Content current as of:
02/06/2026

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Today, the U.S. Food and Drug Administration is announcing its intent to take decisive steps to restrict GLP-1 active pharmaceutical ingredients (APIs) intended for use in non-FDA-approved compounded drugs that are being mass-marketed by companies — including Hims & Hers and other compounding pharmacies — as similar alternatives to FDA-approved drugs. These actions are aimed to safeguard consumers from drugs for which the FDA cannot verify quality, safety, or efficacy. We take seriously any potential violations of the Federal Food, Drug, and Cosmetic Act.

Source: FDA

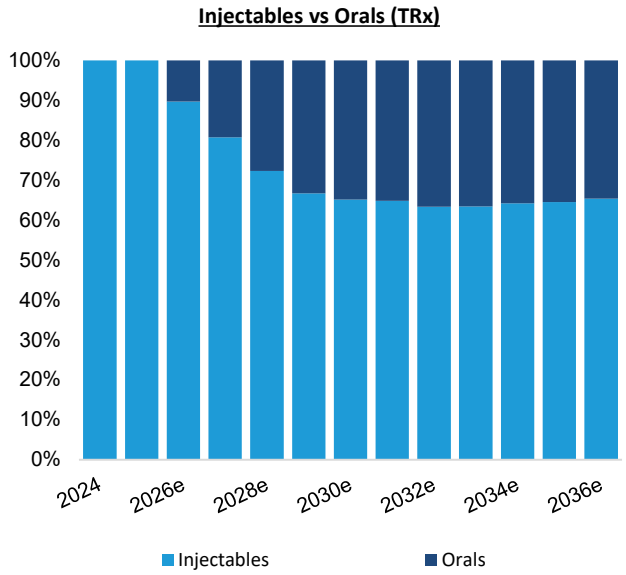
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KEY THEMES ACROSS TO 2036

1/ Orals Endgame: What Share Will Orals command?

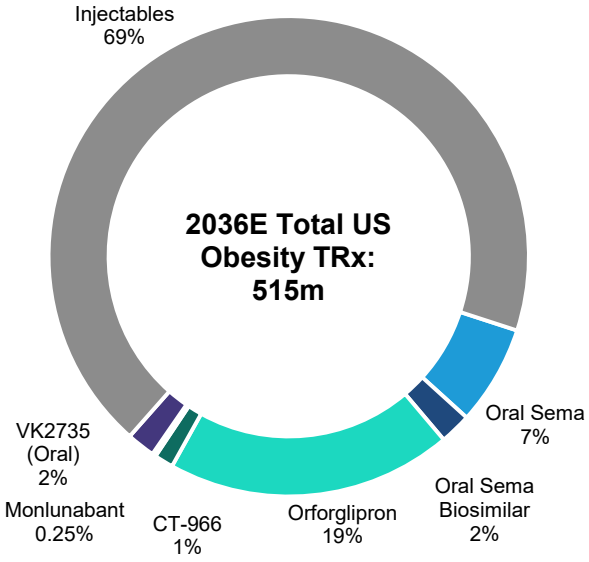
- **The launch of obesity orals (tablets) has attracted a lot of attention, but don't forget the importance of injectables.** Overall, we project the orals market will peak at 35% of the total US obesity TRx volumes, and begin to trend down thereafter as assets currently in the pipeline come to market (Exhibit 68). We think at "steady-state" (but beyond our forecast horizon) orals should account for 20% of the total TRx volumes globally. In 2036E, we forecast that orals will account for 30% of US TRx volumes (Exhibit 69).

EXHIBIT 68: **We think that Orals will command c.30% share of the US obesity market (TRx)**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

EXHIBIT 69: **Split of US Oral shares 2036e**



Source: Company Reports, IQVIA, Bernstein analysis and estimates

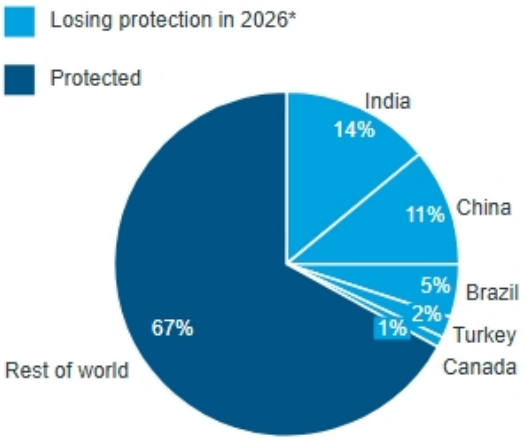
- **We don't forecast orals to replace injectables as the primary weight-loss medication in the US.** Our reasoning is twofold. First, the vast majority of pipeline assets are weekly injectables. In our obesity model (Exhibit 84), we include 19 "branded" obesity products on the market and in the pipeline, as well as compounding drugs and biosimilars for products containing semaglutide. Of these, 5 have oral formulations modeled. Second, we are yet to see an oral consistently achieve better weight loss than an injectable and, while the patients continue to value weight-loss above all else, we think orals comprising 30% of total TRx volumes in the US seems fair (Exhibit 69). Click here to go to our section on Wegovy Pill (oral semaglutide) vs orforglipron.

2/ Semaglutide Loss of Exclusivity in the US (2032)

- **Semaglutide (active ingredient in Wegovy, Wegovy Pill, Ozempic, Rybelsus and CagriSema) loses exclusivity in the US in 2032.** Novo has already lost (or is about to lose) compound exclusivity over semaglutide in several key locations in 2026 which represent 1/3 of the world's obese-adult population (Exhibit 70).
    - **As of February 2026, products containing the semaglutide molecule has generated revenues of > DKK 100bn for Novo since Ozempic's launch in 2017.** This molecule is the active pharmaceutical ingredient (API) in four products currently on the market (Exhibit 71) and with the launch of Wegovy Pill and CagriSema (Cagrillinitde + Semaglutide; launches expected in 2027 (obesity) and 2028 (diabetes)) we forecast further portfolio concentration in the short run (Exhibit 72). It is worth noting that our increase in non-sema revenues are driven primarily by the fall in sema revenues.

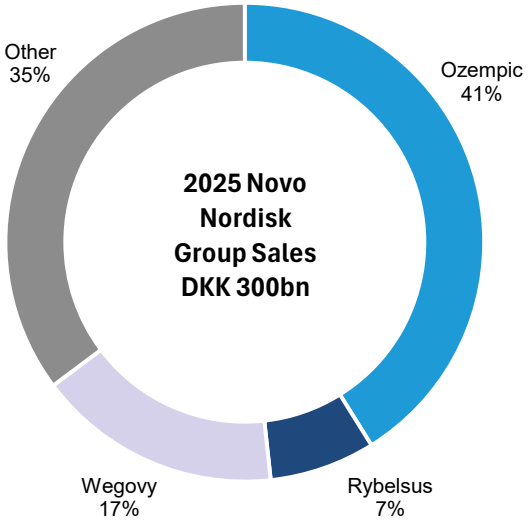
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EXHIBIT 70: **We estimate that 1/3 of the obesity TAM will be living in off-patent locations from this year**



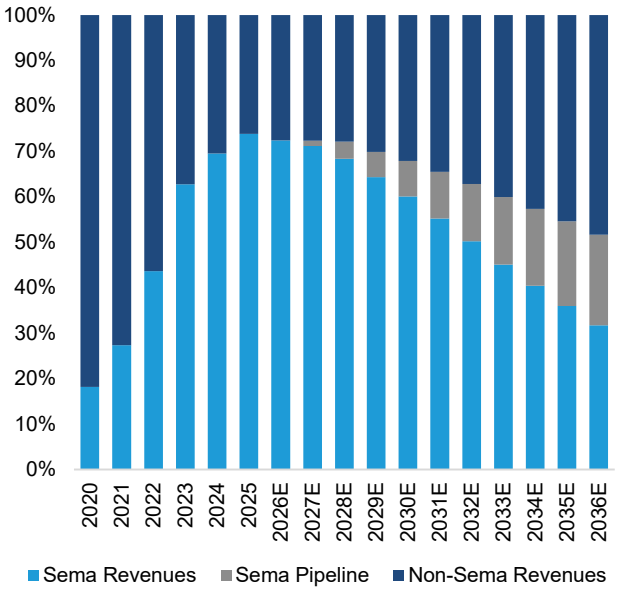
TAP = Total Addressable Population
Source: IQVIA

EXHIBIT 71: **74% of Novo Sales came from just three products in 2025 all of which contained the same molecule**



Source: Company Reports, Bernstein analysis

EXHIBIT 72: **We believe we are past "peak sema" revenues now...**



Source: Company Reports, Bernstein analysis and estimates

- **We have modeled a very benign loss of exclusivity impact across products containing semaglutide.** We have included no additional price deflation (maintaining -10% across 2027-36) and modest volumes declines across Wegovy injectable, Wegovy Pill, and CagriSema (Exhibit 73). We thus believe that our estimates might, if anything, be on the conservative side. Our forecasting here has been guided by three factors:
  - **Ultra efficient production processes:** Novo uses a yeast-based manufacturing method to produce semaglutide at large scale. (Novo has applied its learning's from decades of insulin manufacturing to perfect the sema production process).

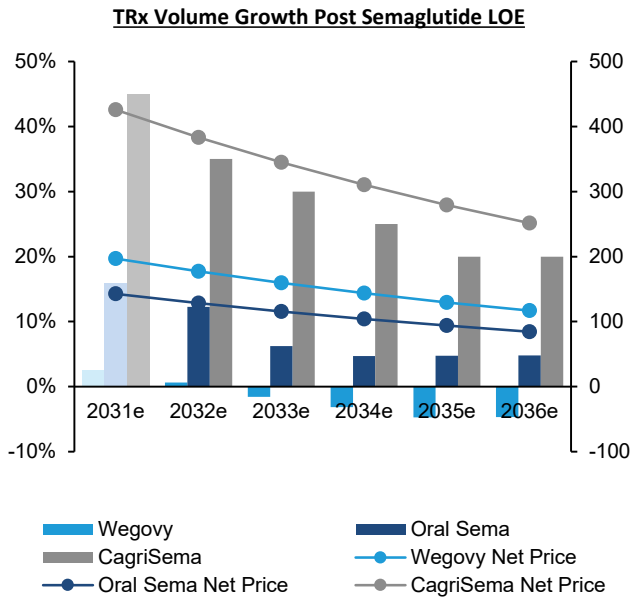
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Based on our recent call with an independent bio-manufacturing expert, we understand that this gives Novo a structural cost advantage over potential generics entrants. In addition, we have already seen that Novo is willing, when needed, to compete aggressively on price in the US. But we have not modeled any additional price competition post-LOE.

- **High API in the Wegovy Pill:** Our channel checks have also indicated that the low bio-availability of semaglutide may soften the impact of generic competition in the oral market. In particular, we believe that the low bio-availability and lower average price (to the injectable) means the margin on the oral is much lower than on the injectable (even if storage and lack of device may partially offset the COGS hit). We have not modeled negative prescription growth for Wegovy Pill post LOE, but have modeled a slowing in prescription growth rates.

- **CagriSema in the Mix:** We go into much more detail on CagriSema (obesity; here, diabetes; here), but we understand that combining Cagrilintide and Semaglutide is difficult due to the differences in pH (semaglutide is stable around pH 7 whereas Cagrilintide is stable around a pH 4). Our channel checks have indicated this could be a rate limiting step for generic entry.

EXHIBIT 73: **We've modeled a benign loss of exclusivity impact on products containing semaglutide**



Source: Company Reports, Bernstein analysis and estimates

3/ New product launches

- **Excluding the role compounders play, we expect the number of obesity drugs on the market to double between FY25 and FY26. By 2032, we think there could be >20 drugs on the market** (Exhibit 74)**.** This includes biosimilars for semaglutide containing products from 2032. We believe the launches of new products will help drive market growth. We thus forecast that 70% of the US obesity market value in 2036 is currently not on the market (Exhibit 75).

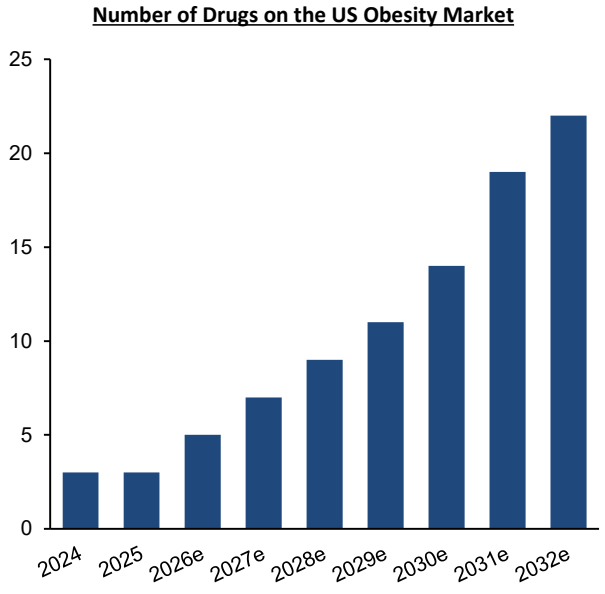
  - We expect CagriSema (Novo) and retatrutide (Lilly) to launch in 2027. In 2028, we expect the first non-Novo, non-Lilly obesity drugs to enter the market (survodutide; Boehringer Ingelheim/ Zealand Pharma (NC)) and MariTide; Amgen (MP; covered by Courtney Breen)).

CagriSema: consensus is too optimistic in our view:

CagriSema combines semaglutide with an amylin analogue called cagrilintide. Novo expects to launch CagriSema in obesity in 2027, and in diabetes in 2028. The product is formulated as a dual chamber pen because of the difference in acidity between the two molecules (as referenced earlier). In our view the key controversies surrounding CagriSema in obesity prior are:

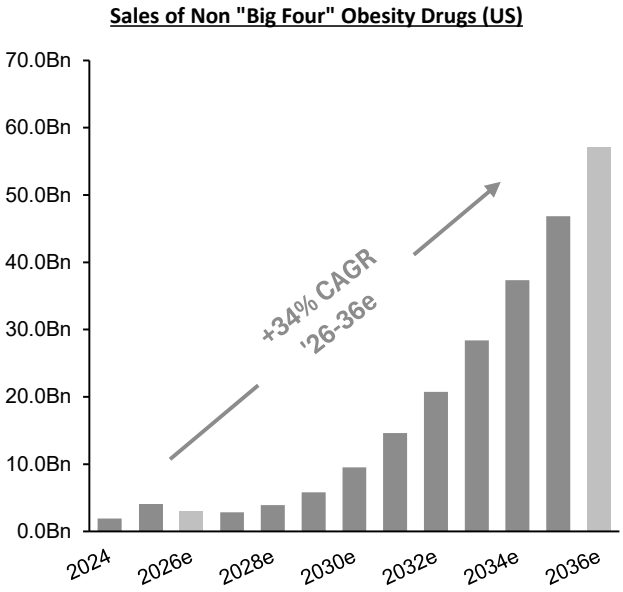
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EXHIBIT 74: **We forecast new obesity drugs to come to market each year 2026-32**



Source: Company Reports, Bernstein analysis and estimates

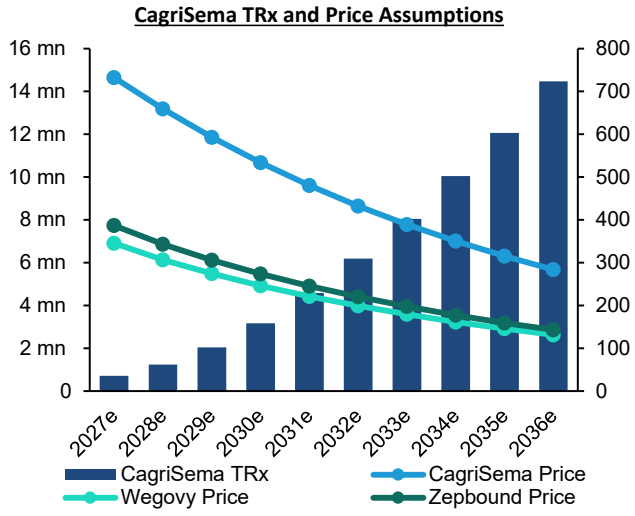
EXHIBIT 75: **We forecast 67% of the 2036E US obesity market value will come from drugs currently in the pipeline**



Source: Company Reports, Bernstein analysis and estimates

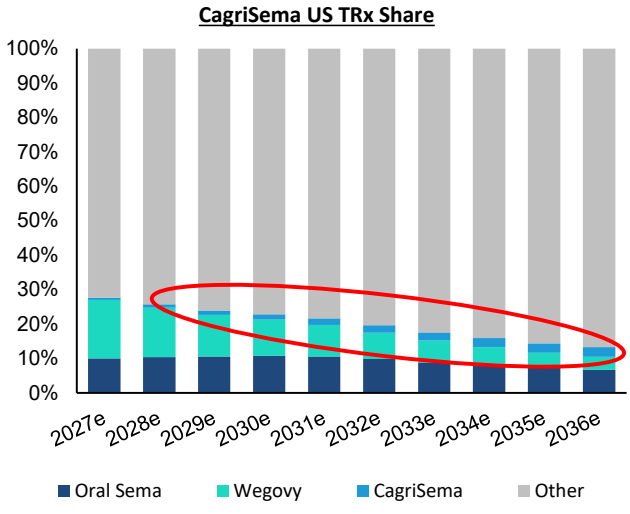
- **Target market (more of a niche than a super-blockbuster):** based on our channel checks, we understand that CagriSema is **not** going to be pitched as a mass-market obesity medication (i.e. it is not the "next Wegovy"). Instead, CagriSema is going to be targeted at patients who need really need extra weight loss. This thus believe that CagriSema is likely to become more of a "niche" obesity product than what is discounted by consensus. We have modeled a higher price (2x the average of Wegovy and Zepbound blended net prices in year of launch; $730) but much lower year 1 TRx numbers to reflect the smaller patient population Exhibit 77.

EXHIBIT 76: **We assume CagriSema will be priced higher than Wegovy and Zepbound because it will be a "niche" obesity product**



Source: Company Reports, Bernstein analysis and estimates

EXHIBIT 77: **We think CagriSema will hold 3% of the US TRx volumes in 2036**



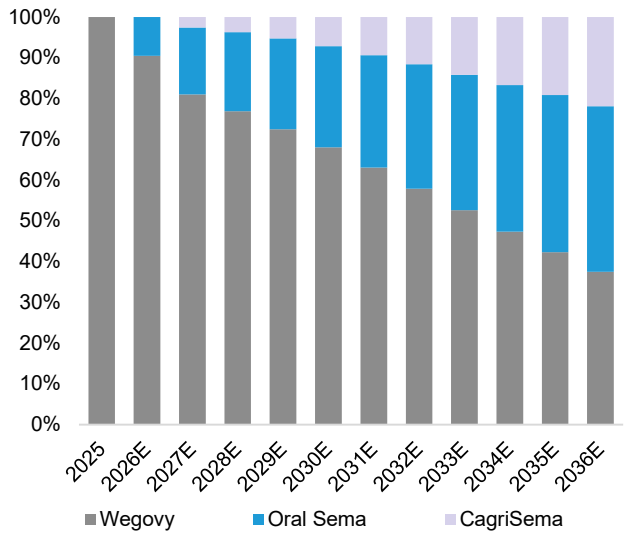
Source: Company Reports, Bernstein analysis and estimates

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We think CagriSema will account for 20% of Novo's obesity portfolio (CagriSema, Wegovy, and Wegovy Pill) by 2036e (Exhibit 78) and sales will peak at 50% of Wegovy Pill sales. We think this reflects CagriSema's positioning as a "niche" obesity product.

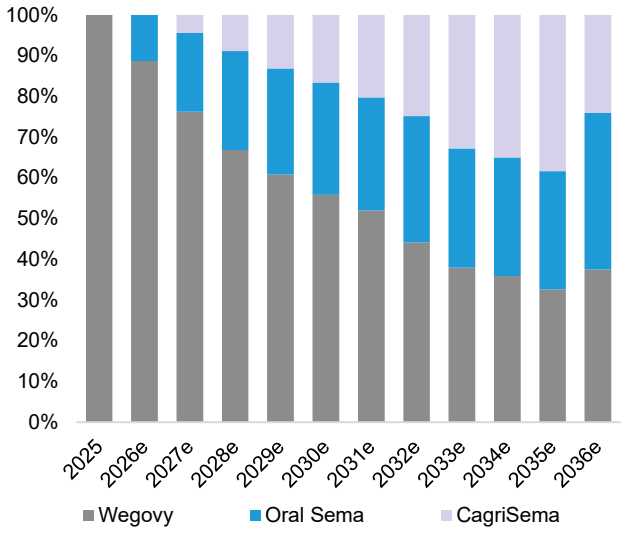
- In contrast, we think consensus views CagriSema more like a "Wegovy replacement" than a niche product. Using consensus numbers from Bloomberg, we estimate that consensus thinks CagriSema will account for 38% of Novo's obesity portfolio (CagriSema, Wegovy, and Wegovy Pill) will peak sales matching those of Wegovy Pill. In our view, this would make CagriSema a mass-market obesity medication which, from Novo's IR team, we understand will not be the case.

EXHIBIT 78: **As a "niche" obesity product, we forecast CagriSema to be around 20% of Novo's obesity portfolio by 2036e...**



LOE=Loss of Exclusivity
Source: Company Reports, Bernstein analysis and estimates

EXHIBIT 79: **But consensus has CagriSema peaking at nearly 40% of the same portfolio**

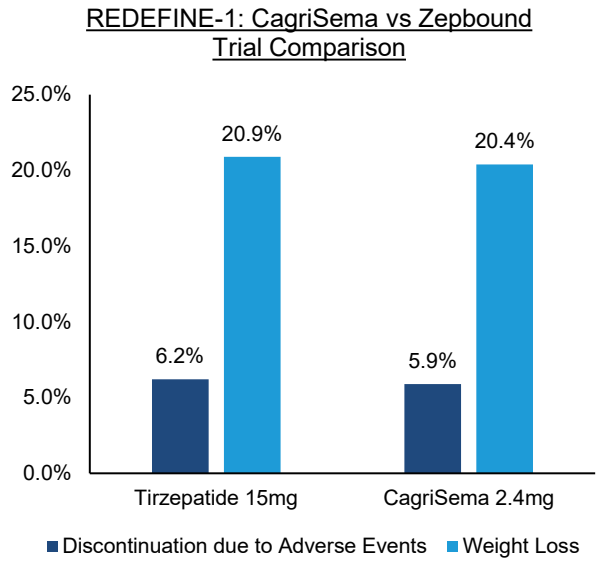


LOE = Loss of Exclusivity
Source: Company Reports, BBG Consensus, Bernstein analysis and estimates

- **REDEFINE-4:** this head-to-head phase 3 trial of CagriSema vs Zepbound (reported in February 2026). The importance of the trial was elevated after CagriSema missed investors' weight loss expectations in REDEFINE-1 in December 2024 (Novo was expecting to CagriSema to be superior to Zepbound and achieve 25% weight loss. But CagriSema only achieved 23% weight loss).
  - In REDEFINE-4, CagriSema again failed to meet 25% weight loss (achieving 23%, Zepbound achieved 25% weight loss) and also failed to meet the primary end-point of non-inferiority (i.e. the trial showed CagriSema is inferior to Zepbound). We don't see how CagriSema can be a big disruptor in the obesity market going forwards. We are still waiting for full publication of the trial results at the time of writing.
  - In Exhibit 80, we compare weight loss and discontinuation due to adverse events across REDEFINE-1 (CagriSema) and SURMOUNT-1 trial (Zepbound). While the focus has been on REDEFINE-4, management have advised that we may not see peak weight loss until REDEFINE-11 (which should report in 3Q26). But we do not think any further phase III trials data is likely to materially change the positioning of CagriSema as a niche obesity product.
- **Generic competition:** Despite being used in combination with cagrilintide, CagriSema will be at risk of generic competition in the US when semaglutide molecule patent expires in 2032. We understand that the dual chamber (DC) mechanism will not be a specific barrier to entry post semaglutide LOE, although our channel checks have indicated that the DC will make production ramp up slower than is normally the case for injectables. We also note that the cagrilintide patent will last well into the late 2030s (Novo have not confirmed when).

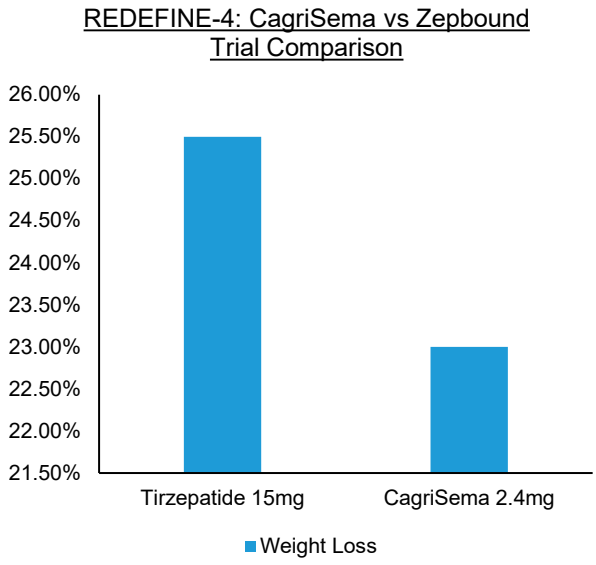
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EXHIBIT 80: **REDEFINE-1 trial results did not show superior weight loss to Zepbound (on an indirect trial comparison)**



Source: Company Reports, NEJM, Bernstein Analysis

EXHIBIT 81: **CagriSema failed prove it is "non-inferior" to Zepbound in REDEFINE-4 (head-to-head trial comparison)**

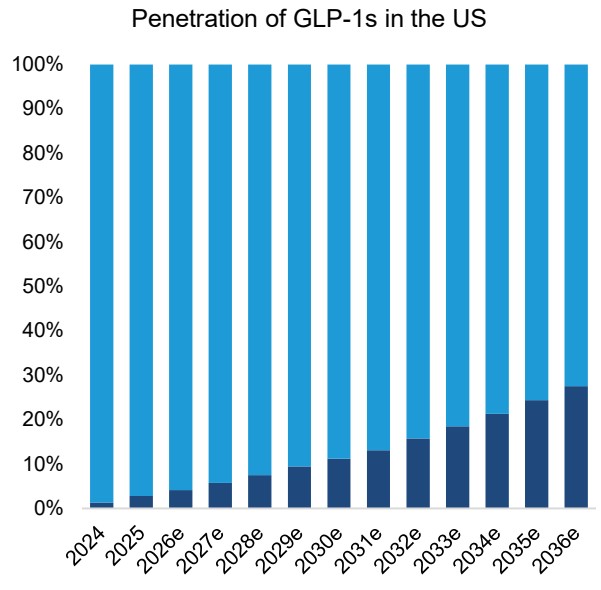


Source: Company Reports, Bernstein Analysis

WHERE DO WE SEE US GLP-1 PENETRATION IN 2036?

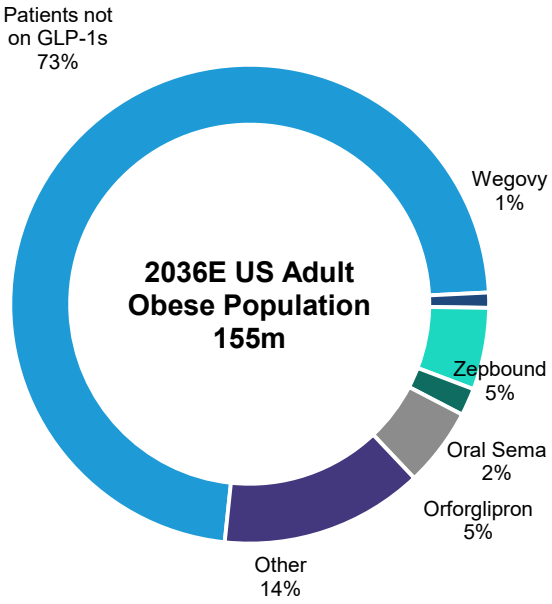
Our obesity model (Exhibit 84) implies that 28% of US adults with obesity will be using a GLP-1 for weight loss by 2036.

EXHIBIT 82: **We estimate 28% of US obesity population will be using a GLP-1 by 2036E**



Source: US Census Bureau, Bernstein analysis and estimates

EXHIBIT 83: **US patient penetration of the "big four" obesity medications in 2036**



Source: US Census Bureau, Bernstein analysis and estimates

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EXHIBIT 84: **Our US Obesity Model Summary Sheet**

US obesity model	2024	2025	2026e	2027e	2028e	2029e	2030e	2031e	2032e	2033e	2034e	2035e	2036e
US prescription volumes													
Wegovy	9,057,345	12,334,302	15,382,672	17,389,058	19,299,446	20,654,228	21,403,442	21,941,417	22,079,425	21,732,149	21,048,953	20,057,648	19,114,878
Biosimilar Wegovy									1,710,104	5,985,364	8,978,045	13,832,699	15,719,682
Zepbound	6,353,517	20,186,435	37,715,843	53,223,995	64,259,353	72,675,717	78,015,953	82,527,385	86,809,126	91,452,715	95,658,512	99,247,879	102,926,792
Biosimilar Zepbound													350,000
Subtotal 'high profile' injectables	**15,410,862**	**32,520,737**	**53,098,515**	**70,613,053**	**83,558,799**	**93,329,945**	**99,419,395**	**104,468,802**	**110,598,655**	**119,170,228**	**125,685,511**	**133,138,227**	**138,111,352**
Wegovy in a pill'	0	0	5,005,000	10,207,750	13,917,463	17,921,194	21,693,062	25,161,092	28,245,036	30,007,795	31,426,628	32,924,276	34,505,878
Biosimilar 'Wegovy in a pill'	0	0	0	0	0	0	0	0	6,599,319	8,491,932	8,576,852	9,653,713	10,835,029
orforglipron	0	0	2,500,000	9,444,706	23,224,682	38,686,783	47,798,054	54,982,657	62,774,046	71,129,851	79,986,800	89,259,687	98,841,113
Generic orforglipron													
Subtotal 'high profile' orals	**0**	**0**	**7,505,000**	**19,652,456**	**37,142,144**	**56,607,977**	**69,491,116**	**80,143,749**	**97,618,401**	**109,629,579**	**119,990,280**	**131,837,676**	**144,182,020**
Subtotal other products	7,738,519	16,300,627	12,225,471	11,946,197	13,311,218	20,010,668	33,251,922	53,823,184	78,997,525	110,580,352	147,813,855	187,871,294	231,204,887
TOTAL OBESITY TRx	**23,149,381**	**48,821,365**	**72,828,986**	**102,211,706**	**134,012,161**	**169,948,590**	**202,162,432**	**238,435,735**	**287,214,581**	**339,380,159**	**393,489,646**	**452,847,197**	**513,498,259**
TOTAL OBESITY TRx Growth		111%	49%	40%	31%	27%	19%	18%	20%	18%	16%	15%	13%
Market shares (TRx)													
Wegovy	39%	25%	21%	17%	14%	12%	11%	9%	8%	6%	5%	4%	4%
Biosimilar Wegovy	0%	0%	0%	0%	0%	0%	0%	0%	1%	2%	2%	3%	3%
Zepbound	27%	41%	52%	52%	48%	43%	39%	35%	30%	27%	24%	22%	20%
Biosimilar Zepbound	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%
Subtotal 'high profile' injectables	**67%**	**67%**	**73%**	**69%**	**62%**	**55%**	**49%**	**44%**	**39%**	**35%**	**32%**	**29%**	**27%**
Wegovy in a pill'	0%	0%	7%	10%	10%	11%	11%	11%	10%	9%	8%	7%	7%
Biosimilar 'Wegovy in a pill'	0%	0%	0%	0%	0%	0%	0%	0%	2%	3%	2%	2%	2%
orforglipron	0%	0%	3%	9%	17%	23%	24%	23%	22%	21%	20%	20%	19%
Generic orforglipron	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%
Subtotal 'high profile' orals	**0%**	**0%**	**10%**	**19%**	**28%**	**33%**	**34%**	**34%**	**34%**	**32%**	**30%**	**29%**	**28%**
Amycretin	0%	0%	0%	0%	0%	1%	2%	3%	3%	4%	4%	4%	4%
CagriSema	0%	0%	0%	1%	1%	1%	1.56%	1.92%	2.15%	2.37%	2.55%	2.66%	2.82%
CagriSema Biosimilar	0%	0%	0%	0%	0%	0%	0.00%	0.00%	0.39%	0.49%	0.61%	0.74%	0.89%
Retatrutide	0%	0%	0%	2%	2%	3%	3%	4%	5%	5%	6%	6%	7%
Compounding drugs	33%	33%	17%	9%	6%	4%	3%	2%	1%	1%	1%	1%	1%
Other other	0%	0%	0%	0%	1%	3%	7%	12%	16%	20%	24%	27%	29%
Subtotal other products	**33%**	**33%**	**17%**	**12%**	**10%**	**12%**	**16%**	**23%**	**28%**	**33%**	**38%**	**41%**	**45%**
TOTAL OBESITY TRx	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%
Prices ($)													
Wegovy blended by channel	733	625	365	311	274	244	219	197	177	160	144	130	117
Zepbound blended by channel	775	669	432	338	292	255	226	201	179	159	142	127	114
Medicare	0	0	223	225	203	182	164	148	133	120	108	97	87
Medicaid	0	0	223	216	194	175	157	141	127	115	103	93	84
Commercial	0	0	275	216	194	175	157	141	127	115	103	93	84
Cash-Pay	0	0	222	216	194	175	157	141	127	115	103	93	84
Wegovy 'in a pill' blended by channel	0	0	231	217	195	176	158	143	128	116	104	94	84
Oforglipron' blended by channel	0	0	283	265	229	204	183	164	147	132	119	107	96
Amycretin	0	0	0	0	0	0	249	249	249	249	249	249	249
CagriSema	0	0	0	0	649	584	526	473	426	383	345	310	279
CagriSema Biosimilar	0	0	0	0	0	0	0	0	214	214	214	214	214
Retatrutide	0	0	0	0	0	283	283	283	283	283	283	283	283
Compounders	0	250	250	250	250	250	250	250	250	250	250	250	250
US sales ($m)													
Wegovy	6,637	7,712	5,614	5,401	5,290	5,042	4,687	4,320	3,911	3,467	3,026	2,601	2,235
Biosimilar Wegovy	0	0	0	0	0	0	0	0	94	329	494	761	865
Zepbound	4,926	13,514	16,286	18,056	18,753	18,517	17,632	16,575	15,524	14,561	13,589	12,608	11,696
Biosimilar Zepbound	0	0	0	0	0	0	0	0	0	0	0	0	107
Wegovy in a pill'	0	0	1,157	2,211	2,715	3,150	3,435	3,589	3,629	3,471	3,273	3,087	2,913
orforglipron	0	0	709	2,498	5,312	7,878	8,724	9,020	9,254	9,423	9,520	9,544	9,493
Generic orforglipron	0	0	0	0	0	0	0	0	0	0	0	0	0
Amycretin	0	0	0	0	0	582	1,019	1,579	2,289	3,090	3,863	4,635	5,562
CagriSema	0	0	0	458	722	1,072	1,495	1,951	2,371	2,774	3,120	3,370	3,640
CagriSema Biosimilar	0	0	0	0	0	0	0	0	237	355	515	721	721
Retatrutide	0	0	0	0	799	1,239	1,858	2,695	3,772	5,093	6,621	8,276	9,931
Eloralinitide	0	0	0	0	0	582	1,019	1,681	2,605	3,777	5,099	6,374	7,648
Compounders	1,935	4,075	3,056	2,369	1,895	1,563	1,329	1,163	1,046	968	920	874	830
Other other	0	0	0	0	473	757	2,789	5,534	9,288	13,337	18,078	23,514	29,747
TOTAL US OBESITY SIZE (mns)	**13,497**	**25,301**	**26,822**	**30,994**	**35,959**	**40,381**	**43,986**	**48,105**	**54,020**	**60,644**	**68,118**	**76,363**	**85,387**

Source: IQVIA, Company Reports, Bernstein analysis and estimates

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US DIABETES MARKET:

SIMILAR DYNAMICS, SIMILAR CONCLUSIONS

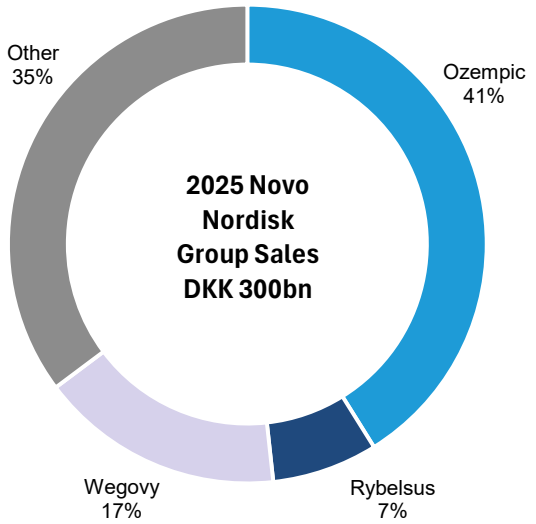
Ozempic is Novo's largest product by global revenue (Exhibit 85) **but we forecast yoy revenue declines even prior to the US semaglutide LOE (2032).** In our forecasts, the cumulative revenues from (global) sales of Wegovy, Oral Sema, CagriSema (in obesity and diabetes) and all other portfolio and pipeline assets will only just compensate falling Ozempic revenues (Exhibit 86).

The US Most Favored Nation (MFN) pricing and TrumpRx deals included Ozempic and Mounjaro (Lilly's GLP-1 for diabetes). Thus, we expect contractions in the net price of a similar magnitude to the obesity market (which you re-review here). Ozempic is now available at around $350 per month on TrumpRx and will be available at $250/month via US Medicare (everyone with Type 2 Diabetes (TD2) >65 years of age eligible) and US Medicaid (all households with income below the Federal Poverty Level eligible) from April 2026.

We do not believe that the cash channel will be as important in diabetes as it is in obesity. However, we expect lower cash channel prices (and the lower medicare and medicaid prices) to have spillover effects into the commercial channel. We expect this to result in a lowering of net US diabetes drug prices. In our detailed bottom up US diabetes model, we forecast -30% US price declines in FY26,-15% in 2027, and -10% across 2028-36. Similar to the obesity market, 10-20% yoy price declines are normal in the diabetes segment in the US.

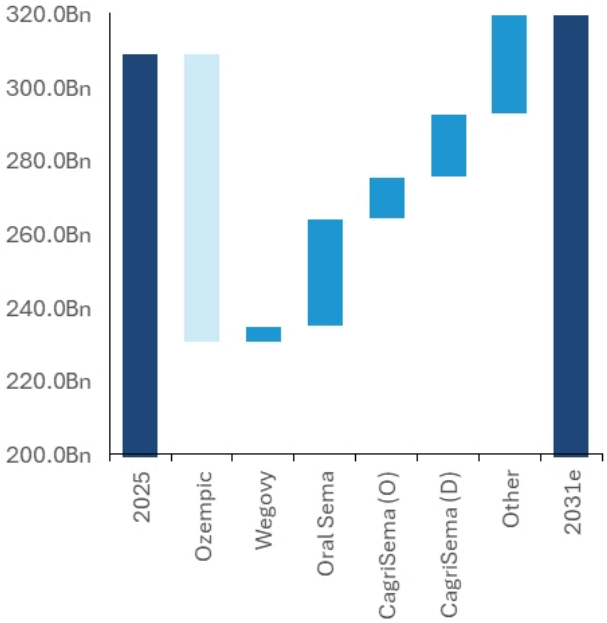
Unlike in Obesity, we don't model an inflection of GLP-1 prescriptions in 2027, despite the steep price cuts in 2026. In our view, the GLP-1 segment of the US diabetes market is much more mature than obesity (we are approaching a decade of Ozempic use in the US; launched in 2017) and is far less geared towards to the "consumer". Put simply, the pool of patients actively seeking weight loss is substantially larger than the pool focused solely on tighter glycemic control. Further, we note that many diabetes patients may wish to use Wegovy (or Zepbound) instead of Ozempic (or Mounjaro) in order to benefit from the weight loss as well as reductions in HbA1c levels (the active molecule in Zepbound and Mounjaro is the same; tirzepatide).

EXHIBIT 85: **Ozempic accounted for 41% of group sales in 2025**



Source: Company Reports, Bernstein Analysis

EXHIBIT 86: **Novo sources of revenue growth 2025-32**



LOE = loss of exclusivity
Source: Company Reports, Bernstein analysis and estimates

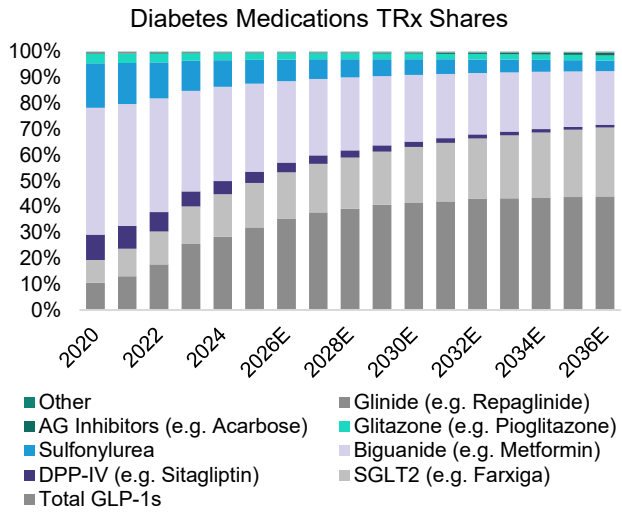
The diabetes market also has multiple established drug classes (e.g. metformin, SGLT2 inhibitors, DPP-4 inhibitors, GLP-1/GIP agonists, insulins), and combination regimens are common as disease progresses. Metformin (been off patent for >20 years in the US), a biguanide, remains first-line for most patients, with early addition of SGLT2 inhibitors (soon to be off-

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patent in the US too) and/or GLP-1 receptor agonists in those with cardiovascular disease, heart failure, chronic kidney disease, or obesity. Other medications, such as DPP-4 inhibitors, sulfonylureas and pioglitazone (which are also all off-patent in the US) are used as alternatives or add-on therapies depending on comorbidities, glycemic control, tolerability and cost.

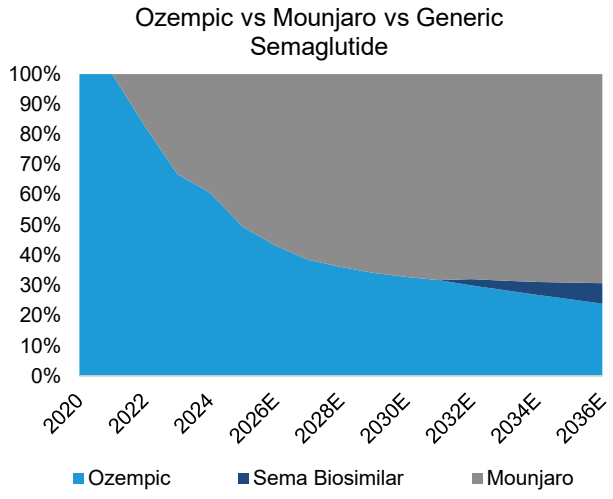
We have witnessed GLP-1s take an increasing share of US prescription volumes in the diabetes market, and we think this will continue, albeit at a slower rate across 2026-36 (Exhibit 87)**.** We expect SGLT-2s and GLP-1s to gain 22% market share (+10% and +12% respectively). Many SGLT-2s will lose patent protection over the next few years (e.g. Farxiga (LOE in 2027); AstraZeneca (O)) and are already priced relatively cheaply. Thus, we forecast an increasing share of diabetes TRx volumes for this class over the next decade in the US. We also forecast that GLP-1 medications will account for 44% of TRx volumes by 2036 (up from 32% in 2025).

EXHIBIT 87: **We think GLP-1s will end up with 40% of diabetes TRx volumes in '36e**



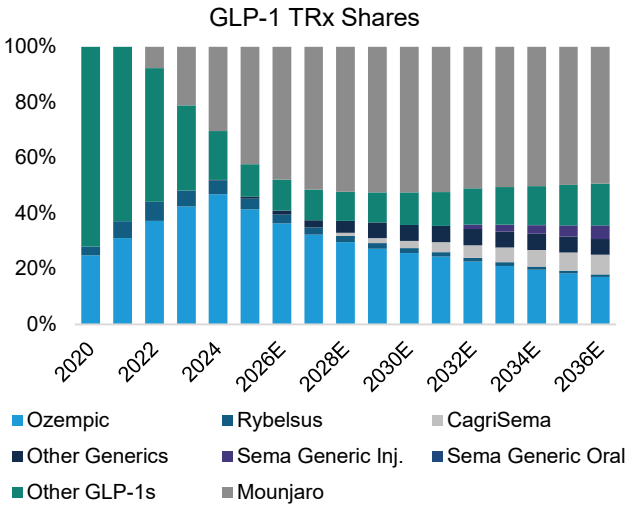
Source: IQVIA, Bernstein analysis and estimates

EXHIBIT 88: **We forecast Ozempic share of TRx to fall further vs Mounjaro**



Source: IQVIA, Bernstein analysis

EXHIBIT 89: **We forecast Ozempic's share of the GLP-1 market to fall from 13% in 2025 to 7% in 2036**



Source: IQVIA, Bernstein analysis and estimates

Within GLP-1s, we think Mounjaro will continue to take share from Ozempic (Exhibit 88)**.** We see a similarly entrenched trend in the US diabetes market as we have described above in the US obesity market. In our view, Mounjaro achieves superior

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Hb1Ac reduction (Exhibit 90) and this has been reflected in the volume share battle seen since 2020 (Exhibit 89). We think it is unlikely that these trends will change significantly over the medium-and-long term.

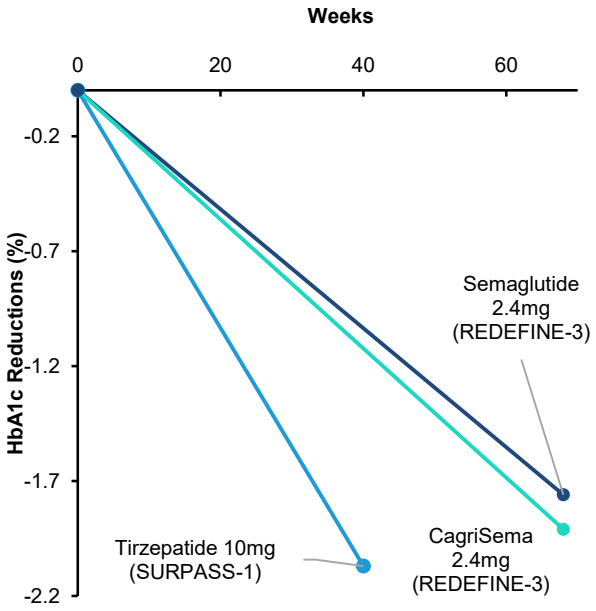
CAGRISEMA: IT WON'T BE ENOUGH

In the wider GLP-1 market, we think CagriSema can help stabilize Novo's share loss in the diabetes market. We understand that Novo will launch CagriSema in diabetes post the launch in obesity (we expect the diabetes launch to be in 2028). In our model, using the same price assumed in our obesity model ($730 - a +100% premium to our assumed blended net price of Ozempic and Mounjaro) and a similar TRx curve to CagriSema in obesity, we forecast CagriSema to have a peak share of 3% of the total US diabetes TRx and 7% of GLP-1 TRx (Exhibit 89).

REDEFINE-3 (read out in February 2026) demonstrated that patients taking CagriSema achieved greater reductions in Hb1Ac than patients using semaglutide. We're not aware of any head-to-head data published vs tirzepatide yet (and our understanding is that REDFINE-4 did not have a comparative Hb1Ac reduction as a secondary endpoint), but an indirect ('meta') analysis between REDEFINE-3 and SURPASS-1 suggests to us that Mounjaro (tirzepatide) has more effective Hb1Ac reduction (Exhibit 90).

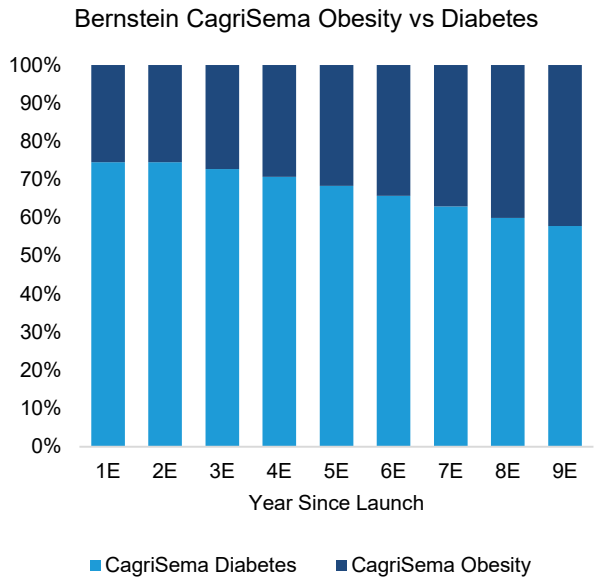
All things considered, we think that CagriSema is better diabetes drug than Ozempic, but Mounjaro will remain the leader. We believe that this is reflected fairly in our 2036 market share forecasts (Exhibit 89). On our estimates, diabetes revenues will comprise 60-70% of "total" CagriSema revenues (taking the sum of both obesity and diabetes) - Exhibit 91.

EXHIBIT 90: **Mounjaro (tirzepatide) is the most effective GLP-1 for Hb1Ac reduction**



Source: Company Reports, NEJM, Bernstein Analysis

EXHIBIT 91: **We think diabetes will be 60-70% of total CagriSema sales on a like-for-like basis**



Source: Bernstein analysis and estimates

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EXHIBIT 92: **Bernstein Diabetes Model Summary Sheet**

US Diabetes model	2020	2021	2022	2023	2024	2025	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E	CAGR 26-31	CAGR 31-36
US prescription volumes																			
Sulfonylurea	28,373,580	26,954,611	25,325,218	23,373,586	22,092,004	20,964,583	19,916,354	18,920,536	17,974,509	17,075,784	16,221,995	15,410,895	14,640,350	13,908,333	13,212,916	12,552,270	11,924,657	-5%	-5%
Biguanide (e.g. Metformin)	80,792,182	79,946,792	80,033,799	78,797,190	78,575,563	77,242,650	75,119,179	73,056,485	71,595,356	70,163,448	68,760,180	67,384,976	66,037,276	64,716,531	63,422,200	62,153,756	60,910,681	-2%	-2%
Glitazone (e.g. Pioglitazone)	6,052,521	6,167,897	6,179,359	6,000,008	5,922,371	5,900,989	5,841,979	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	5,783,559	0%	0%
AG Inhibitors (e.g. Acarbose)	354,923	342,676	326,831	316,152	317,695	322,170	442,357	556,736	672,853	813,189	982,794	1,187,773	1,435,505	1,734,905	2,096,750	2,534,065	3,062,590	22%	21%
Glinide (e.g. Repaglinide)	855,699	816,914	779,746	744,941	741,751	757,859	773,016	788,477	804,246	820,331	836,738	853,472	870,542	887,953	905,712	923,826	942,302	2%	2%
DPP-IV (e.g. Sitagliptin)	16,141,516	15,003,417	13,551,445	11,906,091	10,951,810	9,756,282	8,780,654	7,902,588	7,112,330	6,401,097	5,760,987	5,184,888	4,666,399	4,199,759	3,779,784	3,401,805	3,061,625	-10%	-10%
SGLT2 (e.g. Farxiga)	14,586,719	17,956,035	23,114,337	29,283,170	35,427,925	39,104,580	43,015,038	46,456,241	50,172,740	53,935,696	57,711,195	61,462,422	65,150,168	68,733,427	72,170,098	75,417,753	78,434,463	7%	5%
Total GLP-1s	17,414,850	22,222,381	32,169,372	51,738,206	60,961,059	72,433,202	83,869,001	93,508,948	99,912,339	106,419,906	110,737,180	114,186,299	119,575,290	121,838,344	124,059,910	126,439,495	128,919,412	6%	2%
Semaglutide	**4,883,081**	**8,255,442**	**14,212,827**	**24,976,573**	**31,613,640**	**32,817,291**	**33,150,519**	**32,639,237**	**32,997,622**	**33,096,715**	**33,360,458**	**33,844,503**	**35,960,594**	**36,659,351**	**37,272,420**	**37,876,106**	**38,710,998**	0%	3%
Ozempic	**4,333,741**	**6,905,349**	**12,012,578**	**21,999,298**	**28,543,745**	**30,008,604**	**30,526,489**	**30,194,826**	**29,590,929**	**28,999,111**	**28,419,128**	**27,850,746**	**27,015,223**	**25,664,462**	**24,381,239**	**23,162,177**	**22,004,068**	-2%	-5%
Sema Generic Inj.	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**1,830,048**	**2,836,574**	**3,829,375**	**4,978,188**	**6,222,735**		
Rybelsus	**549,340**	**1,350,093**	**2,200,249**	**2,977,275**	**3,069,895**	**2,808,687**	**2,624,030**	**2,444,411**	**2,273,302**	**2,114,171**	**1,966,179**	**1,828,547**	**1,700,548**	**1,581,510**	**1,470,804**	**1,367,848**	**1,272,099**	-7%	-7%
Sema Generic Oral	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**79,075**	**118,613**	**148,267**	**170,507**		
CagriSema	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**0**	**1,133,391**	**1,983,434**	**2,975,151**	**4,165,211**	**5,414,774**	**6,497,729**	**7,472,388**	**8,219,627**	**9,041,590**		17%
Mounjaro	0	0	2,482,961	10,936,988	18,513,183	30,666,970	40,126,415	48,113,138	52,177,911	55,830,365	58,063,579	59,805,486	61,001,596	61,611,612	62,227,728	62,850,006	63,478,506	8%	1%
Other GLP-1s	12,531,769	13,966,939	15,473,584	15,824,645	10,834,236	8,948,941	10,592,067	12,756,573	13,603,415	15,509,392	16,337,992	16,371,099	17,198,326	17,069,653	17,087,374	17,493,756	17,688,320	9%	2%
Other	33,481	26,885	22,333	17,936	14,655	9,674	7,739	6,191	4,953	3,962	3,170	2,536	2,029	1,623	1,298	1,039	831	-20%	-20%
Total	164,605,471	169,437,608	181,502,440	202,177,280	215,004,833	226,491,989	237,765,316	246,979,762	254,032,885	261,416,972	266,797,796	271,456,821	278,161,118	281,804,434	285,432,228	289,207,568	293,040,120	3%	2%
Market Shares (TRx)																			
Sulfonylurea	17%	16%	14%	12%	10%	9%	8%	8%	7%	7%	6%	6%	5%	5%	5%	4%	4%		
Biguanide (e.g. Metformin)	49%	47%	44%	39%	37%	34%	32%	30%	28%	27%	26%	25%	24%	23%	22%	21%	21%		
Glitazone (e.g. Pioglitazone)	4%	4%	3%	3%	3%	3%	2%	2%	2%	2%	2%	2%	2%	2%	2%	2%	2%		
AG Inhibitors (e.g. Acarbose)	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	1%	1%	1%	1%	1%		
Glinide (e.g. Repaglinide)	1%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%		
DPP-IV (e.g. Sitagliptin)	10%	9%	7%	6%	5%	4%	4%	3%	3%	2%	2%	2%	2%	1%	1%	1%	1%		
SGLT2 (e.g. Farxiga)	9%	11%	13%	14%	16%	17%	18%	19%	20%	21%	22%	23%	23%	24%	25%	26%	27%		
Total GLP-1s	11%	13%	18%	26%	28%	32%	35%	38%	39%	41%	42%	42%	43%	43%	43%	44%	44%		
Semaglutide	**3%**	**5%**	**8%**	**12%**	**15%**	**14%**	**14%**	**13%**	**13%**	**13%**	**13%**	**12%**	**13%**	**13%**	**13%**	**13%**	**13%**		
Ozempic	**3%**	**4%**	**7%**	**11%**	**13%**	**13%**	**13%**	**12%**	**12%**	**11%**	**11%**	**10%**	**10%**	**9%**	**9%**	**8%**	**8%**		
Sema Generic Inj.	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**1%**	**1%**	**1%**	**2%**	**2%**		
Rybelsus	**0%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**1%**	**0%**	**0%**		
Sema Generic Oral	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**		
CagriSema	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**0%**	**1%**	**1%**	**2%**	**2%**	**2%**	**3%**	**3%**	**3%**		
Mounjaro	0%	0%	1%	5%	9%	14%	17%	19%	21%	21%	22%	22%	22%	22%	22%	22%	22%		
Other GLP-1s	8%	8%	9%	8%	5%	4%	4%	5%	5%	6%	6%	6%	6%	6%	6%	6%	6%		
Other	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%	0%		
Total	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%	100%		
Prices ($; blended net price)																			
Ozempic	588	533	456	416	428	446	312	265	239	215	193	174	157	141	127	114	103	-11%	-10%
Rybelsus	508	500	515	539	510	475	333	283	255	229	206	186	167	150	135	122	110	-11%	-10%
Mounjaro	0	0	148	442	483	445	312	265	238	215	193	174	156	141	127	114	103	-11%	-10%
CagriSema	0	0	0	0	0	0	0	0	730	657	591	532	479	431	388	349	314		-10%
US Sales ($mn)																			
Ozempic	2,546	3,683	5,476	9,144	12,210	13,373	9,534	8,021	7,062	6,229	5,494	4,845	4,230	3,617	3,092	2,644	2,260	-13%	-14%
Rybelsus	279	674	1,132	1,605	1,565	1,335	875	694	579	484	405	339	284	238	199	167	139	-17%	-16%
Mounjaro	0	0	367	4,834	8,950	13,651	12,500	12,737	12,438	11,977	11,211	10,392	9,540	8,672	7,883	7,166	6,513	-4%	-9%
CagriSema	0	0	0	0	0	0	0	0	827	1,303	1,759	2,217	2,593	2,801	2,899	2,870	2,841		5%

Source: IQVIA, Company Reports, Bernstein analysis and estimates

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APPENDIX 1: RARE DISEASE

MASH/EFRUXIFERMIN

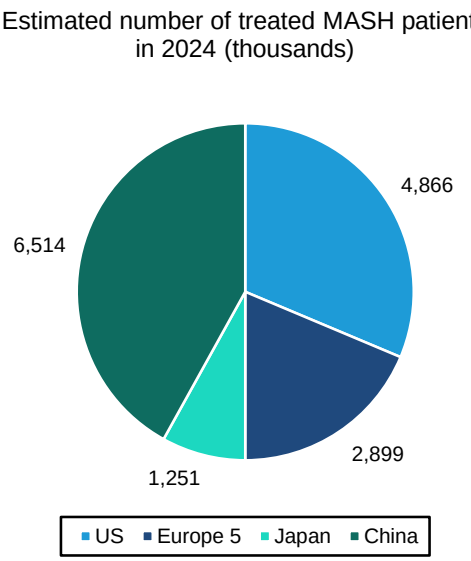
In 3Q25, Novo Nordisk confirmed its acquisition of Akero therapeutics for ~$5.2Bn in cash and contingent value rights to gain access to its lead asset efruxifermin. The drug is a bivalent fibroblast growth factor 21 (FGF21) and beta-klotho receptor analogue used in the treatment of Metabolic-dysfunction Associated Steato-Hepatitis (MASH). MASH is a disease of the liver with a high unmet clinical need, and we believe efruxifermin will be a best-in class drug.

What is MASH?

Metabolic-dysfunction Associated Steato-Hepatitis (MASH), previously known as Non-Alcoholic Steato-Hepatitis (NASH), is a disease process involving the build up of excess fatty tissues within the liver. The fat is deposited through a variety of biological processes, and can irritate healthy liver cells and cause inflammation and scarring to build up.

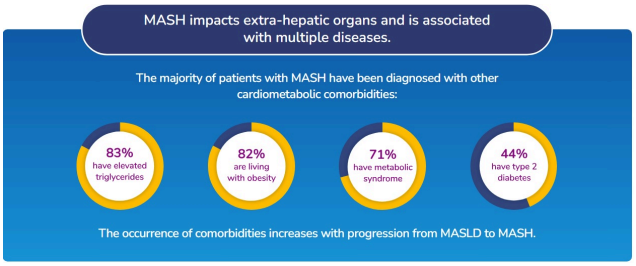
Scarring in the liver is more broadly known as fibrosis, or cirrhosis in severe cases which can significantly increase the risk of liver cancer. MASH can cause fibrosis, and although the two are strongly linked are strictly speaking separate. Hence, one can exist without the other.

EXHIBIT 93: **MASH epidemiology**



Source: AstraZeneca, Bernstein Analysis

EXHIBIT 94: **MASH co-morbidities**

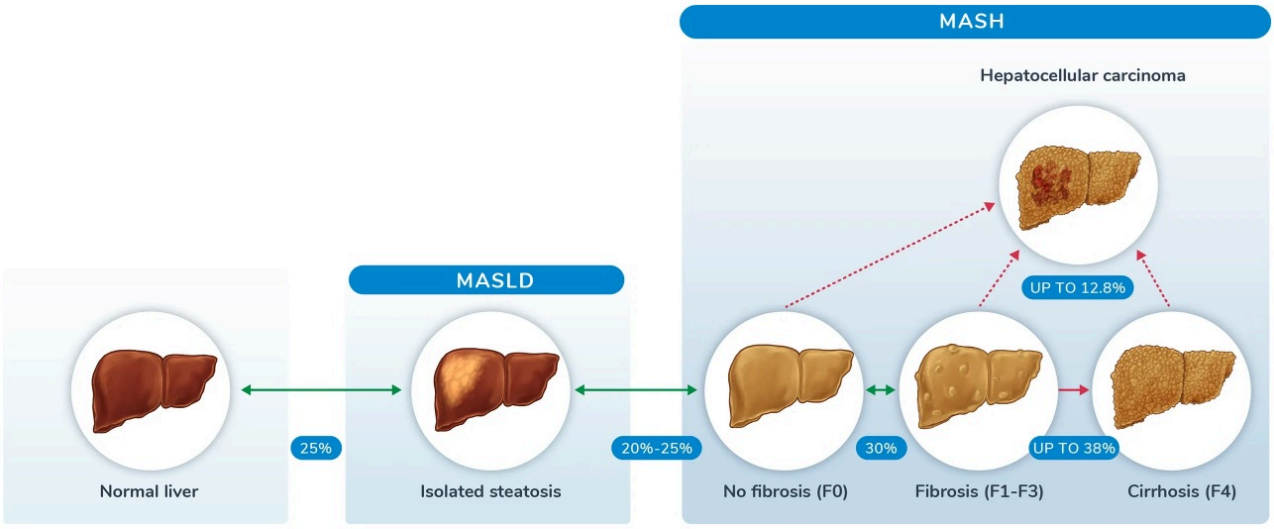


Source: 89Bio

The exact cause of MASH is unknown, and a multitude of metabolic factors are thought to contribute such as obesity, diabetes and high cholesterol levels. Although these other health problems do not necessarily lead to MASH, they are ubiquitous in MASH patients.

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EXHIBIT 95: **Stages of MASH and associated risk of hepatocellular carcinoma (liver cancer)**



MASLD= metabolic-dysfunction associated steatotic liver disease - a milder version of MASH, F= A commonly used staging category for the severity of fibrosis going from F1 (light) to F4 (severe), diagnosed either on ultrasound or biopsy
Source: 89bio

In the US alone, approximately 25% of the population will develop metabolic-dysfunction associated steatotic liver disease (MASLD), a milder version of MASH in their lifetime, with 20-25% of those patients progressing to MASH (Exhibit 95). The prevalence of MASH in the US is predicted to increase 63% from 16.5 million cases in 2015 to 27.0 million cases in 2030, with similar rates across Europe.

MASH can often be silent with no symptoms, hence the discrepancy between the prevalence of the disease and the number of patients actually diagnosed and treated.

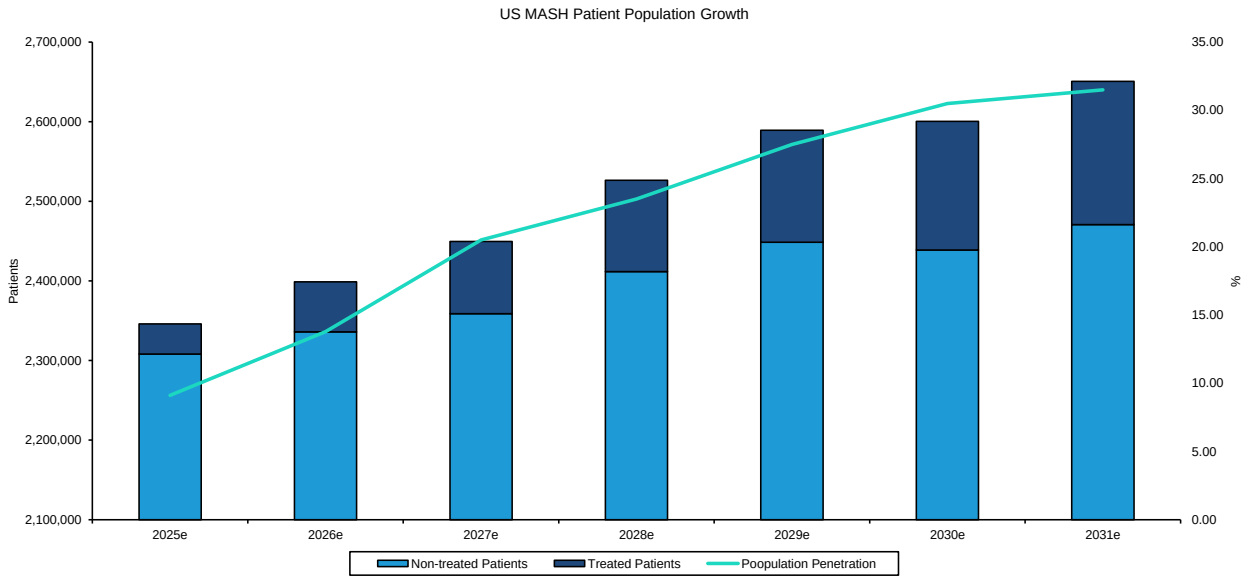
Those who do have symptoms suffer with tiredness, general weakness, some discomfort in the tummy and occasionally some itching and bruising. Although by itself it is rarely a debilitating disease, MASH is a stepping stone to more severe scarring leading to cancer and death. As such, it is considered a serious health problem, and the second leading driver of liver transplantation worldwide.

Disease burden and treatment paradigm

Given MASH often forms part of a constellation of health problems, estimating its economic impact can be difficult. Nevertheless, conservative estimates place the medical costs of MASH at $78.6Bn by 2040 in the US alone, whilst MASH associated productivity loss will be close to $246Bn during same timescale. Per patient medical costs in the US are expected to be $3,601 by 2040, whilst individual patient productivity loss annually is expected to top $11,300 in the same year. Epidemiological research by Madrigal Therapeutics (NR; a leader in MASH), demonstrates that in addition to a growing MASH patient population, the percentage of patients being treated will also grow substantially in the medium to long term as awareness of the disease increases.

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EXHIBIT 96: **US MASH patient growth & treatment penetration**



Source: Madrigal therapeutics, Bernstein Analysis

To diagnose MASH, physicians may use bloodwork to examine liver function tests, ultrasound scans, and (rarely) a biopsy (removal of a sample via surgery) of the liver are conducted to confirm the disease pathology. With progress in ultrasound technology and protocols, the need for biopsy has reduced significantly and clinicians are growing increasingly confident in treating the disease in the absence of a confirmed tissue sample.

In terms of treatment, a healthy diet and physical activity is often the first line to minimize circulating fats in the bloodstream, and mitigate the metabolic risk factors contributing to MASH.

Bariatric surgery (forced stomach shrinkage by decreasing the diameter of the gastric tract) to treat obesity is evidenced to be up to 90% effective in treating MASH. Meanwhile, liver transplantation can be offered in cases of MASH where liver function is most severely restricted.

Medical (i.e. drug) treatment usually starts with cholesterol lowering medication and good diabetic control. Although medical treatment specific to MASH is uncommon with only a few approved agents licensed for the disease.

Current therapeutic landscape

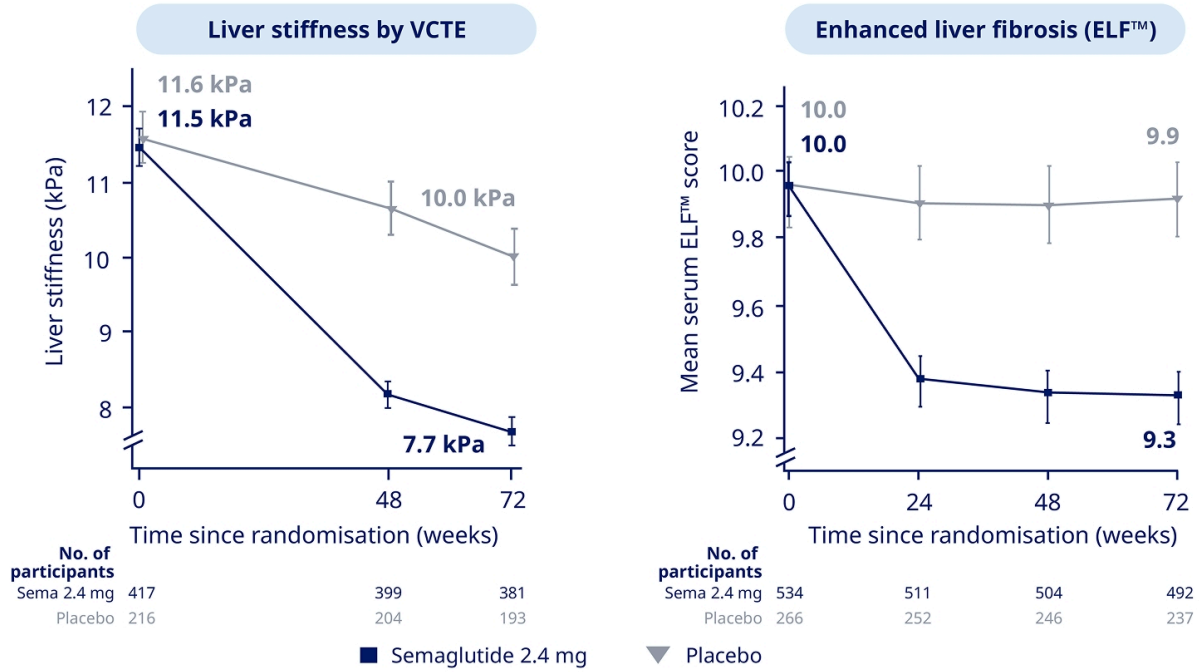
Rezdiffra (Madrigal therapeutics; unrated) is a drug that mimics the thyroid hormone to promote fat breakdown and dissolution. More specifically, Rezdiffra binds to the thyroid hormone receptor beta (THR-beta) in liver fat to promote its breakdown.

In the phase III MAESTRO-NASH trial program, Rezdiffra was compared to placebo to assess fibrosis and MASH on ultrasound imaging. 25.9% of patients achieved at least a 1 stage improvement in their F score without worsening of MASH compared to 14.2% in the placebo group ($p<0.001$). No concerning safety signals were reported and the drug was approved as the first ever licensed treatment exclusively for MASH in 1Q2024. Following sales of $180m in 2024, consensus (Bloomberg) projects Rezdiffra peak sales of $5Bn in 2031 for a CAGR (25-31e) of 61%.

The second, and only other medication with an indication in MASH is semaglutide, known as Wegovy (Novo Nordisk; O), an analogue of the glucagon like peptide 1 (GLP-1) receptor. This drug acts on the hormone pathway that promotes appetite suppression, lowers blood sugar levels and improves control of body weight. Originally indicated for tackling obesity, the phase III ESSENCE study trialed Wegovy against placebo in MASH patients, evaluating whether there was improvement in liver stiffness and fibrosis reflected on ultrasound imaging and bloodwork.

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EXHIBIT 97: **Results of phase3 ESSENCE trial (Wegovy in MASH)**



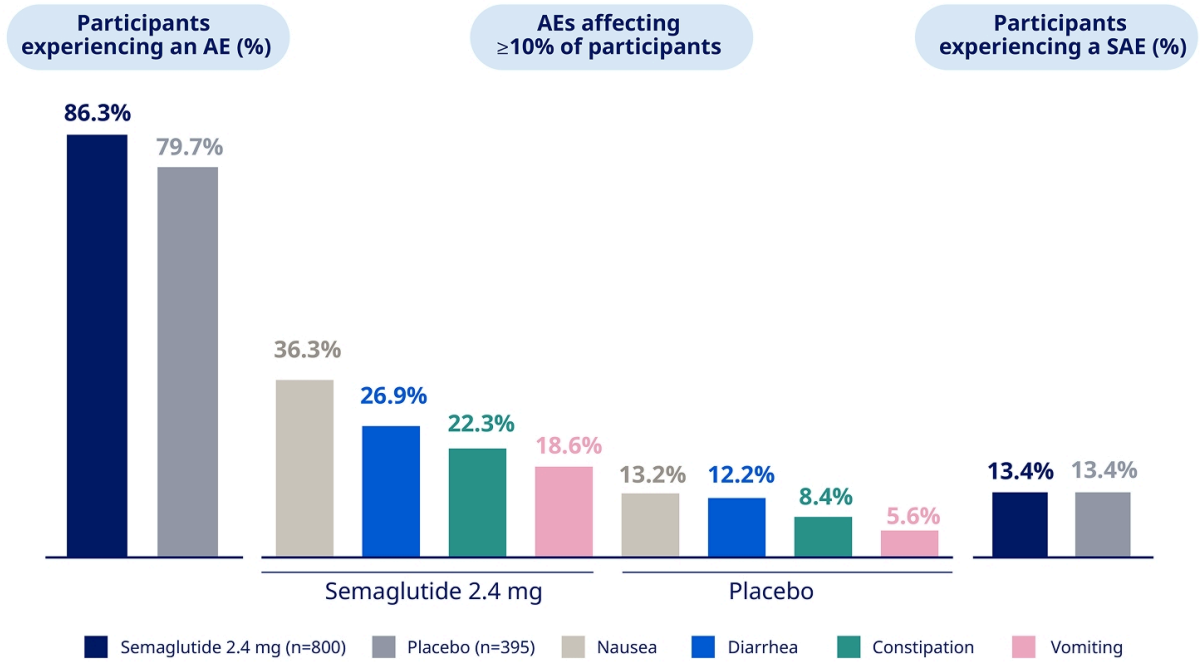
VCTE = Vibration controlled transient elastography - a liver ultrasound technique, ELF = enhanced liver fibrosis score, a combination blood test score for liver scarring
Source: Novo Nordisk

A reduction in liver fibrosis without worsening of MASH was reported in 36.8% of patients taking Wegovy compared to 22.4% in the placebo group ($p<0.001$; Exhibit 97). This was a clinically meaningful result that led to Wegovy's FDA approval for MASH in August 2025. Although Wegovy commanded sales of DKK 58.2Bn in FY24, sales specific to the MASH indication are forecast by Bernstein to peak at DKK 5Bn in 2032e after which biosimilar ('generic') competition is expected.

In our view the adverse event profile of Wegovy in ESSENCE mirrors that of this drug's use in obesity, i.e highly favorable with no concerning safety signal (Exhibit 98).

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EXHIBIT 98: **Semaglutide adverse event profile in ESSENCE trial**



Source: Novo Nordisk

In our view MASH has a high economic and quality of life cost to patients and society. This is because we believe that this disease is not adequately managed by current treatments. MASH is the second leading cause for liver transplants worldwide, setting up a potential future bottleneck (as there simply aren't enough liver donors). As such, we believe that the MASH pharmacotherapeutic space has significant unmet clinical need, high commercial potential and there is a clamor for new entrants to the market.

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Spotlight on efruxifermin

Efruxifermin is a member of a new class of MASH medications that mimic the activity of fibroblast growth factor 21 (FGF21 . FGF21 is a naturally occurring hormone that is often described as a "master metabolic regulator" (Exhibit 99) which controls and reduces fat accumulation throughout the body and thus comprises healthy metabolism. This drug is administered by subcutaneous (under the skin) injection on a weekly basis.

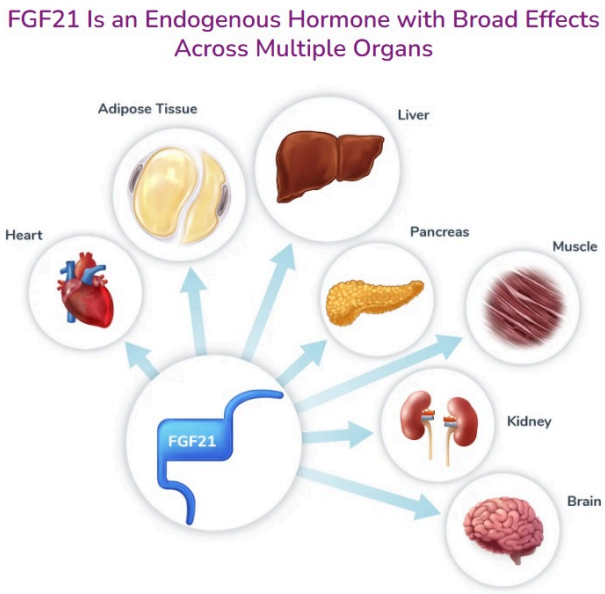
By providing an analogue that is longer acting than the hormone present in the body, efruxifermin prolongs and amplifies the positive effects of normally present FGF21. Some of these desirable effects include healing of the scar tissue in the liver, and slowing down the MASH disease process.

With MASH induced fibrosis in particular, FGF21 has demonstrated impressive potential in vitro (in a test tube). As such, we believe this platform is of particular interest in next generation MASH therapy, and we believe that it could compete within the sparsely populated treatment landscape for MASH.

In the phase IIb HARMONY study, efruxifermin reduced fibrosis by >1 F stage in at least 75% of patients compared to 24% in placebo ($p<0.001$). No concerning safety signals were reported and the adverse event profile was favorable in our view.

This primary endpoint in HARMONY was measured on liver histology at 96 weeks and given the statistically significant positive results, efruxifermin was in our view a clinically meaningful treatment that will affect the standard of care. Efruxifermin is currently in three different phase III trials (SYNCHRONY program), evaluating the same endpoints as in phase II but in more severe MASH patients and those with cirrhosis already established. The first phase 3 trial in this program is expected to read out in 1H26.

EXHIBIT 99: **Fibroblast growth factor 21 - master metabolic regulator**

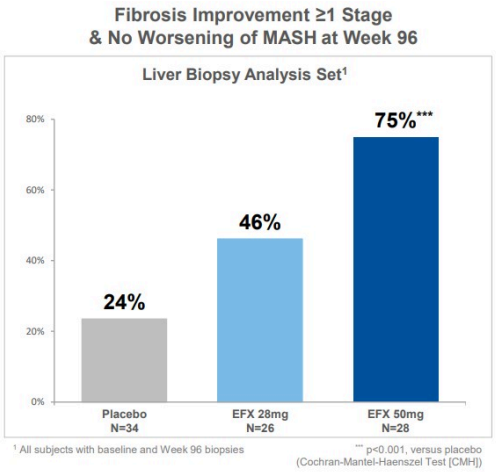


Source: 89Bio

In addition to the FGF21 receptor, efruxifermin is unique in this competitive landscape as it also partially stimulated the beta-klotho receptor. This receptor has an even broader role across metabolic cellular processes than FGF21, and in combination with FGF21, we believe efruxifermin is a unique and effective therapeutic approach. No concerning safety signals have been reported thus far.

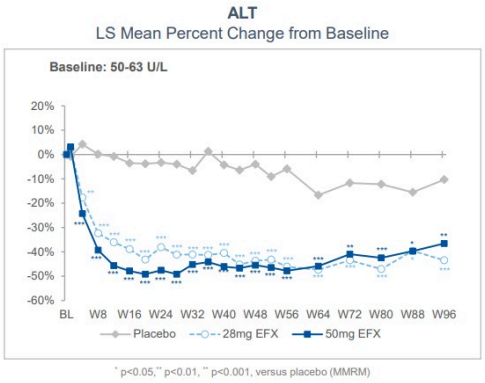
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EXHIBIT 100: **HARMONY Phase II trial data**



EFX = efruxifermin
Source: Akero therapeutics

EXHIBIT 101: **HARMONY phase II blood test data**



ALT= Alanine-amino transferase, a highly selective blood biomarker of all-cause liver damage
Source: Akero

Competitive landscape

In this exciting time for entrants into the MASH therapy landscape, several other assets are also in phase III which, in our view, have demonstrated results in phase II that are similar to efruxifermin.

In September 2025 Roche announced the acquisition of 89bio (unrated) for $3.5Bn including contingent value rights for 89bio's lead asset **pegozafermin**, an FGF21 analogue designed to treat fatty diseases of the liver, in particular MASH. This drug's phase III trials are set to read out in 1H28, and Roche is guiding to >3Bn CHF in nominal peak year sales, its highest guidance bucket.

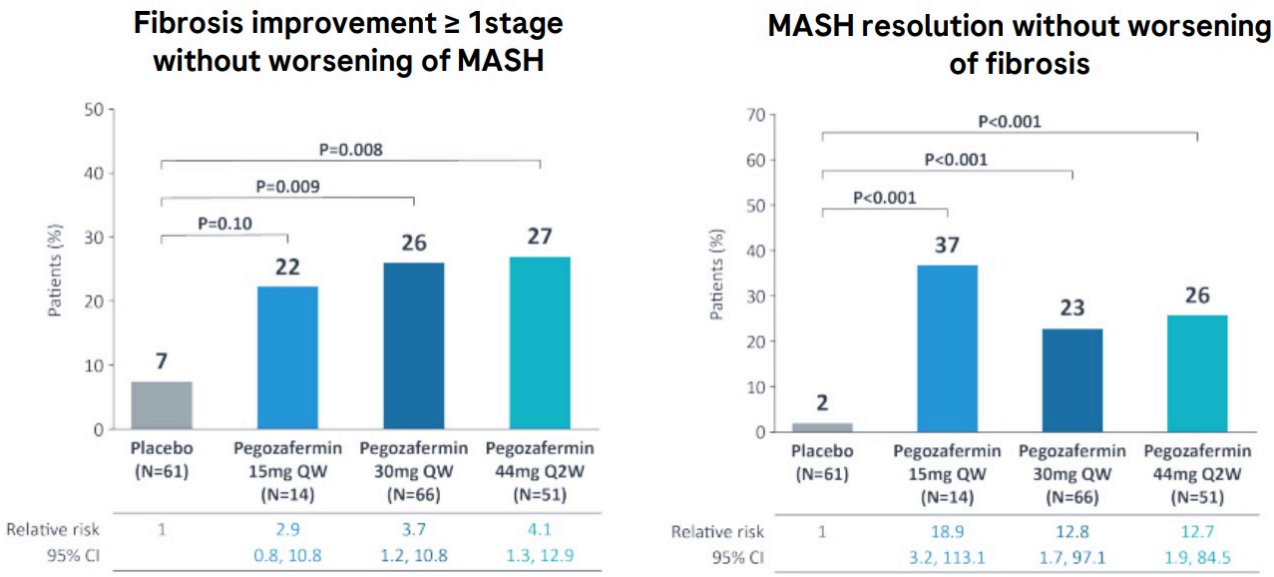
In phase II thus far, the ENLIVEN trials tested three different dosages of pegozafermin (15,30 and 44mg) against placebo, reporting results in 2023. The trial evaluated as its primary endpoint an improvement in F stage on liver ultrasound as well as biopsy both at baseline and at the conclusion of the trial (Exhibit 102).

At 24 weeks, a statistically significant increase in the odds of improving fibrosis by at least one F stage at the intermediate pegozafermin dose of 30mg weekly (Relative risk 3.7, 1.2-10.8 p<0.009) was reported.

At the lowest dose of15mg weekly, the chance of resolving MASH completely and without worsening of fibrosis significantly increased (RR 18.9, 3.2-113.1 p <0.001).

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EXHIBIT 102: **Phase II ENLIVEN week 24 results in patients with MASH and fibrosis**



Source: 89Bio

In addition to the imaging measures of success already reported, the blood levels of alanine amino transferase (ALT) - a surrogate for liver damage - also saw sustained declines through to the end of the study. In the most severe F4 patients with borderline cirrhosis, nearly all experienced a significant improvement in their F scores which brought their livers back from the brink of cancer to a more healthy state.

With respect to adverse events, the profile of pegozafermin was generally favorable in ENLIVEN and no concerning safety signals were reported. The majority of events centered around diarrhea and nausea, and there were almost no events severe enough to warrant stoppage of treatment (p value not reported).

In phase III, 89bio is running the ENLIGHTEN fibrosis and ENLIGHTEN cirrhosis trials which are both set to read out in 2H28. The fibrosis trial is similar to the phase II trial, with an improvement of F stage as its primary endpoint. In addition, ENLIGHTEN fibrosis will add a greater number of biopsies to its protocol over multiple years, to support its ultrasound based conclusions.

The pegozafermin cirrhosis phase III study has similar intervention arms and timescales, except that the primary endpoint is now an improvement in cirrhosis on biopsy at 24months and 60 months. We believe that Pegozafermin will be the main competitor to efruxifermin in the long run, with efruxifermin benefitting from an earlier read out date and potential first-in-class advantage.

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EXHIBIT 103: **Summary of phase III MASH pipeline**

Drug	Mechanism of Action	Manufacturer	Status	Significant Result
Current Treatments				
Rezdiffra	THR-Beta agonist	Madrigal	Approved	25.9% of patients with >1 stage of fibrosis improvement without worsening of MASH
Semaglutide	GLP-1 Analogue	Novo Nordisk	Approved	36.8% of patients with >1 stage of fibrosis improvement without worsening of MASH
Phase III pipeline				
Pegozafermin	FGF21 Analogue	Roche	Phase III	Fibrosis improvement >1 stage increased in intervention arm(RR 3.7)
Efruxifermin	FGF21/ Beta Klotho Analogue	Novo Nordisk	Phase III	Fibrosis improvement >1 stage in 75% of patients compared to 24% in placebo
Efimsofermin Alfa	FGF21 Analogue	GSK	Phase III	89% of patients reduced liver fat by 30% in imagery compared to 7% in placebo
Survodutide	GLP1/Glucagon analogue	Boehringer Ingellheim	Phase III	Resolution of MASH with no worsening of fibrosis in 62% of patients compared to 14% in placebo

Source: Clinical trials.gov, Bernstein analysis

GSK (O) recently announced its purchase of efimosfermin alfa, another long acting FGF21 analogue medication invented by Boston Pharmaceuticals. In phase II, the drug demonstrated at least a 30% reduction in an ultrasound measure of liver fat in 89% of patients, compared to just 7% in the placebo group. This effect was dose dependent, with up to 100% reduction in the highest dose group.

The tolerability profile was favorable, with 66% of patients experiencing mild treatment related adverse events compared to 49% in the placebo group. In Phase III, theZENITH 1 and 2 trials are testing two dosage levels of the drug on patients with biopsy confirmed MASH to evaluate improvement in F stage and number of adverse events as their primary endpoints. These trials will each last 52 weeks, are currently recruiting and estimated readouts will be in 2H28.

Survodutide (Boehringer Ingelheim (private company)) is a dual GLP1 and glucagon analogue. (Glucagon is the hormone which raises blood sugar when levels fall too low, promoting the breakdown of stored sugar (glycogen) in the liver. Designed with a focus on MASH, phase II evidence for this drug demonstrated resolution of MASH with no worsening of fibrosis stage in 62% patients compared to 14% in the placebo group ($p<0.001$). No concerning safety signals were reported with adverse events at only 8% in the intervention arms compared to 7% in placebo.

Two phase III trials are ongoing through the LIVERAGE program, with primary endpoints of all cause liver dysfunction and mortality at 76 weeks post treatment. These trials are set to read out in 1H29.

Tirzepatide (Eli Lilly; Breen; O), is a dual agonist of the GLP-1 and gastric inhibitory polypeptide (GIP) receptors and has been explored for MASH. In phase II, the SYNERGY-NASH trial found that up to 62.4% ($p<0.001$) of patients experienced resolution of MASH without worsening of F stage at 52 weeks post treatment on biopsy. Having met its primary end-point, with no safety signals reported, the drug awaits registration into phase III trials. (This drug is already FDA approved for use in diabetes (brand

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name = Mounjaro) and obesity ((brand name = Zepbound)).

MASH has for a long time been a graveyard for pharmaceutical R&D. The previous need for biopsy left a diagnostic bottleneck that has now been resolved with advances in ultrasound in our view. Given the overall increase in metabolic disorders in recent decades, these factors have created the environment for a sustained increase in the number of MASH patients which, in our view, shows no sign of slowing.

In our view, evaluating the extent of the economic and clinical unmet need in MASH specifically, as opposed to its comorbid conditions is difficult. Nonetheless, we believe that this unmet need is material. In our view, FGF21 is an evident next step in the treatment paradigm for MASH, and we believe that efruxifermin may be a potential best in class medication.

APPENDIX 2: A GENERALIST'S GUIDE TO GLP-1S: WITH A ZOOM IN ON SEMAGLUTIDE

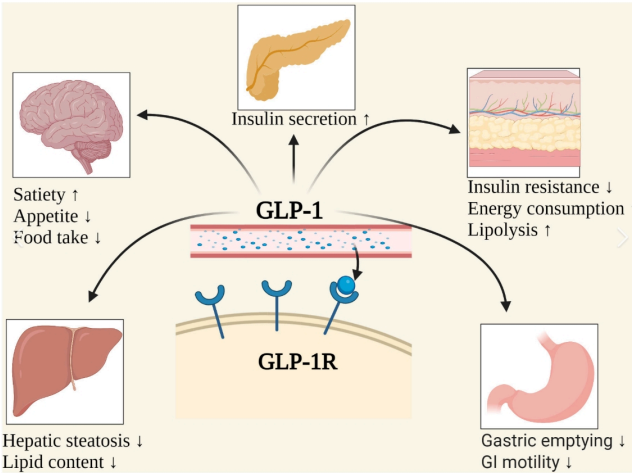
The Main Mechanisms of Action

Obesity drugs, in general, target our "hunger-related" hormones. As the body's digestive system (and it's links with the brain) are extremely complicated, there are numerous hormones needed for the body to function. A drug (such as semaglutide; active ingredient in Wegovy) may only target a single hormone (and is termed a "single agonist") others (such as tirzepatide, active ingredient in Zepbound) target two hormones ("double agonist"). Below we list the main mechanisms of actions:

- **GLP-1 receptor agonist.** Mimics the Glucagon-Like Peptide 1 (a hormone) by inhibiting the release of glucagon (hormone raising blood sugar levels) and stimulating the production of insulin (a hormone that lowers blood sugar levels) thus reducing hyperglycemia which leads to lower blood sugar levels. Furthermore, GLP-1s reduce hunger and slow the GI (gastro intestinal) food flow leading to a decrease in appetite.
- **Amylin receptor agonist.** Mimics amylin (hormone secreted with insulin) which inhibits glucagon secretion and affects gastric emptying plus food intake.
- **GIP receptor agonist.** Mimics the Glucose-dependant Insulinotropic Peptide (hormone) which stimulates insulin production and sensitivity leading to a better insulin penetration in the bloodstream, thus lowering blood sugar levels. The hormone also reduces stomach acid production, leading to a slow GI food flow resulting in a decrease in appetite.
- **Glucagon receptor agonist.** Mimics glucagon which increases blood glucose levels and breaks down lipids and amino acids. Exogenous intake of glucagon leads to increased energy expenditure with a lower food intake.
- **CB1 receptor inverse agonist.** The peripheral cannabinoid receptor 1 modulates appetite and energy homeostasis but also metabolic inflammation induced by insulin resistance.

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EXHIBIT 104: **GLP-1 effects on the body...**



Source: Wikimedia Commons

Side Effects and Tolerance

The most common side effects from GLP-1s are gastro-intestinal (GI) problems. The primary mechanism of action of GLP-1s is to alter the way our "hunger hormones" behave and interact with each other. It is, therefore, unsurprising that gut-related issues are the primary side effect from taking GLP-1s.

- **Nausea, vomiting, constipation, bloating, and digestive reflux are all common GI adverse events.** Especially towards the start of treatment, where the body's digestive process is undergoing a "re-wiring", side effects may manifest themselves. For most people, the side effects are manageable (or tolerated for the benefit of the weight loss).

- **Fatigue, headaches, dizziness, and taste changes can also be caused by GLP-1 usage.** As the body readjusts to a new level of caloric intake, the body undergoes a "metabolic stress test". These side effects can be prominent when people are adjusting to new dosing levels, and can exacerbated by dehydration (GLP-1s also suppress the "thirst hormones").

Whilst we think the tolerance profile of a drug is important, the main driver behind the success of an anti-obesity product remains headline the weight loss. Patients are willing to accept a worse tolerance profile in order to achieve larger amounts of weight loss. Most drugs (including those in the pipeline) have a similar discontinuation due to adverse events rates - certainly not different enough to shape patient behavior. We think the weight loss-tolerance trade off will become more important as more drugs enter the market, but weight-loss will remain the driving force.

Semaglutide: A Deep(er) Dive

Semaglutide is a long-acting glucagon-like peptide 1 receptor agonist (GLP-1 RA). Glucagon-like peptide-1 is a naturally occurring hormone that helps the body regulate blood sugar (glucose) levels, appetite, and the speed at which nutrients are moved from the stomach into the bloodstream.

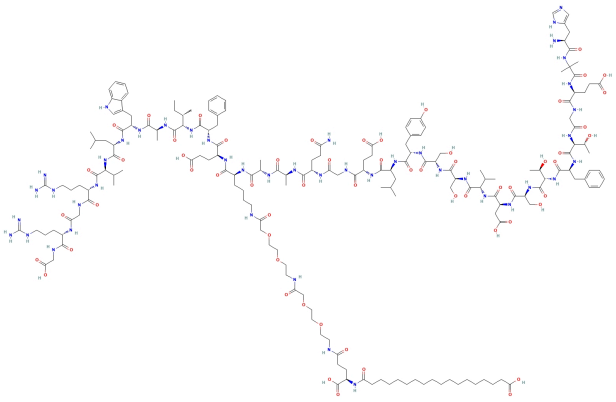
Semaglutide is a modified version of the naturally occurring hormone. The GLP-1 drugs increase the length of time it lasts in the body and activates the GLP-1 receptors (starting signals) more steadily. **Semaglutide has three primary effects on the body:**

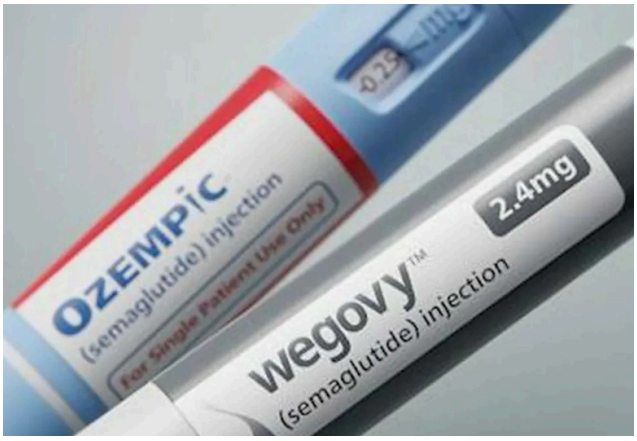
1. **Pancreas:** Increases the amount of insulin secreted when glucose is high (normally just after eating) but not when glucose levels are low (lowering the risk of hypoglycemic - crashes in blood sugar levels - attacks).

2. **Stomach:** Slows gastric emptying (slows the rate that nutrients enter the bloodstream) which "smooths" out the body's glucose response. This lowers strain on the pancreas (reducing the risk of developing diabetes in the first instance, as well as slowing the pancreatic strain in diabetes patients) and helps patients feel "fuller" for longer.

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3. **Brain:** Increases the body's "fullness" signal and reduces the "hungry" signals. This means that patients feel they need to eat less (both in volume and frequency).





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Ticker	Rating	Cur	16 Mar 2026 Closing Price	Price Target	TTM Rel. Perf.	Adjusted EPS Cur	Adjusted EPS 2025A	Adjusted EPS 2026E	Adjusted EPS 2027E	Adjusted P/E (x) 2025A	Adjusted P/E (x) 2026E	Adjusted P/E (x) 2027E
NOVOB.DC (Novo Nordisk)	U	DKK	248.30	175.00	(62.9)%	DKK	23.02	17.51	18.83	10.8	14.2	13.2
EDME			1,496.09									

COVERAGE INITIATION

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

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Novo Nordisk A/S

Our price target (DKK175) is the average of an EV/EBITA-multiples based valuation (applying a 36% discount to 2026-28e peer group) and DCF (WACC 8.0%, terminal growth 0%).

RISKS

Novo Nordisk A/S

Upside risks: 1. The major FDA approved growth drivers (Ozempic, Wegovy and Oral-Sema) and pipeline (in particular Cagri-Sema) deliver the consensus estimates 2. Novo’s manufacturing scale means that patent expiries are even more benign than we expect 3. Management engages in major, value accretive, bridging M&A.

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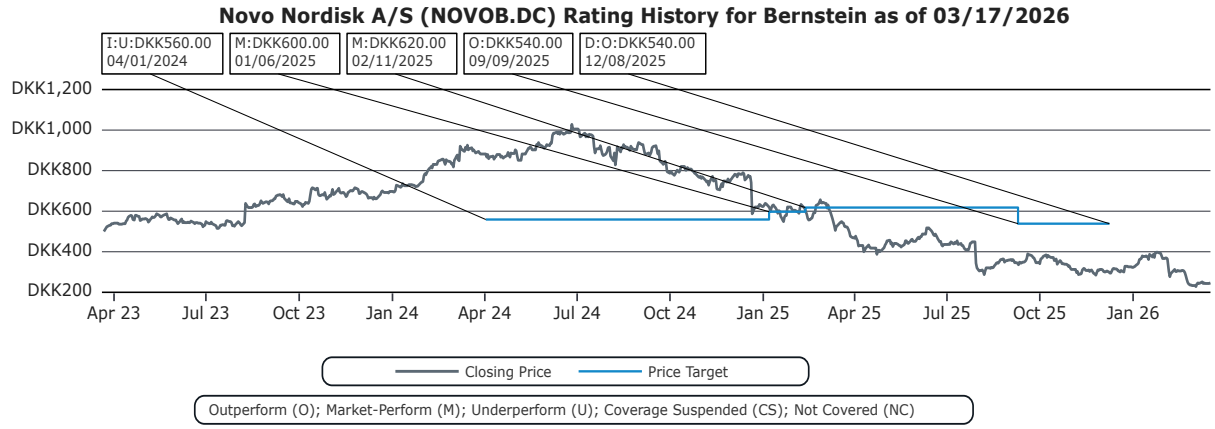
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